AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE OF PAGES
	1 | 6

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00014	4/6/2020	0000HCCP-2020-43721	

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Centers for Disease Control and Prevention (CDC) Office of Acquisition Services (OAS) 2900 Woodcock Blvd, MS TCU-4 Atlanta, GA 30341-4004	8219		

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2970 CLAIRMONT RD STE 720 ATLANTA, GA 30329-1636		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
CODE 838206503 FACILITY CODE		10B. DATED *(See Item 13)* 11/28/2016

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods: (a) By completing Items 8 and 15, and returning _____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA *(If required)*

939ZNJH 2512 2020 75-X-0946 W614111601 Increase (b)(4)

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES *(such as changes in paying office, appropriation date, etc.)* SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).
	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:
X	D. OTHER *(Specify type of modification and authority)* FAR 52.232-22, Limitation of Funds

E. IMPORTANT: Contractor ☐ is not, ☒ is required to sign this document and return 1 copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*

See page 2.

(b)(6)

COR: Andrew Weatherm, acw5@cdc.gov

CO: Lauren Peel, ijt9@cdc.gov

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*	16A. NAME OF CONTRACTING OFFICER
WARREN J STRAUSS - PRESIDENT KARNA, LLC	Lauren Peel
15B. CONTRACTOR/OFFEROR	**15C. DATE SIGNED**	**16B. UNITED STATES OF AMERICA**	**16C. DATE SIGNED**
(Signature of person authorized to sign)	04/06/2020	BY *(Signature of Contracting Officer)*	4/6/2020

The purpose of this modification is to:

a. Replace Section G.11 Government Furnished Materials, Information and Assistance in full, as shown below;
b. Increase the ceilings of Option Period 1 CLINs, 1001 – 1006, and increase the funding to fully fund each of the increased ceilings under CLINs 1001 – 1006, for a total increase of (b)(4) as reflected in the chart below:

CLIN	Previous CLIN Ceiling	Revised CLIN Ceiling	Total Increase
1001	(b)(4)		
1002			
1003			
1004			
1005			
1006			
1007			
1008			
Total OY 1			

c. Increase the ceiling of CLIN 3006 by (b)(4)
d. Fully fund CLIN 3006 by increasing the funding by (b)(4)
e. Increase the funding of CLINs 3001-3003, for a total increase of (b)(4) as reflected in the chart below:

CLIN	Current CLIN Ceiling	Previously Funded Amount	Revised Funding Amount	Total Funding Increase
3001	(b)(4)			
3002				
3003				
3004				
3005				
3006				
3007				
Total OY 3				

f. As a result, the total contract value is increased by (b)(4) and the total contract funding is increased by (b)(4)

Until this contract is fully funded, the requirements of the clause at FAR 52.232–22, Limitation of Funds, shall govern. Once the contract is fully funded, the requirements of the clause at FAR 52.232-20, Limitation of Cost, govern.

Section B - Supplies Or Services and Prices/Costs

Option Period One Items:

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
1001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)			
1002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)			
1003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)			
1004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)			

1005	Finance and Accounting All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)			
1006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)			

Option Period Three Items:

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
3001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/30/2020	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)			
3002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/30/2020	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)			

3003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/30/2020	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)			
3006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/30/2020	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)			

Section G - Contract Administration Data

G.11 Government Furnished Materials, Information and Assistance
CDCA_H040 Government Property (Jul 2017)

(a) Government-Furnished Property (GFP). In accordance with the terms of FAR 52.245-1, Government Property, the Government reserves the right to supply the Contractor, as Government-furnished property, any additional supplies, equipment, and materials determined by the Contracting Officer to be necessary and in the best interest of the Government.

(b) Contractor-Acquired Property (CAP). The Contractor must receive written consent from the Contracting Officer prior to purchase of any CAP not expressly identified in the contract, and as defined in FAR 52.245-1.

(c) Accountable and Sensitive Government Property. The Government will provide property labels and other identification for contractor-acquired Government property that is considered Accountable as defined in the HHS Logistics Management Manual (LMM) https://intranet.hhs.gov/abouthhs/manuals/lmm/index.html or considered Sensitive as defined in CDC's Sensitive Items List (http://intranet.cdc.gov/ofr/documents/contracts/Authorized-Prohibited-List.pdf)

(d) The contractor shall be responsible for the control and accountable record keeping of any Government property used in the performance of this contract predominately outside the confines of a Government controlled workspace in accordance with the HHS Contracting Guide found on the OSSAM Government Property and Contractors Property intranet page. (http://intranet.cdc.gov/ossam/property-shipping-receiving/property-management/government-property-contractors/index.html)

(e) The Chief of the Office of Safety, Security and Asset Management (OSSAM), Asset Management Services Office, Centers for Disease Control and Prevention (CDC), is hereby designated as the Property Administrator for this contract. The Contractor shall identify each item of equipment furnished by the Government to the Contractor or acquired by the Contractor using contract funds, with a suitable decal, tag, or other marking, as prescribed by the Property Administrator, and shall follow the guidance set forth in the HHS Contracting Guide.

(End of Clause)

G.11.a Government-Furnished Equipment

APPLIANCE, SECURITY	MCAFEE	(b)(4)
MODULE, FAIL-OPEN, COPPER	MCAFEE	
MODULE, FAIL-OPEN, COPPER	MCAFEE	
MODULE, FAIL-OPEN, COPPER	MCAFEE	
MODULE, FAIL-OPEN, COPPER	MCAFEE	
SERVER	ORACLE	
DRIVE, TAPE, LTO	IBM - INTERNATIONAL BUSINESS MACHINES	
ROUTER	CISCO SYSTEMS, INC.	
SWITCH	BROCADE COMMUNICATIONS SYSTEMS	
SWITCH	BROCADE COMMUNICATIONS SYSTEMS	
SWITCH, 48-PORT	CISCO SYSTEMS, INC.	
SWITCH, 48-PORT	CISCO SYSTEMS, INC.	
UNIT, EXPANSION, STORAGE	HITACHI	
UNIT, BASE, STORAGE	HITACHI	
FIREWALL	MCAFEE	
FIREWALL	MCAFEE	
FIREWALL	MCAFEE	
SERVER	IBM - INTERNATIONAL BUSINESS MACHINES	
SWITCH	F5 NETWORKS	
SERVER	IBM - INTERNATIONAL BUSINESS MACHINES	
SERVER	IBM - INTERNATIONAL BUSINESS MACHINES	

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE OF PAGES
	1 | 3

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00015	04/13/2020	0000HCCP-2020-44432	

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Centers for Disease Control and Prevention (CDC) Office of Acquisition Services (OAS) 2900 Woodcock Blvd, MS TCU-4 Atlanta, GA 30341-4004	8219		

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2970 CLAIRMONT RD STE 720 ATLANTA, GA 30329-1638		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
		10B. DATED *(See Item 13)* 11/28/2016
CODE 828268503	FACILITY CODE	

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended.
Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods:
(a) By completing Items 8 and 15, and returning ____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA *(If required)*
939ZNJH 2512 2020 75-X-0946 W614111601 Increase (b)(4)

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES *(such as changes in paying office, appropriation date, etc.)* SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).
	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:
x	D. OTHER *(Specify type of modification and authority)* FAR 52.217-9 - Option to Extend the Term of the Contract

E. IMPORTANT: Contractor ☒ is not, ☐ is required to sign this document and return _____ copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*

See page 2.

(b)(6)

COR: Andrew Weathers, aqw5@cdc.gov

CO: Lauren Peel, ijt9@cdc.gov

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*		16A. NAME OF CONTRACTING OFFICER	
		Lauren Peel	
15B. CONTRACTOR/OFFEROR	15C. DATE SIGNED	16B. UNITED STATES OF AMERICA	16C. DATE SIGNED
_____ *(Signature of person authorized to sign)*		BY _____ *(Signature of Contracting Officer)*	4/13/2020

The purpose of this modification is to:

a. Exercise Option Period 4 of the contract for the period from June 1, 2020, through May 31, 2021;
b. Incrementally fund Option Period 4 in the amount of (b)(4), as reflect by CLIN in the chart below; and

CLIN	CEILING AMOUNT	FUNDED AMOUNT
4001	(b)(4)	
4002		
4003		
4004		
4005		
4006		
4007		
Total		

c. Increase the funded amount of the contract by (b)(4)

Until this contract is fully funded, the requirements of the clause at FAR 52.232–22, Limitation of Funds, shall govern. Once the contract is fully funded, the requirements of the clause at FAR 52.232-20, Limitation of Cost, govern. The total value of option period 4 is (b)(4)

All other terms and conditions remain in effect.

Section B - Supplies Or Services and Prices/Costs

Option Period Four Items:

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
4001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2020 - 05/30/2021	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)			
4002	Member Svs Medical Benefits Management All work to be performed in accordauce with the Performauce Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2020 - 05/30/2021	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)			

4003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2020 - 05/30/2021	1 Each	(b)(4)
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)		
4004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2020 - 05/30/2021	1 Each	(b)(4)
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)		
4005	Finance and Accounting Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2020 - 05/30/2021	1 Each	(b)(4)
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)		
4006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2020 - 05/30/2021	1 Each	(b)(4)
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)		
4007	Call Center All work to be performed in accordance with the Performance Work Statement. This is a Firm Fixed Price CLIN. Period of Performance: 06/01/2020 - 05/30/2021	1 Each	(b)(4)
	Line(s) Of Accounting: 939ZNJH 2512 2020 75-X-0946 W614111601 (b)(4)		

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE	OF	PAGES
	1		1

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00001	04/28/2017		

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070	3635	Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070	3635

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2987 CLAIRMONT RD STE 205 ATLANTA, GA 30329-1687		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
CODE 828268503 FACILITY CODE		10B. DATED *(See Item 13)* 11/28/2016

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods:

(a) By completing Items 8 and 15, and returning _____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA *(If required)*

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
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	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:
	D. OTHER *(Specify type of modification and authority)*

E. IMPORTANT: Contractor ☒ is not, ☐ is required to sign this document and return _____ copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*

The purpose of this administrative modification is to delegate Contracting Officer's Representative duties to Carol Lavrich.

Carol's contact information is as follows:

Carol Lavrich, Program Analyst
Phone: 202-245-0663
Mobile: 202-674-6015
Email: clavrich@cdc.gov

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*		16A. NAME OF CONTRACTING OFFICER	
		Abbie Jemmott	
15B. CONTRACTOR/OFFEROR	15C. DATE SIGNED	16B. UNITED STATES OF AMERICA	16C. DATE SIGNED
(Signature of person authorized to sign)		BY Abbie G. Jemmott -S *(Signature of Contracting Officer)*	Digitally signed by Abbie G. Jemmott -S DN: c=US, o=U.S. Government, ou=HHS, ou=CDC, ou=People, 0.9.2342.19200300.100.1.1=1001638535, cn=Abbie G. Jemmott -S Date: 2017.04.29 16:19:50 -04'00'

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE OF PAGES
	1 | 6

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00002	05/18/2017	0000HCCP-2017-10080	

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070	3635	Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070	3635

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2987 CLAIRMONT RD STE 205 ATLANTA, GA 30329-1687		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
		10B. DATED *(See Item 13)* 11/28/2016

CODE 828268503 FACILITY CODE

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12. ACCOUNTING AND APPROPRIATION DATA *(If required)*

939ZNJH 2512 2017 75-X-0946 W614111601 Increase (b)(4)

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
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	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:
X	D. OTHER *(Specify type of modification and authority)* FAR 52.217-9 Option to Extend the Term of the C ontract

E. IMPORTANT: Contractor ☐ is not, ☒ is required to sign this document and return 1 copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*

See Section 2

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*	16A. NAME OF CONTRACTING OFFICER
(b)(6)	Abbie Jemmott
15B. CONTRACTOR/OFFEROR	**15C. DATE SIGNED**
(Signature of person authorized to sign)	5/30/2017
16B. UNITED STATES OF AMERICA	**16C. DATE SIGNED**
BY Abbie G. Jemmott -S *(Signature of Contracting Officer)*	Digitally signed by Abbie G. Jemmott -S DN: c=US, o=U.S. Government, ou=HHS, ou=CDC, ou=People, 0.9.2342.19200300.100.1.1=1001638535, cn=Abbie G. Jemmott -S Date: 2017.05.31 08:27:44 -04'00'

SECTION 2
The purpose of this modification is to:

1. Revise the Health Insurance Portability & Accountability Act language, as found in Section H.7 of the contract. The new language hereby incorporated as H.7 is as follows:

H.7 HEALTH INSURANCE PORTABILITY & ACCOUNTABILITY ACT OF 1996 (MAY 2014)
To the extent that the Business Associate performs functions or activities on behalf of, or provides certain services to, the Covered Entity where the Business Associate creates, receives, maintains, or transmits "protected health information" (PHI), the following "HIPAA provisions" apply: the Health Insurance Portability and Accountability Act of 1996 (HIPAA) (Pub. L. 104–191; 42 U.S.C. § 1320d); the Health Information Technology for Economic and Clinical Health (HITECH) Act[1] (Pub. L. 111-5; 42 U.S.C. §§ 300jj *et seq.*); the HIPAA Privacy, Security, Breach Notification, and Enforcement Rules (45 C.F.R. pts. 160, 162, and 164); and HHS HIPAA policies.
For purposes of this contract, "Business Associate" shall mean Karna, LLC; "Covered Entity"[2] shall mean the WTC Health Program and any other NIOSH, CDC, or HHS components to the extent that they assist in administering the WTC Health Program and where PHI is involved. These terms are used as defined in 45 C.F.R. § 160.103.

The Business Associate Agreement provides the detailed legal obligations and requirements of the Business Associate and the Covered Entity, and is incorporated into this contract as Attachment #3.

At all times throughout the duration of this contract and until the Business Associate fulfills its obligations under this contract and the Business Associate Agreement, the Business Associate is subject to and shall comply with all applicable HIPAA provisions regarding business associates, as well as any updates to those provisions.

The parties acknowledge that the Business Associate is directly liable under HIPAA for the following violations:[3]

- Impermissible uses and disclosures of PHI;
- Failure to provide breach notification to the Covered Entity;
- Failure to provide access to a copy of electronic PHI to either the Covered Entity, the individual, or the individual's designee, as specified in the Business Associate Agreement;
- Failure to disclose PHI where required by the Secretary of HHS to investigate or determine the Business Associate's compliance with HIPAA;
- Failure to provide an accounting of disclosures; and
- Failure to comply with the requirements of the Security Rule.

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[1] The HITECH Act was passed as Title XIII of Division A and Title IV of Division B of the American Recovery and Reinvestment Act of 2009 (ARRA).
[2] The term "covered entity" is used in this section for ease of understanding. However, a more precise description of the application of HIPAA to the WTC Health Program is as follows:
HHS is a hybrid entity under HIPAA, meaning HHS is a covered entity that conducts business activities, including both covered and non-covered functions, and designates "health care components" in accordance with 45 C.F.R. § 164.105(a)(2)(iii)(D). 45 C.F.R. § 164.103. As a hybrid entity, HHS must designate any component that would "meet the definition of a covered entity or business associate if it were a separate legal entity" as a health care component; a health care component also may include a component only to the extent that it performs covered functions. 45 C.F.R. § 164.105(a)(2)(iii)(D). Accordingly, the WTC Health Program is a "health care component" of the covered entity, HHS; as are any other NIOSH, CDC, or HHS components if they would meet the definition of a covered entity or business associate if they were separate legal entities and only to the extent that they perform covered functions.
[3] Modifications to the HIPAA Privacy, Security, Enforcement, and Breach Notification Rules Under the Health Information Technology for Economic and Clinical Health Act and the Genetic Information Nondiscrimination Act; Other Modifications to the HIPAA Rules [Omnibus Rule], 78 Fed. Reg. 5566, 5598-99 (Jan. 25, 2013).

2. Incorporate additional language to supplement FAR 52.227-14, Rights in Data – General and FAR 52.227-17, Rights in Data – Special Works to the contract as H.14 Rights in Data. Supplemental language hereby incorporated as H.14 is as follows:

H.14 RIGHTS IN DATA

FAR 52.227-14, Rights in Data – General (May 2014), and FAR 52.227-17, Rights in Data—Special Works (Dec 2007), are incorporated by reference into this contract. Pursuant to FAR 52.227-14(d)(2) and FAR 52.227-17(d), this contract expressly provides additionally as follows:
Subject to pre-existing patent rights and copyrights in data not first produced in the performance of this contract, all data, including data collected, deliverables and products first produced or developed in the performance of this contract or necessary to perform this contract shall become and remain the property of the Government.
Data, deliverables and products first produced or developed in the performance of this contract or necessary to perform this contract include, but are not limited to the following:

- Warehouse data that includes all member information and all claims-related data at any stage of the claims process;
- Data related to member certifications and case management activities;
- All software systems, including source code, required for the collection and processing of program information, member information, claims, code books and formularies, and the provider network information;
- Contact information for all medical professionals who participate in the provider network.

All data, including data collected, deliverables and products first produced or developed in the performance of this contract or necessary to perform this contract shall be delivered to the Government upon the written direction of the Contracting Officer or the Contracting Officer's Representative at any time during the period of performance of this contract and at the end of this contract. All data delivered during the period of performance of this contract and at the end of this contract must be in a format and transferred in a manner acceptable to the Government.

Subcontracting. The Contractor shall include this clause in all subcontracts. The Contractor shall obtain from its subcontractors all data and rights as set forth above necessary to fulfill the Contractor's obligations to the Government under this contract.

If a subcontractor at any tier refuses to accept terms affording the Government these rights, the Contractor shall promptly notify the Contracting Officer of the refusal and shall not proceed with the subcontract award without authorization in writing from the Contracting Officer.

3. Exercise Option Period 1 of the contract and incrementally fund Option Period 1 by (b)(4)

4. Increase the overall cost ceiling of the Base Period by (b)(4) and incrementally fund the Base Period by the same.

5. The following represents the anticipated schedule for the Base Period and Option Period 1 CLINs:

BASE PERIOD: **12/01/17 - 05/31/17**

CLINs	FFP	Estimated Cost	Fixed Fee	Total Est CPFF + FFP	Obligation previous mods	Obligation current mod	Obligation Total
0001 0002 0003 0004 0005 0006 0007 0008	(b)(4)						
Base Pd Total							

OPTION PERIOD 1: 06/01/17 - 05/31/18

CLINs	FFP	Estimated Cost	Fixed Fee	Total Est CPFF + FFP	Obligation previous mods	Obligation current mod	Obligation Total
1001 1002 1003 1004 1005 1006 1007 1008				(b)(4)			
Opt Pd 1 Total							

6. Until this contract is fully funded, the requirements of the clause at FAR 52.232–22, Limitation of Funds, shall govern. Once the contract is fully funded, the requirements of the clause at FAR 52.232-20, Limitation of Cost, govern.

7. The contract is hereby modified with the following appropriation data:

BASE PERIOD ITEMS:

ITEM	SUPPLIES / SERVICES	ESTIMATED COST
0001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 0008. Period of Performance: 12/01/2016 - 05/30/2017 **Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601** (b)(4)	(b)(4)
0002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 0008. Period of Performance: 12/01/2016 - 05/30/2017 **Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601** (b)(4)	
0003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 0008. Period of Performance: 12/01/2016 - 05/30/2017 **Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601** (b)(4)	
0004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 0008. Period of Performance: 12/01/2016 - 05/30/2017 **Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601** (b)(4)	
0005	Finance and Accounting Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 0008. Period of Performance: 12/01/2016 - 05/30/2017	

Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

0006 Claims Adjudications Management

All work to be performed in accordance with the Performance Work Statement.
This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 0008.
Period of Performance: 12/01/2016 - 05/30/2017

(b)(4)

Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

ITEM	**SUPPLIES / SERVICES**	**FIRM FIXED PRICE**
0007	Call Center	(b)(4)

All work to be performed in accordance with the Performance Work Statement.
This CLIN effort is Firm Fixed Price.
Period of Performance: 12/01/2016 - 05/30/2017

Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

ITEM	**SUPPLIES / SERVICES**
0008	Fixed Fee

This CLIN includes the Fixed Fee for CLINs 0001, 0002, 0003, 0004, 0005, 0006.
Period of Performance: 12/01/2016 - 05/30/2017

Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

OPTION PERIOD 1 ITEMS:

ITEM	**SUPPLIES / SERVICES**	**ESTIMATED COST**
1001	Program Support/Data Management	(b)(4)

All work to be performed in accordance with the Performance Work Statement.
This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008.
Period of Performance: 06/01/2017 - 05/30/2018

Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

1002 Member Svs Medical Benefits Management

All work to be performed in accordance with the Performance Work Statement.
This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008.
Period of Performance: 06/01/2017 - 05/30/2018

Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

1003 Technical Architecture

All work to be performed in accordance with the Performance Work Statement.
This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008.
Period of Performance: 06/01/2017 - 05/30/2018

Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

1004 Solutions Architecture

All work to be performed in accordance with the Performance Work Statement.
This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008.
Period of Performance: 06/01/2017 - 05/30/2018

Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

1005	Finance and Accounting	(b)(4)

All work to be performed in accordance with the Performance Work Statement.
This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008.
Period of Performance: 06/01/2017 - 05/30/2018

Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

1006	Claims Adjudications Management	(b)(4)

All work to be performed in accordance with the Performance Work Statement.
This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008.
Period of Performance: 06/01/2017 - 05/30/2018

Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

ITEM	**SUPPLIES / SERVICES**	**FIRM FIXED PRICE**
1007	Call Center	(b)(4)

All work to be performed in accordance with the Performance Work Statement.
This CLIN effort is Firm Fixed Price.
Period of Performance: 06/01/2017 - 05/30/2018

Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

ITEM	**SUPPLIES / SERVICES**	**FIXED FEE**
1008	Fixed Fee	(b)(4)

This CLIN includes the Fixed Fee for CLINs 1001, 1002, 1003, 1004, 1005, 1006.
Period of Performance: 06/01/2017 - 05/30/2018

Line(s) Of Accounting: 939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE	OF	PAGES
	1		4

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00003	08/10/2017	0000HCCP-2017-15566	

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070	3635	Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070	3635

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2987 CLAIRMONT RD STE 205 ATLANTA, GA 30329-1687		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
		10B. DATED *(See Item 13)* 11/28/2016

CODE 828268503 FACILITY CODE

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods:

(a) By completing Items 8 and 15, and returning _____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may ba made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA *(If required)*

939ZNJH 2512 2017 75-X-0946 W614111601 Increase (b)(4)

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES *(such as changes in paying office, appropriation date, etc.)* SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).
	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:
X	D. OTHER *(Specify type of modification and authority)* FAR 52.232-22, Limitation of Funds

E. IMPORTANT: Contractor ☐ is not, ☒ is required to sign this document and return 1 copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*

See Section 2

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*	16A. NAME OF CONTRACTING OFFICER
(b)(6)	Abbie Jemmott

15B. CONTRACTOR/OFFEROR	15C. DATE SIGNED	16B. UNITED STATES OF AMERICA	16C. DATE SIGNED
(Signature of person authorized to sign)	8/10/2017	BY Abbie G. Jemmott -S *(Signature of Contracting Officer)*	Digitally signed by Abbie G. Jemmott -S Date: 2017.08.10 15:55:56 -04'00'

SECTION 2

The purpose of this modification is to:

1. Shift funds among Option Period 1 CLINs. A funding decrease in the amount of (b)(4) has been made for the following CLINs: 1002, 1004, 1005, and 1008 and shifted to the following CLINs: 1001, 1003, and 1006.

2. Obligate incremental funds in the amount of (b)(4) to Option Period 1.

3. Implement a limitation of spending for CLIN 1001, Case Management Activities. Case Management Activities shall not exceed (b)(4) for the period 06/01/2017 – 10/31/2017. Reimbursement shall not be anticipated for any costs, which exceed this limit.

4. The following represents the anticipated schedule for the Base Period and Option Period 1 CLINs:

BASE PERIOD: 12/01/17 - 05/31/17

CLINs	FFP	Estimated Cost	Fixed Fee	Total Est CPFF + FFP	Obligation previous mods	Obligation current mod	Obligation Total
0001	(b)(4)						
0002							
0003							
0004							
0005							
0006							
0007							
0008							
Base Pd Total							

OPTION PERIOD 1: 06/01/17 - 05/31/18

CLINs	FFP	Estimated Cost	Fixed Fee	Total Est CPFF + FFP	Obligation previous mods	Obligation current mod	Obligation Total
1001	(b)(4)						
1002							
1003							
1004							
1005							
1006							
1007							
1008							
Opt Pd 1 Total							

5. Until this contract is fully funded, the requirements of the clause at FAR 52.232–22, Limitation of Funds, shall govern. Once the contract is fully funded, the requirements of the clause at FAR 52.232-20, Limitation of Cost, govern.

6. The contract is hereby modified with the following appropriation data:

Option Period 1 Items:

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
1001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: **939ZNJH 2512 2017 75-X-0946 W614111601** (b)(4)			
1002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: **939ZNJH 2512 2017 75-X-0946 W614111601** (b)(4)			
1003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: **939ZNJH 2512 2017 75-X-0946 W614111601** (b)(4)			
1004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: **939ZNJH 2512 2017 75-X-0946 W614111601** (b)(4)			
1005	Finance and Accounting All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: **939ZNJH 2512 2017 75-X-0946 W614111601** (b)(4)			

1006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)

Line(s) Of Accounting:
939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

1007	Call Center All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Firm Fixed Price. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)

No Funding Action

1008	Fixed Fee This CLIN includes the Fixed Fee for CLINs 1001, 1002, 1003, 1004, 1005, 1006. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)

Line(s) Of Accounting:
939ZNJH 2512 2017 75-X-0946 W614111601 (b)(4)

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE OF PAGES
	1 | 4

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00004	11/03/2017	0000HCCP-2018-17510	

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070	3635	Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070	3635

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2987 CLAIRMONT RD STE 205 ATLANTA, GA 30329-1687		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
		10B. DATED *(See Item 13)* 11/28/2016

CODE 828268503 FACILITY CODE

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods:

(a) By completing Items 8 and 15, and returning _____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA *(If required)*

939ZNJH 2512 2018 75-X-0946 W614111601 Increase (b)(4)

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES *(such as changes in paying office, appropriation date, etc.)* SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).
	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:
X	D. OTHER *(Specify type of modification and authority)* FAR 52.232-22, Limitation of Funds

E. IMPORTANT: Contractor ☒ is not, ☐ is required to sign this document and return _____ copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*

See Section 2

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*		16A. NAME OF CONTRACTING OFFICER	
		Abbie Jemmott	
15B. CONTRACTOR/OFFEROR *(Signature of person authorized to sign)*	15C. DATE SIGNED	16B. UNITED STATES OF AMERICA BY Abbie G. Jemmott -S *(Signature of Contracting Officer)*	16C. DATE SIGNED Digitally signed by Abbie G. Jemmott -S Date: 2017.11.03 12:42:11 -04'00'

SECTION 2

The purpose of this modification is to:

1. Obligate incremental funds in the amount of (b)(4) to Option Period 1.

2. Implement a limitation of spending for CLIN 1001, Case Management Activities. Case Management Activities shall not exceed (b)(4) for the period 06/01/2017 – 05/31/2018. Reimbursement shall not be anticipated for any costs, which exceed this limit.

3. The following represents the anticipated schedule for the Base Period and Option Period 1 CLINs:

BASE PD 12/01/17 -05/31/17

CLINs	FFP	Estimated Cost	Fixed Fee	Total Est CPFF + FFP	Obligation previous mods	Obligation current mod	Obligation Total
0001 0002 0003 0004 0005 0006 0007 0008	(b)(4)						
Base Pd Total							

OPTION PD 1 06/01/17 -05/31/18

CLINs	FFP	Estimated Cost	Fixed Fee	Total Est CPFF + FFP	Obligation previous mods	Obligation current mod	Obligation Total
1001 1002 1003 1004 1005 1006 1007 1008	(b)(4)						
Opt Pd 1 Total							

4. Until this contract is fully funded, the requirements of the clause at FAR 52.232–22, Limitation of Funds, shall govern. Once the contract is fully funded, the requirements of the clause at FAR 52.232-20, Limitation of Cost, govern.

5. The contract is hereby modified with the following appropriation data:

Option Period 1 Items

ITEM	SUPPLIES / SERVICES	UNIT	UNIT PRICE	EXTENDED PRICE
1001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	

Line(s) Of Accounting:
939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)

1002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	

Line(s) Of Accounting:
939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)

1003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	

Line(s) Of Accounting:
939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)

1004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	

Line(s) Of Accounting:
939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)

1005	Finance and Accounting All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)

Line(s) Of Accounting:
939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)

1006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)

Line(s) Of Accounting:
939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)

1008	Fixed Fee This CLIN includes the Fixed Fee for CLINs 1001, 1002, 1003, 1004, 1005, 1006. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)

Line(s) Of Accounting:
939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE OF PAGES
	1 | 3

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00005	02/27/2018	0000HCCP-2018-18647	

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070	3635	Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070	3635

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2987 CLAIRMONT RD STE 205 ATLANTA, GA 30329-1687		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
		10B. DATED *(See Item 13)* 11/28/2016

CODE 828268503 FACILITY CODE

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods:

(a) By completing Items 8 and 15, and returning _____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA *(If required)*

939ZNJH 2512 2018 75-X-0946 W614111601 Increase (b)(4)

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES *(such as changes in paying office, appropriation date, etc.)* SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).
X	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF: **WITHIN SCOPE SUPPLEMENTAL AGREEMENT BY MUTUAL AGREEMENT**
	D. OTHER *(Specify type of modification and authority)*

E. IMPORTANT: Contractor ☐ is not, ☒ is required to sign this document and return __1__ copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*

See Section 2

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*		16A. NAME OF CONTRACTING OFFICER	
(b)(6)		Abbie Jemmott	
15B. CONTRACTOR/OFFEROR *(Signature of person authorized to sign)*	15C. DATE SIGNED 3/26/2018	16B. UNITED STATES OF AMERICA BY Abbie G. Jemmott -S *(Signature of Contracting Officer)*	16C. DATE SIGNED Digitally signed by Abbie G. Jemmott -S Date: 2018.03.27 09:55:05 -04'00'

SECTION 2

The purpose of this modification:

1. Removes the requirements of FAR 52.219-14, Limitation on Subcontracting, as the contractor is no longer a small business concern and adds the following clauses by reference: FAR 52.219-8 Utilization of Small Business Concerns (Nov 2016), FAR 52.219-9 Small Business Subcontracting Plan Alt IV (Jan 2017), and FAR 52.219-16 Liquidated Damages – Subcontracting Plan (Jan 1999).

2. Adds the following clause in full text to Section G:

   Electronic Subcontracting Reporting System (eSRS) (Dec 2005)
   The contractor shall register with the Electronic Subcontracts Reporting System (eSRS) for the submission of its Individual Subcontract Report (SF 294) and the Annual Summary Reports (SF 295). Before registering in eSRS, the contractor information must be correct in Central Contractor Registration database. The eSRS is a world wide webbased application available at: http://www.esrs.gov. The eSRS website provides training and instruction for data submission.

3. Increases the cost ceiling of CLIN 1001 from (b)(4) by (b)(4) and incrementally funds CLIN 1001 in the amount of (b)(4)

OPTION PD 1	
CLIN 1001	(b)(4)
CLIN 1002	
CLIN 1003	
CLIN 1004	
CLIN 1005	
CLIN 1006	
CLIN 1007	
CLIN 1008	
Total	

4. Implements a limitation of spending for CLIN 1001, Case Management Activities. Case Management Activities shall not exceed (b)(4) for the period 06/01/2017 – 05/31/2018. Reimbursement shall not be anticipated for any costs, which exceed this limit.

5. The following represents the anticipated schedule for Option Period 1 CLINs:

OPTION PD 1	**06/01/17**	**-05/31/18**					
CLINs	**FFP**	**Estimated Cost**	**Fixed Fee**	**Total Est CPFF + FFP**	**Obligation previous mods**	**Obligation current mod**	**Obligation Total**
1001	(b)(4)						
1002							
1003							
1004							
1005							
1006							
1007							
1008							
Opt Pd 1 Total							

6. Until this contract is fully funded, the requirements of the clause at FAR 52.232–22, Limitation of Funds, shall govern. Once the contract is fully funded, the requirements of the clause at FAR 52.232-20, Limitation of Cost, govern.

7. The contract is hereby modified with the following appropriation data:

Option Period 1 Items:

ITEM	SUPPLIES / SERVICES	UNIT	NOT TO EXCEED
1001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. **This CLIN has a limitation of spending for Case Management activities capped at** (b)(4) This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 YEAR	(b)(4)

Line(s) Of Accounting:
939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE OF PAGES
	1 | 3

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00006	04/23/2018	0000HCCP-2018-21740	

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070	3635	Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070	3635

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2970 CLAIRMONT RD STE 720 ATLANTA, GA 30329-1638		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
		10B. DATED *(See Item 13)* 11/28/2016

CODE 828268503 FACILITY CODE

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods:

(a) By completing Items 8 and 15, and returning _____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may ba made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA *(If required)*

939ZNJH 2512 2018 75-X-0946 W614111601 Increase (b)(4)

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES *(such as changes in paying office, appropriation date, etc.)* SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).
X	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF: MUTUAL AGREEMENT OF THE PARTIES
	D. OTHER *(Specify type of modification and authority)*

E. IMPORTANT: Contractor ☐ is not, ☒ is required to sign this document and return 1 copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*

See Section 2

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*	16A. NAME OF CONTRACTING OFFICER
(b)(6)	Abbie Jemmott

15B. CONTRACTOR/OFFEROR	15C. DATE SIGNED	16B. UNITED STATES OF AMERICA	16C. DATE SIGNED
Karna *(Signature of person authorized to sign)*	4/26/2018	BY Abbie G. Jemmott -S *(Signature of Contracting Officer)*	Digitally signed by Abbie G. Jemmott -S Date: 2018.04.26 11:34:36 -04'00'

SECTION 2

The purpose of this modification is to:

1. Adjust the cost ceiling of several CLINs in Option Period 1 to account for the contractor's estimate to complete the period of performance. CLIN 1001 is increased by (b)(4) CLIN 1002 is decreased by (b)(4) CLIN 1003 is increased by (b)(4) CLIN 1004 is decreased by (b)(4) ; CLIN 1005 is decreased by (b)(4) and CLIN 1006 is increased by (b)(4) This results in a net de-scope to Option Period 1 in the amount of (b)(4)

OPTION PD 1	
CLIN 1001	(b)(4)
CLIN 1002	
CLIN 1003	
CLIN 1004	
CLIN 1005	
CLIN 1006	
CLIN 1007	
CLIN 1008	
Total	

2. Fund Option Period 1 in the amount of (b)(4)

3. The following represents the anticipated schedule for Option Period 1 CLINs:

OPTION PD 1	**06/01/17**	**-05/31/18**					
CLINs	**FFP**	**Estimated Cost**	**Fixed Fee**	**Total Est CPFF + FFP**	**Obligation previous mods**	**Obligation current mod**	**Obligation Total**
1001	(b)(4)						
1002							
1003							
1004							
1005							
1006							
1007							
1008							
Opt Pd 1 Total							

4. Until this contract is fully funded, the requirements of the clause at FAR 52.232–22, Limitation of Funds, shall govern. Once the contract is fully funded, the requirements of the clause at FAR 52.232-20, Limitation of Cost, govern.

5. The contract is hereby modified with the following appropriation data:

Option Period 1 Items:

ITEM	SUPPLIES / SERVICES	UNIT	NOT TO EXCEED
1001	Program Support/Data Management - all work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018 **Line(s) Of Accounting:** **939ZNJH 2512 2018 75-X-0946 W614111601** (b)(4)	1 Year	(b)(4)
1002	Member Svs Medical Benefits Management - all work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018 **Line(s) Of Accounting:** **939ZNJH 2512 2018 75-X-0946 W614111601** (b)(4)	1 Year	(b)(4)
1003	Technical Architecture - all work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018 **Line(s) Of Accounting:** **939ZNJH 2512 2018 75-X-0946 W614111601** (b)(4)	1 Year	(b)(4)
1004	Solutions Architecture - all work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018 **Line(s) Of Accounting:** **939ZNJH 2512 2018 75-X-0946 W614111601** (b)(4)	1 Year	(b)(4)
1005	Finance and Accounting - all work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018 **Line(s) Of Accounting:** **939ZNJH 2512 2018 75-X-0946 W614111601** (b)(4)	1 Year	(b)(4)
1006	Claims Adjudications Management - all work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018 **Line(s) Of Accounting:** **939ZNJH 2512 2018 75-X-0946 W614111601** (b)(4)	1 Year	(b)(4)
1007	Call Center - all work to be performed in accordance with the Performance Work Statement. This CLIN effort is Firm Fixed Price. Period of Performance: 06/01/2017 - 05/30/2018	1 Year	(b)(4)
1008	Fixed Fee This CLIN includes the Fixed Fee for CLINs 1001, 1002, 1003, 1004, 1005, 1006. Period of Performance: 06/01/2017 - 05/30/2018	1 Year	(b)(4)

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE OF PAGES
	1 | 1

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00007	04/30/2018		

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Centers for Disease Control and Prevention (CDC) Office of Acquisition Services (OAS) 2920 Brandywine Road Atlanta, GA 30341-5539	8219		3635

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2970 CLAIRMONT RD STE 720 ATLANTA, GA 30329-1638		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
CODE 828268503 FACILITY CODE		10B. DATED *(See Item 13)* 11/28/2016

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods:

(a) By completing Items 8 and 15, and returning _____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA *(If required)*

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
x	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES *(such as changes in paying office, appropriation date, etc.)* SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).
	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:
	D. OTHER *(Specify type of modification and authority)*

E. IMPORTANT: Contractor ☒ is not, ☐ is required to sign this document and return _____ copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*

This modification removes Abbie Jemmott as Contracting Officer and transfers authority to Sherrie Randall, effective April 30, 2018. All correspondence and deliveries made to the Contracting Officer shall be received by Sherrie Randall via email at SRandall@cdc.gov and telephone 770.488.2866.

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*		16A. NAME OF CONTRACTING OFFICER	
		Abbie Jemmott	
15B. CONTRACTOR/OFFEROR *(Signature of person authorized to sign)*	15C. DATE SIGNED	16B. UNITED STATES OF AMERICA BY Abbie G. Jemmott -S *(Signature of Contracting Officer)*	16C. DATE SIGNED Digitally signed by Abbie G. Jemmott -S Date: 2018.04.27 09:34:14 -04'00'

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE OF PAGES
	1 | 7

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00012	01/22/2019	0000HCCP-2019-29758	

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Centers for Disease Control and Prevention (CDC) Office of Acquisition Services (OAS) 2920 Brandywine Road Atlanta, GA 30341-5539	8219		

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2970 CLAIRMONT RD STE 720 ATLANTA, GA 30329-1638		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
		10B. DATED *(See Item 13)* 11/28/2016
CODE 828268503	FACILITY CODE	

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended.
Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods:

(a) By completing Items 8 and 15, and returning _____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA *(If required)*
See Section B

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES *(such as changes in paying office, appropriation date, etc.)* SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).
	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:
x	D. OTHER *(Specify type of modification and authority)* FAR 43.103(a)(3) Bilateral

E. IMPORTANT: Contractor ☐ is not, ☒ is required to sign this document and return __1__ copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*

See page 2.

(b)(6)

COR: Andrew Weathers, aqw5@cdc.gov

CO: Lauren Peel, ijt9@cdc.gov

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*		16A. NAME OF CONTRACTING OFFICER	
(b)(6)		Annette Wright	
15B. CONTRACTOR/OFFEROR	15C. DATE SIGNED	16B. UNITED STATES OF AMERICA	16C. DATE SIGNED
(Signature of person authorized to sign)	1/23/2019	BY Annette Wright -S Digitally signed by Annette Wright -S Date: 2019.01.24 09:05:15 -05'00' *(Signature of Contracting Officer)*	

The purpose of this modification is to:

a. Increase the cost ceilings and funded amounts of CLINs 2001 – 2007 as reflected in the chart below:

CLIN	From*	To*	Difference
2001	(b)(4)		
2002			
2003			
2004			
2005			
2006			
2007			
Total			

b. Increase the cost ceilings of CLINs 3001 – 3007 and 4001 – 4007 as reflected in the chart below:

CLIN	From	To	Difference
3001	(b)(4)		
3002			
3003			
3004			
3005			
3006			
3007			
Total			
4001			
4002			
4003			
4004			
4005			
4006			
4007			
Total			

c. Increase the total value of the contract by (b)(4) from (b)(4) to (b)(4)

d. Increase the total funded amount of the contract by (b)(4) from (b)(4) to (b)(4) and

e. Replace Attachment J-1, Health Program Support Performance Work Statement.

*Charts reflect total estimated costs including fixed fees. Estimated costs and fixed fees are broken down separately in Section B below.

Section B - Supplies Or Services and Prices/Costs

Option Two Items:

ITEM	SUPPLIES / SERVICES	EST. COST	FIXED FEE	TOTAL EST. CPFF
2001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2018 - 05/30/2019	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2019 75-X-0946 W614111601 (b)(4)			
2002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2018 - 05/30/2019	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2019 75-X-0946 W614111601 (b)(4)			
2003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2018 - 05/30/2019	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2019 75-X-0946 W614111601 (b)(4)			
2004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2018 - 05/30/2019	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2019 75-X-0946 W614111601 (b)(4)			

2005	Finance and Accounting Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2018 - 05/30/2019	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2019 75-X-0946 W614111601 (b)(4)			
2006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2018 - 05/30/2019	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)			
ITEM	SUPPLIES / SERVICES	UNIT	UNIT PRICE	EXTENDED PRICE
2007	Call Center All work to be performed in accordance with the Performance Work Statement. This is a Firm Fixed Price CLIN. Period of Performance: 06/01/2018 - 05/30/2019	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2019 75-X-0946 W614111601 (b)(4)			

Option Three Items:

ITEM	SUPPLIES / SERVICES	EST. COST	FIXED FEE	TOTAL EST. CPFF
3001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/30/2020	(b)(4)		

3002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/30/2020	(b)(4)		
3003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/30/2020	(b)(4)		
3004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/30/2020	(b)(4)		
3005	Finance and Accounting Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/30/2020	(b)(4)		
3006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/30/2020	(b)(4)		
ITEM	SUPPLIES / SERVICES	UNIT	UNIT PRICE	EXTENDED PRICE
3007	Call Center All work to be performed in accordance with the Performance Work Statement. This is a Firm Fixed Price CLIN. Period of Performance: 06/01/2019 - 05/30/2020	1 Each	(b)(4)	

Option Four Items:

ITEM	SUPPLIES / SERVICES	EST. COST	FIXED FEE	TOTAL EST. CPFF
4001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2020 - 05/30/2021	(b)(4)		
4002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2020 - 05/30/2021	(b)(4)		
4003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2020 - 05/30/2021	(b)(4)		
4004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2020 - 05/30/2021	(b)(4)		
4005	Finance and Accounting Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2020 - 05/30/2021	(b)(4)		
4006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2020 - 05/30/2021	(b)(4)		
ITEM	SUPPLIES / SERVICES	UNIT	UNIT PRICE	EXTENDED PRICE

4007	Call Center All work to be performed in accordance with the Performance Work Statement. This is a Firm Fixed Price CLIN. Period of Performance: 06/01/2020 - 05/30/2021	1 Each	(b)(4)	

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE	OF	PAGES
	1		3

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00013	05/29/2019	0000HCCP-2019-33665	

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Centers for Disease Control and Prevention (CDC) Office of Acquisition Services (OAS) 2920 Brandywine Road Atlanta, GA 30341-5539	8219		

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2970 CLAIRMONT RD STE 720 ATLANTA, GA 30329-1638		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
		10B. DATED *(See Item 13)* 11/28/2016
CODE 828268503	FACILITY CODE	

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods:

(a) By completing Items 8 and 15, and returning _____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA *(If required)*

N/A

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES *(such as changes in paying office, appropriation date, etc.)* SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).
	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:
X	D. OTHER *(Specify type of modification and authority)* FAR 52.217-9 - Option to Extend the Term of the Contract

E. IMPORTANT: Contractor ☒ is not, ☐ is required to sign this document and return ___ copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*

See page 2.

(b)(6)

COR: Andrew Weathers, aqw5@cdc.gov

CO: Lauren Peel, ijt9@cdc.gov

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*		16A. NAME OF CONTRACTING OFFICER Annette Wright	
15B. CONTRACTOR/OFFEROR _____ *(Signature of person authorized to sign)*	15C. DATE SIGNED	16B. UNITED STATES OF AMERICA BY _____ *(Signature of Contracting Officer)*	16C. DATE SIGNED

The purpose of this modification is to:

a. Exercise Option Period 3 of the contract for the period from June 1, 2019, through May 31, 2020; and
b. Incrementally fund Option Period 3 in the amount of (b)(4)
c. Increase the funded amount of the contract by (b)(4) from (b)(4) to (b)(4) and
d. Until this contract is fully funded, the requirements of the clause at FAR 52.232 22, Limitation of Funds, shall govern. Once the contract is fully funded, the requirements of the clause at FAR 52.232-20, Limitation of Cost, govern. The total value of option 3 is (b)(4)

All other terms and conditions remain in effect.

Section B - Supplies Or Services and Prices/Costs

Option Period Three Items:

ITEM	SUPPLIES / SERVICES	EST. COST	FIXED FEE	TOTAL EST. CPFF
3001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/31/2020	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2019 75-X-0946 W614111601 (b)(4)			
3002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/31/2020	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2019 75-X-0946 W614111601 (b)(4)			
3003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/31/2020	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2019 75-X-0946 W614111601 (b)(4)			

3004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/31/2020	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2019 75-X-0946 W614111601 (b)(4)			
3005	Finance and Accounting Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/31/2020	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2019 75-X-0946 W614111601 (b)(4)			
3006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2019 - 05/31/2020	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2019 75-X-0946 W614111601 (b)(4)			
ITEM	SUPPLIES / SERVICES	UNIT	UNIT PRICE	EXTENDED PRICE
3007	Call Center All work to be performed in accordance with the Performance Work Statement. This is a Firm Fixed Price CLIN. Fully funded Period of Performance: 06/01/2019 - 05/31/2020	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2019 75-X-0946 W614111601 (b)(4)			

Information Technology Security aud Privacy

1. Information Security Requirements

1.1. Applicability.

This statement applies whether the entire contract or order (hereafter "contract"), or portion thereof, includes either or both of the following:

1.1.1 Access (Physical or Logical) to Federal Information: A Contractor (and/or any subcontractor) employee will have, or the ability to have, physical or logical (electronic) access to federal information.

1.1.2. Operate a Federal System Containing Information: A Contractor (and/or any subcontractor) will operate a federal information technology system containing information that directly supports the CDC mission. In addition to the FAR Subpart 2.1 definition of "information technology" (IT), the term as used in this section, includes computers, ancillary equipment (including imaging peripherals, input, output, and storage devices necessary for security and surveillance), peripheral equipment designed to be controlled by the central processing unit of a computer, software, firmware and similar procedures, services (including support services) and related resources. This section does not apply to national security systems as defined by FISMA.

1.2. Contractor Respousibilities.

The Contractor (and/or any subcontractor) is responsible for the following:

1.2.1. Protecting federal information and information systems in order to ensure their:

1.2.1.1. Integrity

Which means guarding against improper information modification or destruction, and ensuring information non-repudiation and authenticity;

1.2.1.2. Confidentiality

Which means preserving authorized restrictions on access and disclosure, including means for protecting personal privacy and proprietary information;

1.2.1.3 Availability

Which means ensuring timely and reliable access to and use of information.

1.2.2. Providing security of any Contractor systems, and information contained therein, connected to a CDC network or operated by the Contractor on behalf of CDC regardless of location.

1.2.3. Adopting, and implementing, at a minimum, the policies, procedures, controls, and standards of the CDC Information Security Program to ensure the integrity, confidentiality, and availability of federal information and federal information systems for which the Contractor is responsible under this contract or to which the Contractor may otherwise have access under this contract. The CDC security requirements are outlined in the CDC Cyber Security Action Plan (CSAP), which is available on the CDC Office of the Chief Information Security Officer's (OCISO) website.

1.3. Information Type(s).

The following information type(s) apply to this contract: Administrative, Management and Support Information and Information Systems:

- Budget Execution
- Workforce Planning
- Management Improvement
- Budgeting & Performance Integration
- Contingency Planning
- Continuity of Operations
- Service Recovery

- Customer Services
- Official Information Dissemination
- Product Outreach
- Public Relations
- Facilities, Fleet, and Equipment Management
- Help Desk Services
- Security Management
- Travel
- Workplace Policy Development and Management
- Asset and Liability Management
- Reporting and Information
- Funds Control
- Accounting
- Cost Accounting/ Performance Measurement
- HR Strategy
- Staff Acquisition
- Organization and Position Management
- Compensation Management
- Benefits Management
- Employee Performance Management
- Employee Relations
- Labor Relations
- Separation Management
- Human Resources Development
- System Development
- Lifecycle/Change Management
- System Maintenance
- IT Infrastructure Maintenance
- Information System Security
- Record Retention
- Information Management
- System and Network Monitoring
- Information Sharing

1.4. Security Categories and Levels.
The following risk levels apply to this contract:
Security Categories and Levels

Confidentiality Level:	[ ] Low	[X] Moderate	[ ] High
Integrity Level:	[ ] Low	[X] Moderate	[ ] High
Availability Level:	[ ] Low	[X] Moderate	[ ] High
Overall Security Category:	[ ] Low	[X] Moderate	[ ] High

1.5. Internet Protocol Version 6 (IPv6).
All acquisitions for information technology using Internet Protocol must comply with the Federal Acquisition Regulation (FAR).

1.6. Roster.
The Contractor (and/or any subcontractor) shall submit a roster by name, position, e-mail address, phone number and responsibility, of all staff working under this acquisition where the Contractor will develop, have the ability to access, or host and/or maintain a federal information system(s). The roster shall be submitted to the Contracting Officer's Representative (COR), with a copy to the Contracting Officer, within **14 calendar days** of the effective date of this contract. Any revisions to the roster as a result of

staffing changes shall be submitted within **15 calendar days** of the change. The Contracting Officer will notify the Contractor of the appropriate level of investigation required for each staff member.

1.7. Document Information.
The Contractor shall contact the Contracting Officer and/or COR for any templates, policies, forms and other documents necessary to comply with the requirements of this section.

1.8. Standard for Encryption Language

1.8.1. If the Government has determined that CDC information under this contract is considered sensitive and/or the system is categorized "MODERATE or HIGH", the Contractor (and/or any subcontractor) shall use Federal Information Processing Standard (FIPS) 140-2, Security Requirements for Cryptographic Module, as amended, compliant encryption to protect all instances of CDC sensitive information during storage and transmission (Note: The Government has determined that CDC information under this contract is considered sensitive in accordance with CDC Standard for the Definition of Sensitive Information, dated May 2009 or latest revision).

1.8.2. The Contractor (and/or any subcontractor) shall verify that the selected encryption product has been validated under the Cryptographic Module Validation Program (see http://csrc.nist.gov/cryptval/) to confirm compliance with FIPS 140-2 (as amended). The Contractor shall provide a written copy of the validation documentation to the Contracting Officer and the COR.

1.8.3. The Contractor (and/or any subcontractor) shall use the Key Management Key (see FIPS 201, Chapter 4, as amended) on the CDC personal identification verification (PIV) card; or alternatively, the Contractor (and/or any subcontractor) shall establish and use a key recovery mechanism to ensure the ability for authorized personnel to decrypt and recover all encrypted information (See OMB Encryption Guidance (http://csrc.nist.gov/drivers/documents/ombencryption-guidance.pdf)). The Contractor shall notify the Contracting Officer and the COR of personnel authorized to decrypt and recover all encrypted information.

1.8.4. The Contractor (and/or any subcontractor) shall securely generate and manage encryption keys to prevent unauthorized decryption of information in accordance with FIPS 140-2 (as amended).

1.8.5. The Contractor (and/or any subcontractor) shall ensure that this standard is incorporated into the Contractor's property management/control system or establish a separate procedure to account for all laptop computers, desktop computers, and other mobile devices and portable media that store or process sensitive CDC information.

1.9. Contractor Responsibilities Upon Physical Completion of the Contract.
The Contractor shall return all CDC information and IT resources provided to the Contractor during contract performance and certify that all CDC information has been properly sanitized and purged, in accordance with the Department and/or WTCHP policies, from Contractor-owned (and/or any subcontractor) systems, including backup systems, used in contract performance.

1.10 Failnre to Comply.
Failure on the part of the Contractor (and/or any subcontractor) to comply with the terms of this section shall be grounds for the Contracting Officer to terminate this contract.

2. Information Security and Privacy Training

2.1. Mandatory Training.
All Contractor (and/or any subcontractor) employees shall complete the applicable CDC Information Security Awareness, Privacy, and Records Management training (http://intranet.cdc.gov/ocio/information-systems-security/security-awareness/training/index.html) before performing any work under this contract.

Thereafter, the employees shall complete CDC specified Information Security, Privacy and Records Management training annually, during the life of this contract.

2.2. Role-based Training

2.2.1. HHS requires role-based training when responsibilities associated with a given role or position, could, upon execution, have the potential to adversely impact the security posture of one or more HHS systems. Read further guidance at: HHS Memorandum, as amended, on Role-Based Training Requirement[1].

The Contractor (and/or any subcontractor) shall maintain training records for all information security and privacy training completed by each employee working under this contract. The training records shall be provided to the COR and/or Contracting Officer in conjunction with contract award or upon reqnest and WTCHP requires that the training recorded be submitted to the WTCHP Information Security Officer and the COR responsible for their contract annually.

2.2.2. The Contractor shall provide records of any applicable information security and privacy training completed outside of CDC to the COR responsible for their contract.

3. Rules of Behavior

The Contractor (and/or any subcontractor) shall ensure that all employees comply with the CDC Information Technology General Rules of Behavior (as amended), which are available at http://intranet.cdc.gov/ocio/docs/computers-software-equipment/policy90.pdf and http://intranet.cdc.gov/ocio/docs/computers-software-equipment/HHS%20Rules%20of%20Behavior.pdf and any CDC specific rules as applicable.

All Contractor employees much read and adhere to the Rnles of Behavior before accessing Department data or other information, systems, and/or networks that store/process CDC information and annually thereafter, which may be done as part of annual CDC Information Systems Security Awareness Training.

4. Personnel Security Responsibilities

4.1. Contractor New Employee.
The Contractor shall notify the Contracting Officer, COR, and designated WTCHP Point of Contact (POC) within **five (5) business days** before a new employee (including subcontractors) assumes a position that requires a suitability determination or when an employee with a suitability determination or security clearance stops working under this contract. The Agency will initiate a background investigation on new employees requiring suitability determination and will stop pending background investigations for employees that no longer work nnder this acquisition.

4.2. Contractor Requirement.
The Contractor shall provide the COR with the name, position title, e-mail address, and phone number of all contract employees (including subcontractors) working under the contract and provide the name, position title and suitability determination level held by the former incumbent. If the employee is filling a new position, the Contractor shall provide a position description and the Agency will determine the appropriate suitability level.

4.3. Contractor Separation.
The Contractor (and/or any subcontractor) shall perform and document the actions identified in the CDC In- and Out -Processing of CDC Employees, Non-employees, and Affiliates (http://masoapplications.cdc.gov/Policy/Doc/policy510.pdf) when a contractor (and/or any subcontractor)

[1] HHS Memorandum on Role Based Training Requirement retrieved from: http://intranet.hhs.gov/it/cybersecurity/policies/index.html

employee terminates work under this contract. All documentation shall be made available to the COR and/or Contracting Officer upon request.

5. Commitment to Protect Non-Public Departmental Information and Data (see Appendices A & B)

5.1. Unauthorized Disclosure.
The Contractor (and/or any subcontractor) shall protect information that is deemed sensitive from unauthorized disclosure to persons, organizations or subcontractors who do not have a need to know the information. Information which, either alone or in aggregate, is deemed sensitive or proprietary by the Government shall be protected as instructed in accordance with the magnitude of the loss or harm that could result from inadvertent or deliberate disclosure, alteration, or destruction of the data. This language also applies to all subcontractors that are performing under this contract.

5.2. Contractor Agreement.
The Contractor (and/or any subcontractor), performing under this contract, shall not release, publish, or disclose non-public CDC information to unauthorized personnel, and shall protect such information in accordance with provisions of the following laws and any other pertinent laws and regulations (as amended) governing the confidentiality of such information:

- 18 U.S.C. 641 (Criminal Code: Public Money, Property or Records)
- 18 U.S.C. 1905 (Criminal Code: Disclosure of Confidential Information)
- Public Law 96-511 (Paperwork Reduction Act)

5.3. Contractor Employee Non-Disclosure Agreement (NDA).
Each Contractor (and/or any subcontractor) employee having access to non-public federal information under this contract shall complete the CDC Commitment to Protect Non-Public Information – Contractor Employee Agreement. A copy of each signed and witnessed NDA shall be submitted to the COR prior to performing any work under this acquisition.

6. Incident Response

FISMA requires that agencies report all cyber security incidents to the United States Computer Emergency Readiness Team (US-CERT). All information security and privacy incidents, which include cyber and non-cyber (paper-based) incidents shall be reported to the WTCP Information System Security Officer (ISSO). WTCHP users should report all suspected computer security incidents to their local WTCHP ISSO. The WTCHP ISSO will notify the CDC CSIRT after assessing the situation, responding to any immediate requirements, and collecting essential information about the incident. Please note OMB 06-19 (as amended) requires HHS to report "all incidents involving personally identifiable information (PII) in electronic or physical form" to US-CERT **within one (1) hour**, and that we "should not distinguish between suspected and confirmed breaches". This means that from discovery WTCHP ISSO, CDC CSIRT, HHS CSIRC, and US-CERT need to be notified within that hour. Moreover, the WTCHP shall report all privacy breaches involving the loss or unauthorized disclosure of PII, including paper-based incidents, to the CDC Privacy Incident Response Team (PIRT).

(Note: CSIRT manages cyber-related incidents and reports them to US-CERT; however, PII incidents are generally managed by the WTCHP and CDC Privacy Programs and must be reported to the CDC PIRT.) For more information, please refer to the CDC - IT Security and Privacy Incident Reporting and Response Policy , as amended as well as WTCHPs incident response guidance please see Appendix C)

6.1. The Contractor (and/or any subcontractor) shall report all suspected and confirmed information security and privacy incidents to the WTCHP ISSO, including incidents involving PII in electronic or physical form, **within 1 hour**. The types of information required in a cyber incident report must include at a minimum: company and point of contact information, contract information, and the type of information compromised.

6.2. In the event of a sensitive information incident, the Government may suspend or revoke an existing authority to operate (ATO) (either in part or in whole). If an ATO is suspended or revoked in accordance with this provision, the Contracting Officer may direct the Contractor (and/or any subcontractor) to take additional security measures to secure sensitive information. These measures may include restricting access to sensitive information on the Contractor (and/or any subcontractor) IT system under this contract. Restricting access may include disconnecting the system processing, storing, or transmitting the sensitive information from the Internet or other networks or applying additional security controls.

7. Privacy Requirements

7.1. Privacy Act.
The Contractor (and/or any subcontractor) agrees to comply with the Privacy Act of 1974 (the Act) and WTCHP rules and regulations issued under the Act in the design, development, or operation of any system of records on individuals to accomplish an agency function when the contract specifically identifies:

7.1.1. The systems of records; and

7.1.2. The design, development, or operation work that the Contractor is to perform.

7.2. In the event of violations of the Act, a civil action may be brought against the CDC when the violation concerns the design, development, or operation of a system of records on individuals to accomplish an CDC function, and criminal penalties may be imposed upon the officers or employees of CDC when the violation concerns the operation of a system of records on individuals to accomplish an CDC function per 5 USC 552a (i), as amended. For purposes of the criminal penalties provisions of the Act, when the contract is for the operation of a system of records on individuals to accomplish an CDC function, the Contractor (and/or any subcontractor) is considered to be an employee of CDC per 5 USC 552a(m)(1), as amended.

7.2.1. "Operation of a system of records,"
As used in this document, means performance of any of the activities associated with maintaining the system of records, including the collection, use, and dissemination of records.

7.2.2. "Record,"
As used in this document, means any item, collection, or grouping of information about an individual that is maintained by CDC including, but not limited to, education, financial transactions, medical history, and criminal or employment history and that contains the person's name, or the identifying number, symbol, or other identifying particular assigned to the individual, such as a fingerprint or voiceprint or a photograph.

7.2.3. "System of records on individuals,"
As used in this document, means a group of any records under the control of CDC from which information is retrieved by the name of the individual or by some identifying number, symbol, or other identifying particular assigned to the individual.

7.2.4. The Contractor (and/or any subcontractor) shall report all suspected and confirmed security and privacy incidents, including breaches involving the loss and/or disclosure of PII in electronic or physical form, to the WTCHP ISSO within **one hour** of discovering the incident. The Contractor shall not include any sensitive information in the subject or body of any e-mail. To transmit sensitive information, the Contractor shall attach a document to email and use a FIPS 140-2 compliant encryption solution to protect the sensitive information in attachments to email. The Contractor shall follow-up with the WTCHP ISSO within **two (2) business days** to verify the status of the incident.

7.2.5. The Contractor (and/or any subcontractor) shall obtain notification instructions and approval from the Contracting Officer prior to notifying individuals whose PII resided in the Contractor (and/or any subcontractor) IT system at the time of the incident. The method and content of any notification by the Contractor shall be compliant with CDC Privacy Officer Standard Operating Procedures (SOP)[2] and WTCHP Guidance as well as coordinated and approved by CDC Privacy Officer in consultation with the Contracting Officer, the WTCHP Privacy Officer and/or WTCHP ISSO. Upon receiving notification and approval from the agency Contracting Officer, the Contractor shall send notifications to affected individuals within five (**5**) **business days**.

(Note: The Contractor (and/or any subcontractor) shall not proceed with any notification to individuals unless the Contracting Officer, in consultation with the CDC PIRT and WTCHP Privacy Officer, has determined in writing that the Contractor notification is appropriate.)

7.2.6. All determinations related to sensitive information incidents, including response activities, notifications to affected individuals and/or federal agencies, and related services (e.g., credit monitoring and identity protection) will be made in writing by the Contracting Officer in consultation with the CDC PIRT, CDC CIO and Privacy Officer and in compliance with CDC and/or WTCHP Guidance.

7.2.6.1. The Contractor (and/or any subcontractor) shall provide full access and cooperation for all activities determined by the Government to be required to ensure an effective incident response, including providing all requested images, log files, and event information to facilitate rapid resolution of sensitive information incidents. Incident response activities determined to be required by the Government may include, but are not limited to, the following:

7.2.6.1.1. Inspections,

7.2.6.1.2. Investigations,

7.2.6.1.3. Forensic reviews, and

7.2.6.1.4. Data analyses and processing.

7.3. PII Security Plan ***(draft to be submitted with offeror's proposal)***

7.3.1. The Contractor shall submit a draft PII Security Plan with its proposal that addresses each of the following items:

7.3.1.1. Verify the information categorization to ensure the identification of the PII requiring protection.

7.3.1.2. Verify the existing risk assessment.

7.3.1.3. Identify the Contractor's existing internal corporate policy that addresses the information protection requirements of the SOW.

7.3.1.4. Verify the adequacy of the Contractor's existing internal corporate policy that addresses the information protection requirements of the SOW.

7.3.1.5. Identify any revisions, or development, of an internal corporate policy to adequately address the information protection requirements of the SOW.

7.3.1.6. For PII to be physically transported to or stored at a remote site, verify that the security and privacy controls of NIST Special Publication 800-53, latest version, involving the encryption of transported information will be implemented.

7.3.1.7. When applicable, verify how the NIST Special Publication 800-53, latest version, security and privacy controls requiring authentication, virtual private network (VPN) connections and other technical safeguards will be implemented.

7.3.1.8. When applicable, verify how the NIST Special Publication 800-53, latest version, security controls enforcing allowed downloading of PII will be implemented.

7.3.1.9. Identify measures to ensure subcontractor compliance with safeguarding PII and security and privacy controls in the NIST 800-53, as amended.

7.3.2. The details contained in the Contractor's PII Security Plan must be commensurate with the size and complexity of the contract requirements based on the System Categorization specified above in the subparagraph entitled Security Categories and Levels. The Contractor's PII Security Plan will be evaluated by the Government for appropriateness and adequacy.

7.3.3. Privacy Threshold Analysis/Privacy Impact Assessment (PTA/PIA) – due within 30 days after contract award. The Contractor shall assist the Agency Privacy Officer with conducting a PTA/PIA for the information system and/or information collection project.

8. Position Sensitivity Designations

8.1. The following position sensitivity designations apply to this contract:

[ ] Level 6: Public Trust - High Risk. Contractor (and/or any subcontractor) employees assigned to Level 6 positions shall undergo a Suitability Determination and Background Investigation (BI).

[X] Level 5: Public Trust - Moderate Risk. Contractor (and/or any subcontractor) employees assigned to Level 5 positions with no previous investigation and approval shall undergo a Suitability Determination and a Minimum Background Investigation (MBI), or a Limited Background Investigation (LBI).

[ ] Level 4: Special Sensitive. Contractor (and/or any subcontractor) employees assigned to Level 4 positions with no previous investigation and approval shall undergo a Suitability Determination and a Single Scope Background Investigation (SSBI),

[ ] Level 3: Top Secret – Critical Sensitive. Contractor (and/or any subcontractor) employees assigned to Level 3 positions with no previous investigation and approval shall undergo a Suitability Determination and Single Scope Background Investigation (SSBI),

[ ] Level 2: Confidential or Secret – Noncritical Sensitive. Contractor (and/or any subcontractor) employees assigned to Level 2 positions with no previous investigation and approval shall undergo a Suitability Determination and a Access National Agency Check, Inquiries (ANACI)

[X] Level 1: Non-Sensitive Contractor (and/or any subcontractor) employees assigned to Level 1 positions shall undergo a Suitability Determination and National Agency Check and Inquiry Investigation (NACI).

All Contractor (and/or any subcontractor) employees shall comply with the conditions established for their designated position sensitivity levels prior to performing any work under this contract.

Suitability Investigations are required for contractors (and/or any subcontractor) who will need access to federal information systems and/or to federally controlled physical space.

9. Access to HHS-Controlled Facilities, Systems, and Information (including HHS non-public data)

9.1. Applicability.
This section applies when contractor personnel (and/or any subcontractor) are expected to have (1) routine physical access to an HHS-controlled facility; (2) logical access to an HHS-controlled information system; (3) access to sensitive HHS data or information, whether in an HHS-controlled information system or in hard copy; or (4) any combination of circumstances (1) through (3).

9.2. As part of the acquisition planning process, the COR shall determine whether, based on the nature of the requirement, Contractor personnel may require access to CDC-controlled facilities and/or information systems, including sensitive data/information, in order to perform the contract/order Statement of Work (SOW)/ Performance Work Statement (PWS). If Contractor access is required, the COR must assess, based on information available at that point in the process, the type, frequency, and duration of such access. Following that determination, the COR shall consult with WTCHP and/or local building and IT security officials/staff, and officials/staff involved with personnel security, including the designated personnel security representative, to determine appropriate security requirements and, as necessary, adjust project requirements to minimize security and access issues. The COR shall comply with HSPD-12 and the following implementing guidance in making these judgments and determinations.

9.3. Definitions.
The following definitions apply to this subpart:

9.3.1. Physical Access: "Physical" entry to and/or exit from a federally controlled facility.

9.3.2. Logical Access: Access to a federal information system that requires the Contractor (and/or any subcontractor) to logon with a CDC Windows Active Directory account or some other CDC authorized network operating system account. This does not include access to a public web site, whether by a CDC Contractor or member of the public, because such web sites do not require permission to access. In the case of sensitive data/information that exists in hard copy, "access" means providing a Contractor (and/or any subcontractor) the right to view or use written/typed data or information for the purpose described in a contract.

9.3.3. Routine Access: On a regular, non-intermittent basis, which is at least once per week during the contract or order period of performance.

9.3.4. Long-term Access: For a period exceeding six months or more than 1,040 hours in any one year.

9.3.5. Sensitive data/information: As defined by the Computer Security Act of 1987, any data/information, "the loss, misuse, or unauthorized access to or modification of which, could adversely affect the national interest or the conduct of federal programs, or the privacy to which individuals are entitled under section 552a of the Title 5 of USC (the Privacy Act), but which has not been specifically authorized under criteria established by an Executive order or an act of Congress to be kept secret in the interest of national defense or foreign policy." Examples include individuals' social security numbers; other personal identification information, such as individuals' health, medical, or psychological information; proprietary research data; and confidential legal data.

9.3.6. Foreign Nationals: An alien, i.e., any person not a citizen of the United States or any person who was born outside the jurisdiction of the United States, is a citizen of a foreign country, and has not become a naturalized U.S. citizen under U.S. law. This includes legal permanent residents, also known as permanent resident aliens (22 U.S.C. § 6023(8), as amended, and 8 U.S.C. § 1101(a)(3)), as amended.

10. Personnel Security Responsibilities

10.1. The Contractor, within five (**5**) **business days** before an employee begins working on this contract, shall provide the Contracting Officer, COR, and Information System Security Officer (ISSO) the name, position title, e-mail address, and phone number of all contract employees working under the contract.

10.1.1. If the employee is filling a new position, the Contractor shall provide a position description and the Government will determine the appropriate suitability level.

10.1.2. If the employee is filling an existing position, the Contractor shall provide the name, position title and suitability determination level held by the former incumbent.

10.2. The Contractor shall notify the Contracting Officer, COR, and system ISSO within five (**5**) **business days** before an employee stops working under this contract.

10.2.1. The Contractor shall provide the name, position title, and suitability determination level held by or pending for departing employees.

10.2.2. The Government will stop pending background investigations for employees that no longer work under this acquisition.

10.3. The Contractor (and/or any subcontractor) shall perform and document the actions identified in the CDC In- and Out -Processing of CDC Employees, Non-employees, and Affiliates http://masoapplications.cdc.gov/Policy/Doc/policy510.pdf when a Contractor (and/or any subcontractor) employee terminates work under this contract. All documentation shall be made available to the COR and/or Contracting Officer upon request.

10.4. Within **7 calendar days** after the Government's final acceptance of the work under this contract, or upon termination of the contract, the Contractor shall return all identification badges to the Contracting Officer or designee.

11. Homeland Security Presidential Directive (HSPD)-12

11.1. To gain routine physical access to an HHS facility, logical access to an HHS-controlled information system, and/or access to sensitive data or information, the Contractor (and/or any subcontractor) and its employees shall comply with Homeland Security Presidential Directive (HSPD)-12, Policy for a Common Identification Standard for Federal Employees and Contractors, as amended; Office of Management and Budget memorandum (M-05-24), as amended; and Federal Information Processing Standards Publication (FIPS PUB) Number 201; and with the personal identity verification and investigation procedures contained in the following documents:

11.1.1. HHS Information Security and Privacy Policy (IS2P) as amended.

11.1.2. HHS Personnel Security and Suitability Policy, as amended.

11.1.3. HSPD-12 Implementation Policy for the Use of the Personal Identity Verification (PIV) Card for Strong Authentication, as amended.

11.2. When foreign national Contractor/subcontractor employees perform work under this acquisition, the provisions of HSPD-12 do NOT apply per OSSI requirements.

12. Fingerprinting

12.1. To gain logical access to HHS-controlled information systems, contract employees working under the contract are subject to a fingerprint check with favorable results.

12.2. Where Contractor (and/or any subcontractor) staff must appear at an HHS facility to be fingerprinted, any costs associated with getting to that facility are to be borne by the Contractor.

13. Background Investigations

13.1. Based upon information provided by the COR, the Contracting Officer shall insert references to CDC and/or local procedural guideline(s), if any; indicate if they are readily accessible to the public; and, if so, specify where they may be found. If they are not readily accessible, the Contracting Officer shall attach a copy to the solicitation and contract and reference the guideline(s) here.

13.2. At the time of solicitation, based upon information provided by the COR, the Contracting Officer shall specify all known levels. If the position sensitivity levels are not known at that time, the Contracting Officer shall insert the words "To Be Determined at the Time of Award." However, the Contracting Officer must include the definitive position sensitivity levels in the awarded contract/order.

13.3. The personnel investigation procedures for Contractor personnel (and/or any subcontractor) require that the Contractor (and/or any subcontractor) prepare and submit background check/investigation forms based on the type of investigation required. The minimum Government investigation for a non-sensitive position is a National Agency Check and Inquiries (NACI) with fingerprinting. More restricted positions – i.e., those above non-sensitive, require more extensive documentation and investigation.

13.4. Investigations are expensive and may delay performance, regardless of the outcome of the investigation. Delays associated with rejections and consequent re-investigations may not be excusable in accordance with the FAR section, Excusable Delays – see FAR 52.249-14.

13.5. Typically, the Government investigates personnel at no cost to the Contractor. However, multiple investigations for the same position may, at the Contracting Officer's discretion, justify reduction(s) in the contract price of no more than the cost of the additional investigation(s).

14. Contract Close Out

14.1. General Security Requirements.
IT security requirements must be incorporated into all phases of program planning and execution, from budgeting to close-out and the implementation of those requirements should align with the CDC Enterprise Performance Life Cycle (EPLC). The Contractor (and/or any subcontractor) shall comply with security requirements, EPLC processes, and Enterprise Architecture, to ensure proper and confidential handling of data and information.

14.2. Certification of Sanitization of Government and Government-Activity-Related Files and Information. As part of contract closeout, the Contractor shall submit the certification to the COR and the Contracting Officer following the template provided in NIST Special Publication 800-88, Guidelines for Media Sanitization.

14.2.1. The Contractor (and/or any subcontractor) shall refer to the HHS-OCIO Policy for Information Systems Security and Privacy (IS2P). The Contractor (and/or any subcontractor) shall comply with the HHS Departmental Information Security Policies. The HHS Cybersecurity Program develops policies,

procedures, and guidance to serve as a foundation for the HHS information security program. These documents implement relevant federal laws, regulations, standards, and guidelines that provide a basis for the information security program at the Department. The Contractor (and/or any subcontractor) must comply with Cybersecurity Program guidelines (http://www.hhs.gov/ocio/policy/). At minimum, the Contractor (and/or any subcontractor) shall comply with all configuration, patching, and maintenance requirements as defined in the IS2P and hold an open source product to the same or greater standard as a non-open source product. As with all software, maintenance cost planning should be included in the terms of the contract, with specific service level agreements for remediation of software vulnerabilities discovered in the software.

14.2.2. Contractors (and/or any subcontractors) must follow and adhere to National Institute of Standards and Technology (NIST) Special Publication (SP) 800-64 Security Considerations in the System Development Life Cycle (latest revision) , at a minimum, for system development. This requirement may include providing system documentation at designated intervals within the EPLC that require artifact review.

15. Audit Record Retention

15.1. The vendor shall support a system in accordance with the requirement for federal agencies to manage their electronic records in accordance with 36 CFR § 1236.20 & 1236.22 (ref. a), including but not limited to capabilities such as those identified in:

15.1.1. NARA Bulletin 20013-02, August 29, 2013, Guidance on a New Approach to Managing Email Records,

15.1.2. NARA Bulletin 2010-05 September 08, 2010, Guidance on Managing Records in Cloud Computing Environments (ref 8).

15.2. These provide requirements for maintaining records to retain functionality and integrity throughout the records' full lifecycle including:

15.2.1. Maintenance of links between records and metadata, and

15.2.2. Categorization of records to manage retention and disposal, either through transfer of permanent records to NARA or deletion of temporary records in accordance with NARA-approved retention schedules.

16. Subcontractor Section

16.1. The Contractor shall ensure that its subcontractors (at all tiers) which perform work under this contract comply with the requirements contained in this contract.

16.2. The Contractor shall incorporate the substance of the sections in this contract in all subcontracts that require protection of federal information and information systems, including those subcontracts that:

16.2.1. Have physical or electronic access to CDC computer systems, networks, or IT infrastructure; or

16.2.2. Use information systems to generate, store, process, or exchange data with CDC or on behalf of CDC, regardless of whether the data resides on a CDC or the Contractor's information system.

16.3. The Contractor is prohibited from subcontracting to companies in which foreign governments that support international terrorism have a "significant interest" per 10 U.S.C. § 2327 (as amended).

17 Security and Privacy Requirements for Federal Information Technology (IT) Resources

17.1. Applicability.

The COR will include the following sections on all contracts for Contractor-owned/Contractor-operated information systems that process, store, or transmit federal information. Confer with the System Owner, ISSO, and/or CDC Office of the Chief Information Security Officer to ensure that all applicable sections are included.

17.2. Contractor Security Deliverables.

In accordance with the timeframes specified, the Contractor shall prepare and submit the following security requirements to the Contracting Officer, COR and System Owner or designated representative for review, comment, and acceptance, when applicable and identified in the deliverables section of the contract. All deliverable shall comply with applicable federal laws that include, but are not limited to, the Federal Information Security Modernization Act (FISMA) of 2014 as amended, (Title III of the E-Government Act of 2002, Public Law 107-347), Office of Management and Budget (OMB) Circular A-130, Management of Federal Information Resources, Appendix III, Security of Federal Automated Information Resources, and other applicable federal laws, regulations, NIST guidance and Departmental policies.

17.3. FIPS 199 Standards for Security Categorization of Federal Information and Information Systems Assessment (FIPS 199 Assessment as amended)

Due within 45 days after contract award. The FIPS 199 Assessment shall comply with FIPS 199 and NIST SP 800-60, latest revision, Guide for Mapping Types of Information and Information Systems to Security Categories. After resolution of any comments by the Agency, the Contractor will finalize the FIPS 199 Assessment and submit the final versions to the Contracting Officer for acceptance and inclusion into the contract file.

17.4. Security Assessment and Authorization (SA&A)

Due within 45 days after contract award. The Contractor shall submit written proof to the Contracting Officer that a SA&A was performed for the applicable information system and the system has a valid authority to operate (ATO). If a SA&A has not been performed, the Contractor (and/or any subcontractor) shall perform the SA&A in accordance with the Office of the Chief Information Officer (OCISO), CDC-OCIO Policy for Information Systems Security and Privacy Policy (IS2P); NIST SP 800-37, latest revision, Guide for Applying the Risk Management Framework to Federal Information Systems: A Security Life Cycle Approach, and other applicable guidance. The Agency System Owner or designated representative shall review the draft SA&A deliverables and provide comments to the Contractor for resolution. After resolution of any comments, the Contractor shall submit a final SA&A package to the System Owner for acceptance and approval by the Agency Authorizing Official (AO), or designee, at least **thirty (30) days** prior to the date of operation of the information system. Only after obtaining a valid ATO for the information system, will the Contractor implement and deploy the system into production. Once the ATO package has been approved by the Agency AO, or designee, the Contracting Officer shall incorporate the ATO letter into the contract as a compliance document. The Agency's acceptance of the ATO does not alleviate the Contractor's responsibility to ensure the IT system controls are implemented and operating effectively.

17.5. SA&A Package Deliverables

The Contractor shall include at a minimum the following SA&A deliverables:

17.5.1. System Security Plan (SSP)

Due within 45 days after contract award. The SSP shall be compliant with the NIST SP 800-18 , Guide for Developing Security Plans for Federal Information Systems, the Federal Information

Processing Standard (FIPS) 200, Recommended Security Controls for Federal Information Systems, and NIST SP 800-53 (latest revision), Security and Privacy Controls for Federal Information Systems and Organizations and other applicable NIST guidance. The SSP shall be consistent with and further detail the approach to IT security contained in the Contractor's bid or proposal that resulted in the award of this contract. The SSP shall provide an overview of the system environment and security requirements to protect the information system as well as describe the security controls in place or planned for meeting those requirements. It should provide a structured process for planning adequate, cost-effective security protection for a system. The Contractor shall update the SSP **annually** thereafter.

17.5.2. Security Assessment Report (SAR)
Due within 45 days after contract award. The SAR shall be conducted by an independent assessor and be consistent with NIST SP 800-53A, NIST SP 800-30, latest revision, and any additions or augmentations described in the CDC OCIO Information Systems Security and Privacy Policy. After resolution of any comments provided by the Government on the draft SAR, the Contracting Officer shall accept the SAR and incorporate the Contractor's final version into the contract for Contractor implementation and maintenance. The Contractor shall update the SAR **annually** thereafter.

17.5.3. Plan of Actions and Milestones (POA&M)
Due within 45 days after contract award. The Contractor is responsible for mitigating all security risks found during assessment and authorization and continuous monitoring activities. For POA&M items only, all high-risk vulnerabilities must be mitigated within 30 days and all moderate risk vulnerabilities must be mitigated within 90 days from the date vulnerabilities are formally identified. These timeframes are the outside limits. All mitigation should take place as soon as applicable. CDC will determine the risk rating of vulnerabilities.

17.5.3.1. Identified gaps between required security control baselines, continuous monitoring controls, and the Contractor's implementation, as documented in the SAR, shall be tracked by the Contractor for mitigation in the POA&M document. Depending on the severity of the gaps, CDC may require them to be remediated before an Authorization is issued, as applicable.

17.5.4. Contingency Plan
Due within 45 days after contract award. The Contractor shall develop a Contingency Plan in accordance with NIST SP 800-34, latest revision, and CDC policy. Upon acceptance by the System Owner, the Contractor shall test the Contingency Plan and prepare a Contingency Plan Test Report that includes the test results. The Contractor shall update and test the Contingency Plan **annually** thereafter.

17.5.5. Security Review
The Contractor shall also perform an independent **annual** security control assessment and provide to the Contracting Officer verification that the system ATO remains valid. Evidence of a valid system security authorization includes written results of (A) annual testing of the system contingency plan and (B) the performance of a security control assessment. Upon the ATO award, Contractor-operated systems that input, store, process, output, and/or transmit shall meet or exceed the continuous monitoring requirements identified below.

17.6. Government Access for IT Inspection.
The Contractor (and/or any subcontractor) shall afford the Government access to the Contractor's and subcontractors' facilities, installations, operations, documentation, information systems, and personnel used in performance of this contract to the extent required to carry out a program of IT inspection (to include vulnerability testing), investigation, and audit to safeguard against threats and hazards to the integrity, confidentiality, and availability, of federal data or to the protection of information systems operated on behalf of CDC.

18. Information Security Continuous Monitoring.
All Contractor/subcontractor-owned/operated systems that input, store, process, output, and/or transmit sensitive information shall meet or exceed the information security continuous monitoring requirements identified in FISMA (as amended). The Contractor (and/or any subcontractor) shall also store monthly information security continuous monitoring data at its location for a period not less than **one year** from the date the data is created. The monthly continuous monitoring data shall be encrypted in accordance with FIPS 140-2 Security Requirements for Cryptographic Modules and shall not be stored on systems that are shared with other commercial or Government entities. The Government may elect to perform information security continuous monitoring and IT security scanning of Contractor (and/or any subcontractor) systems from Government tools and infrastructure.

18.1 Asset Management.
The contractor (and/or any subcontractor) shall use any available Security Content Automation Protocol (SCAP)-compliant automated tools for active/passive scans to provide an inventory of all information technology (IT) assets for both hardware and software, (computers, servers, routers, databases, operating systems, etc.) that are processing government owned information/data. It is anticipated that this inventory information will be required to be produced at least monthly. The contractor (and/or any subcontractor) shall be capable of providing detailed IT asset inventory information, to include IP address, machine name, operating system level, security patch level, and SCAP-compliant format information. The contractor (and/or any subcontractor) shall work towards ultimately maintaining a capability to provide an inventory of 100% of its IT assets using SCAP-compliant automated tools.

18.2. Configuration Management.
The contractor (and/or any subcontractor) shall use available SCAP-compliant automated tools for authenticated scans to provide visibility into the security configuration compliance status of all IT assets, (computers, servers, routers, databases, operating systems, application, etc.) that are processing government owned information/data. Compliance will be measured using IT asset and system security configuration guidance provided by the government, for all IT assets. The SCAP-compliant automated tools will compare the installed configuration to the government specific security configuration guidance; contractor may reflect the most restrictive security configuration mode consistent with operational requirements. It is anticipated that this IT asset security configuration information will be required to be produced at least monthly. The contractor shall work towards ultimately maintaining a capability to provide security configuration compliance information for 100% of IT assets using SCAP-compliant automated tools.

18.3. Vulnerability Management.
The contractor (and/or any subcontractor) shall use SCAP-compliant automated tools for authenticated scans to detect any security vulnerabilities in all information technology (IT) assets, (computers, servers, routers, Web applications, databases, operating systems, etc.) that are processing government owned information/data. It is anticipated that this IT asset security vulnerability information will be required to be produced at least monthly. Contractors (and/or any subcontractors) shall actively manage system vulnerabilities using automated tools and technologies where practicable. Automated tools shall be complaint with NIST specified SCAP standards for vulnerability identification and management. Additionally, contractors without in-place automated SCAP-compliant vulnerability management tools and technologies, are required to provide the number of systems (expressed in a percentage) that vulnerability information can be obtained for along with plans for procuring and implementing automated tools that comply with CDC requirements. The contractor (and/or any subcontractor) shall work towards ultimately maintaining a capability to provide security vulnerability scanning information for 100% of IT assets using SCAP-compliant automated tools.

18.4. Data Protection.
Current Federal government security guidance requires that sensitive government information that is stored on laptops and other portable computing devices shall be encrypted using Federal Information Processing Standard (FIPS)-140-2 validated encryption. The contractor shall provide the percentage of portable IT assets that are equipped with FIPS 140-2 validated encryption, to encrypt all sensitive government information, via a report on a **quarterly** basis.

18.5. Remote Access.
Current Federal government security guidance requires that two-factor authentication be implemented when remotely accessing sensitive government owned information/data on IT systems (both government owned and contractor owned systems). Additional Federal government security guidance when remotely accessing government owned information/data include the following: connections shall utilize FIPS-140-2 validated encryption; connections shall be capable of assessing and correcting system configurations upon connection; connections shall be capable of scanning for viruses and malware upon connection; connections shall prohibit split tunneling; and connections shall require timeout after **15 minutes** of inactivity. Each quarter, the contractor shall provide the following information about the contractor's remote access solutions to government owned sensitive information/data: percentage of current connections that allow connection using only a password; percentage of connections that require the use of a government provided personal identity verification (PIV) card as part of a two-factor solution; percentage of connections that require the use of other two- factor authentication solutions; percentage of connections that utilize FIPS-140-2 encryption; percentage of connections that assess and correct system configurations upon connection; percentage of connections that scan for viruses and malware upon connection; percentage of connections that prohibit split tunneling; and percentage of connections that require timeout after **15 minutes** of inactivity.

18.6. Incident Management.
The contractor (and/or any subcontractor) shall install critical security patches or take other security remediation action as directed to Federal agencies by the Department of Homeland Security (DHS) to resolve weaknesses in systems processing government owned information/data. The contractor shall report status and when the directed action has been completed. It is anticipated that this type of urgent security remediation action may be necessary at least **1-2 times per month**.

19. Standard for Security Configurations.
The Contractor (and/or any subcontractor) shall apply approved security configurations to information technology (IT) that is used to process information on behalf of CDC. The following security configuration requirements apply:

19.1 Desktop and/or Laptop Computers Required for Use by the Contractor.

19.1.1. The Contractor (and/or any subcontractor) shall ensure that IT equipment that is used to process information on behalf of CDC are deployed and operated in accordance with approved security configurations, as follows:

19.1.2. The Contractor (and/or any subcontractor) shall configure its computers that contain CDC data in accordance with the latest applicable United States Government Configuration Baseline (USGCB) (see http://usgcb.nist.gov/) and/or other approved CDC IT security configurations.

19.1.3. The Contractor (and/or any subcontractor) shall ensure that its computers have and maintain the latest operating system patch level and anti-virus software level.

19.1.4. The Contractor (and/or any subcontractor) shall ensure hardware and software installation, operation, maintenance, update, and patching will not alter the configuration settings or requirements specified above.

19.1.5. The Contractor (and/or any subcontractor) shall validate the configuration settings after hardware and software installation, operation, maintenance, update, and patching.

19.1.6. The Contractor (and/or any subcontractor) shall use automated means to ensure that its computers remain in compliance with the above configuration requirements by:

19.1.6.1. Configuring its systems to allow for periodic CDC vulnerability and security

configuration assessment scanning; or where such outside scanning is not practicable.

19.1.6.2. Using Security Content Automation Protocol (SCAP)-validated tools with USGCB Scanner capabilities to scan its systems on at least a monthly basis and report the results of these scans to the COR, Project Officer, and any other applicable WTCHP designated POC.

19.2. Other computing Devices Required for Use by the Contractor.
These computing devices include ancillary and peripheral IT devices as well as other computing servers, but not desktop and/or laptop computers).

19.2.1. The Contractor (and/or any subcontractor) shall ensure that information technology (IT) that is used to process information on behalf of CDC are deployed and operated in accordance with approved security configurations, as follows:

19.2.2. The Contractor (and/or any subcontractor) shall configure all ancillary and peripheral IT devices and computers that are not desktops or laptops (e.g. servers, routers, mobile devices, etc.) that store and/or transport CDC data, in accordance with the applicable CDC and/or WTCHP requirements and ensure that they have and maintain the latest operating system patch level and anti-virus software level, when applicable.

19.2.2.1. The Contractor (and/or any subcontractor) shall ensure that all ancillary and peripheral IT devices and other computing devices, as applicable, have and maintain the latest operating system patch level and anti-virus software level.

19.2.2.2. The Contractor (and/or any subcontractor) shall ensure hardware and software installation, operation, maintenance, update, and patching will not alter the configuration settings or requirements specified above.

19.2.2.3. The Contractor (and/or any subcontractor) shall validate the configuration settings after hardware and software installation, operation, maintenance, update, and patching.

19.2.2.4. The Contractor (and/or any subcontractor) shall use automated means to ensure that its computers remain in compliance with the above configuration requirements by:

19.2.2.4.1. Configuring systems to allow for periodic CDC vulnerability and security configuration assessment scanning; or where such outside scanning is not practicable.

19.2.2.4.2. Using Security Content Automation Protocol (SCAP)-validated tools to scan its systems on at least a **monthly** basis and report the results of these scans to the COR.

20. Procurement of IT Computer/Server Equipment
The Contractor (and/or any subcontractor) shall include Federal Information Processing Standard (FIPS) 201-compliant smart card readers (referred to as LACS Transparent Readers) with the purchase of servers, desktops, and laptops; in compliance with FAR Subpart 4.13, Personal Identity Verification.

21. Information Technology Application Design or Support
The Contractor (and/or any subcontractor) shall ensure IT applications designed for end users run in the standard user context without requiring elevated administrative privileges.

22. Security Requirements for Contracts with Cloud Service Providers
The CDC will determine the security category for the cloud system in accordance with Federal Information Processing Standard 199; then, the Contractor (and/or any subcontractor) shall apply the appropriate security control baseline as recommended in the FedRAMP Cloud Computing Security Requirements Baseline document to ensure compliance to security standards. The FedRAMP baseline controls are based on NIST Special Publication 800-53, *Recommended Security Controls for Federal Information Systems and*

Organizations (as amended), and includes a set of additional controls for use within systems providing cloud services to the federal government.

The FedRAMP security controls baseline, Standard Contract Clauses, and Control Specific Contract Clauses, and other relevant documentation can be found at http://www.fedramp.gov

The Contractor (and/or any subcontractor) shall maintain a security management continuous monitoring environment based upon the latest edition of FedRAMP Cloud Computing Security Requirements Baseline and CDC Continuous Monitoring Requirements.

The following sections are based on the FedRAMP guidance listed above and have been modified for CDC contracts with Cloud Service Providers.

23. CDC FedRAMP Privacy and Security Requirements

23.1. Privacy and Security Safeguards.
Coutractor (and/or any subcontractor) shall be respousible for the following privacy and security safeguards:

To the extent required to carry out the FedRAMP and CDC security assessment and authorization (SA&A) process and CDC continuous monitoring, to safeguard against threats and hazards to the security, integrity, and confidentiality of any non-public Government data collected and stored by the Contractor (and/or any subcontractor), the Contractor (and/or any subcontractor) shall afford CDC access to the Contractor's (and/or any subcontractor's) facilities, installations, operations, documentation, databases, IT systems and devices, and personnel used in performance of the contract, regardless of the location. Access shall be provided to the extent required, in CDC's judgment, to conduct an inspection, evaluation, investigation or audit, including vulnerability testing to safeguard against threats and hazards to the integrity, availability and confidentiality of CDC data or to the function of information technology systems operated on behalf of CDC and to preserve evidence of computer crime. This information shall be available to CDC upon request.

If new or unanticipated threats or hazards are discovered by either CDC or the Contractor, or if existing safeguards have ceased to function, then the discoverer shall immediately bring the situation to the attention of the other party.

The Contractor (and/or any subcontractor) shall comply with any additional, current or future, FedRAMP and CDC privacy requirements. The Contractor (and/or any subcontractor) shall also comply with any additional, current or future, Health Insurance Portability and Accountability Act (HIPAA), Privacy Act of 1974, or Health Information Technology for Economic and Clinical Health (HITECH) Act privacy, security, and breach notification requirements as appropriate for the data being handled (as specified by CDC).

CDC has the right to perform manual or automated audits, vulnerability scans, reviews, or other inspections of the Contractor's (and/or any subcontractor's) IT environment being used to provide or facilitate services for the Government. In accordance with the FAR section 52.239-1, Contractor (and/or any subcontractor) shall be responsible for the following privacy and security safeguards:

The Contractor (and/or any subcontractor) shall not publish or disclose in any manner, without the Contracting Officer's written consent, the details of any safeguards either designed or developed by the Contractor (and/or any subcontractor) under this contract or otherwise provided by CDC.

To the extent required to carry out a program of inspection to safeguard against threats and hazards to the security, confidentiality, integrity, and availability of federal data, the Contractor shall afford CDC access to the Contractor's (and/or any subcontractor's) facilities, installations, technical capabilities, operations, documentation, records, and databases within ***72 hours*** of notification. The program of inspection shall include, but is not limited to:

- Authenticated and unauthenticated operating system/network vulnerability scans.
- Authenticated and unauthenticated web application vulnerability scans.
- Authenticated and unauthenticated database application vulnerability scans.

Automated scans can be performed by CDC personnel, or agents acting on behalf of the CDC, using Government operated equipment, and Government specified tools.

If the Contractor (and/or any subcontractor) chooses to run its own automated scans or audits, then the results from these scans may, at CDC's discretion, be accepted in lieu of CDC performed vulnerability scans. In these cases, scanning tools and their configuration shall be approved by CDC. In addition, the results of Contractor-conducted (and/or any subcontractor) scans shall be provided, in full, using NIST Security Content Automation Protocol (SCAP) standards, to CDC, upon request, but no less than every ***30 days***.

24. Cloud Sensitive Information Storage

Controlled Unclassified Information (CUI), sensitive/proprietary data, and/or equipment will only be disclosed to authorized personnel on a Need-To-Know basis. The Contractor (and/or any subcontractor) shall ensure that appropriate administrative, technical, and physical safeguards are established to ensure the security and confidentiality of this information, data, and/or equipment is properly protected. Contractors (and/or any subcontractors) processing CUI incidental to developing a product or service for CDC must meet the requirements of NIST SP 800-171. When no longer required, this information, data, and/or equipment will be returned to CDC control, destroyed, or held until otherwise directed. Destruction of items shall be accomplished by following NIST SP 800-88, Guidelines for Media Sanitization, as amended.

The disposition of all CDC data shall be at the written direction of CDC/WTCHP. This may include documents returned to CDC control; destroyed; or held as specified until otherwise directed.

25. Cloud Protection of Information

The Contractor (and/or any subcontractor) shall be responsible for properly protecting and safeguarding all information used, gathered, or developed as a result of work under this contract. The Contractor (and/or any subcontractor) shall also protect all CDC data, equipment, etc. by treating the information as sensitive. All information about the systems gathered or created under this contract shall be considered as Controlled Unclassified Information (CUI). It is anticipated that this information will be gathered, created, and stored within the primary work location. If Contractor (and/or any subcontractor) must remove any information from the primary work area, then they shall protect it to the same extent they would their proprietary data and/or company trade secrets. The use of any information that is subject to the *Privacy Act of 1974*, HIPAA, and/or HITECH will be utilized in full accordance with all rules of conduct as applicable to *Privacy Act of 1974*, HIPAA, and/or HITECH information, respectfully.

CDC will retain unrestricted rights to government data. The CDC retains ownership of any user created/loaded data and applications hosted on vendor's infrastructure, as well as maintains the right to request full copies of these at any time.

The data that is processed and stored by the various applications within the network infrastructure contains financial data as well as PII. This data and PII shall be protected against unauthorized access, disclosure or modification, theft, or destruction. The Contractor (and/or any snbcontractor) shall ensure that the facilities that house the network infrastructure are physically secure.

The data must be available to CDC upon request within one (1) business day or within the timeframe specified otherwise, and shall not be used for any other purpose other than that specified herein. The Contractor (and/or any subcontractor) shall provide requested data at no additional cost to the CDC.

No data shall be released by the Contractor (and/or any subcontractor) without the consent of CDC in writing. All requests for release must be submitted in writing to the COR/CO.

26. Cloud Security Classification

The preparation of the deliverables in this contract will be completed at a Controlled Unclassified Information (CUI) level and shall be marked accordingly.

27. Cloud Confidentiality and Nondisclosure

The preliminary and final deliverables and all associated working papers and other material deemed relevant by the agency that have been generated by the Contractor (and/or any subcontractor) in the performance of this contract, are the property of CDC and must be submitted to the contracting Officer at the conclusion of the contract. The U.S. Government has unlimited data rights to all deliverables and associated working papers and materials in accordance with FAR 52.227-17 Data Rights – Special Works.

All documents produced for this contract are the property of CDC and cannot be reproduced, or retained by the Contractor (and/or any subcontractor). All appropriate project documentation will be given to the agency during and at the end of this contract. The Contractor shall not release any information without the written consent of the Contracting Officer.

Personnel working on any of the described tasks may, at CDC request, be required to sign formal non-disclosure and/or conflict of interest agreements to guarantee the protection and integrity of CDC information and documents.

Any information made available to the Contractor (and/or any subcontractor) by CDC shall be used only for the purpose of carrying out the provisions of this contract and shall not be divulged or made known in any manner to any persons except as may be necessary in the performance of the contract. In performance of this contract, the Contractor assumes responsibility for protection of the confidentiality of Government records and shall ensure that all work performed by its subcontractors shall be under the supervision of the Contractor or the Contractor's responsible employees. Each officer or employee of the Contractor or any of its subcontractors to whom any CDC record may be made available or disclosed shall be notified in writing by the Contractor that information disclosed to such officer or employee can be used only for that purpose and to the extent authorized herein. Further disclosure of any such information, by any means, for a purpose or to an extent unauthorized herein, may subject the offender to criminal sanctions imposed by 18 U.S.C. §§ 1030.

28. Security Assessment and Authorization

The minimum requirements for Low and Moderate impact cloud systems are contained within the FedRAMP Cloud Computing Security Requirements Baseline as per FedRAMP Security Plan templates and NIST SP 800-53 latest revision (available at https://www.fedramp.gov/resources/templates-3/ and http://csrc.nist.gov/publications/PubsSPs.html). The Contractor and CDC share responsibility to ensure compliance with security requirements.

28.1. Cloud Assessment and Authorization

All new CDC cloud IT acquisitions are required to go through a formal Security Assessment and Authorization (SA&A) process and obtain authorization to operate (ATO) prior to implementation. In addition, FedRAMP requires cloud service providers to utilize a FedRAMP accredited 3PAO, the JAB or an independent entity, to perform an assessment of the cloud service provider's security controls to determine the extent to which security controls are implemented correctly, operate as intended, and produce the desired outcome with respect to meeting security requirements. A listing of accredited 3PAOs is available on the FedRAMP Website. *CDC and WTCHP reserve the right to verify the infrastructure and security test results before issuing an Authorization decision.*

CDC and WTCHP will leverage the Provisional Authorization granted by FedRAMP and any documentation prepared by the Contractor (and/or any subcontractor) to issue a full ATO. A full ATO of a cloud based system is required before any Production, Development or Testing operations may commence, or CDC sensitive information may be placed in the cloud-based environment. When issuing a full ATO, WTCHPs may leverage a FedRAMP Joint Authorization Board (JAB) Provisional ATO, or an Agency FedRAMP ATO, as the baseline for the security controls managed and implemented by the cloud service provider. When leveraging a FedRAMP authorization, CDC are responsible for insuring that all security controls, including CDC specific security controls, have been adequately addressed. Where a cloud provider does not have an existing JAB or Agency ATO, the cloud service provider must work with the FedRAMP Program Management Office (PMO) or the CDC Cloud PMO security staff, as applicable, to obtain an ATO of their cloud system as recommended in the CDC Cloud Computing and Federal Risk and Authorization Management Program Guidance, dated May 2015.

The Contractor (and/or any subcontractor) is advised to review the FedRAMP and CDC guidance documents to determine the level of effort that will be necessary to complete the requirements. All FedRAMP documents and templates are available on the FedRAMP Website.

Revocation of a Cloud Service. In the event of a sensitive information incident, the Government may suspend or cancel the (Contract/Award) and terminate any outstanding orders if the existing ATO (either in part or in whole) is revoked. If an ATO is suspended or revoked in accordance with this provision and the contract is still in effect, the Contracting Officer may direct the Contractor (and/or any subcontractor) to take additional security measures to secure sensitive information. These measures may include restricting access to sensitive information on the Contractor (and/or any subcontractor) IT system under this contract. Restricting access may include disconnecting the system processing, storing, or transmitting the sensitive information from the Internet or other networks or applying additional security controls.

28.2. System Assessment and Authorization Process

The Contractor (and/or any subcontractor) shall comply with FedRAMP and CDC requirements as mandated by federal laws and policies, including making available any documentation, physical access, and logical access needed to support this requirement. The Contractor (and/or any subcontractor) shall create, maintain and update the following documentation using FedRAMP templates.

- Privacy Impact Assessment (PIA)
- FedRAMP Test Procedures and Results
- Security Assessment Report (SAR)
- System Security Plan (SSP)
- System Contingency Plan (CP)
- System Contingency Plan (CP) Test Results
- Plan of Action and Milestones (POA&M)
- Continuous Monitoring Plan (CMP)
- FedRAMP Control Tailoring Workbook
- Control Implementation Summary Table
- Results of Penetration Testing
- Software Code Review

Information systems must be assessed by a FedRAMP Accredited 3PAO JAB or other independent entity, whenever there is a significant change to the system's security posture in accordance with the CDC Continuous Monitoring Plan. A listing of FedRAMP accredited 3PAOs is available for review.

CDC reserves the right to perform Penetration Testing. If CDC exercises this right, the Contractor (and/or any subcontractor) shall allow CDC employees (and/or designated third parties) to conduct Security Assessment activities to include control reviews in accordance with CDC requirements. Review activities include, but are not limited to, scanning operating systems, web applications, wireless scanning; network device scanning to include routers, switches, and firewall, and IDS/IPS; databases and other applicable

systems, including general support structure, that support the processing, transportation, storage, or security of Government information for vulnerabilities.

The Contractor (and/or any subcontractor) shall identify gaps between required FedRAMP Security Control Baselines and Continuous Monitoring controls and the Contractor's (and/or any subcontractor's) implementation as documented in the System Security Assessment Report (SAR) track deficiencies in a Plan of Action and Milestones (POA&M) document until mitigated. Depending on the severity of the gaps, CDC may require them to be remediated before a FedRAMP or CDC Authorization is issued, as applicable.

The Contractor (and/or any subcontractor) is responsible for mitigating all security risks found during Assessment and Authorization and continuous monitoring activities. All high-risk vulnerabilities must be mitigated within ***30 days*** and all moderate risk vulnerabilities must be mitigated within ***90 days*** from the date vulnerabilities are formally identified. CDC will determine the risk rating of vulnerabilities.

28.3. Cloud Reporting and Continuous Monitoring

Maintenance of the FedRAMP Provisional Authorization and the CDC Authorization, as applicable, will be through continuous monitoring and periodic audit of the operational controls within a Contractor's (and/or any subcontractor's) system, environment, and processes to determine if the security controls in the information system continue to be effective over time in light of changes that occur in the system and environment. Through continuous monitoring, security controls and supporting deliverables are updated and submitted to the FedRAMP PMO or CDC PMO, as required by FedRAMP or CDC Requirements. The submitted deliverables (or lack thereof) provide a current understanding of the security state and risk posture of the information systems. The deliverables will allow the FedRAMP JAB Or CDC Cloud PMO to make credible risk-based decisions regarding the continued operations of the information systems and initiate appropriate responses as needed when changes occur. Contractors (and/or any subcontractors) shall provide updated deliverables and automated data feeds as defined in Section 3.1.5 of this document.

29. Cloud Data Jurisdiction

The vendor shall identify all data centers that the data at rest or data backup will reside. All data centers will be guaranteed to reside within the continental United States (CONUS).

Vendors shall provide information concerning, but not limited to,

- the number of data center facilities at different geographic locations
- the number of Internet Exchange Point (IXP) for each price offering
- The Internet bandwidth

30. Interconnection Agreements/Service Level Agreements/Memorandum of Agreements

The Contractor shall understand the terms of the service agreements that define the legal relationships between cloud customers and cloud providers. CDC should understand customer responsibilities, and those of the service provider, before using a cloud service.

31. Cloud FIPS-140 Validated Cryptography for Secure Communications

All deliverables shall be Controlled Unclassified Information (CUI). External transmission/dissemination of CUI to or from a Government computer must be encrypted. Certified encryption modules must be used in accordance with FIPS PUB 140-2 *"Security Requirements for Cryptographic Modules"* (as amended) or the updated cryptographic standard, as applicable.

32. Cloud Audit Record Retention

The vendor shall support a system in accordance with the requirement for federal agencies to manage their electronic records in accordance with 36 CFR § 1236.20 & 1236.22 (ref. a), including but not limited to

capabilities such as those identified in:

- NARA Bulletin 20013-02, August 29, 2013, *Guidance on a New Approach to Managing Email Records*,
  NARA Bulletin 2010-05 September 08, 2010, *Guidance on Managing Records in Cloud Computing Environments* (ref 8).

These provide requirements for maintaining records to retain functionality and integrity throughout the records' full lifecycle including:

- Maintenance of links between records and metadata, and
- Categorization of records to manage retention and disposal, either through transfer of permanent records to NARA or deletion of temporary records in accordance with NARA-approved retention schedules.

33. Cloud Incident Reporting Timeframes

Cloud Service Providers are required to report all computer security incidents to the United States Computer Emergency Readiness Team (US-CERT) in accordance with US-CERT *"Incident Categories and Reporting Timeframes"* in , Appendix J, Table J-1 of NIST SP 800-61, as amended, *"Computer Security Incident Handling Guide"*. Any Category (CAT) 1, CAT 2, or CAT 3 incident, must be reported immediately to the WTCHP Information Security Officer (ISSO) and the Senior Agency Information Security Officer (SAISO). Any incident that involves PII must be reported to US-CERT within ***1 hour*** of detection regardless of the incident category reporting timeframe. In addition, the incidents must be reported to the WTCHP ISSO. WTCHP users should report all suspected computer security incidents to their local WTCHP ISSO, as they are the most familiar with the local computer systems, network, and reporting requirements.

The Contractor (and/or any subcontractor) shall provide full access and cooperation for all activities determined by the Government to be required to ensure an effective incident response, including providing all requested images, log files, and event information to facilitate rapid resolution of sensitive information incidents. The types of information required in a cyber incident report must include at a minimum: company and point of contact information, contract information, and the type of information compromised.

Incident response activities determined to be required by the Government may include, but are not limited to, the following:

Inspections,
Investigations,
Forensic reviews, and
Data analyses and processing.
For more information, please refer to the CDC - *IT Security and Privacy Incident Reporting and Response Policy*, as amended.

34. Cloud Media Transport

The vendor shall document activities associated with the transport of federal agency information stored on digital and non-digital media and employ cryptographic mechanisms to protect the confidentiality and integrity of this information during transport outside of controlled areas.

Digital media, containing federal agency information, that is transported outside of controlled areas must be encrypted using a *[encryption mode]*; non-digital media including but not limited to CD-ROM, floppy disks, etc., must be secured using the same policies and procedures as paper.

Media, containing federal Agency information that is transported outside of controlled areas must ensure accountability. This can be accomplished through appropriate actions such as logging and a documented chain of custody form.

CDC data that resides on mobile/portable devices (e.g., USB flash drives, external hard drives, and SD cards) must be encrypted using FIPS 140-2 guidelines. All CDC data residing on laptop computing devices must be protected with approved encryption software.

35. Cloud Boundary Protection: Trusted Internet Connections (TIC)

The vendor shall ensure that federal information, other than unrestricted information, being transmitted from federal government entities to external entities using cloud services is inspected by Trusted Internet Connections (TIC) processes. Or the vendor shall route all external connections through a Trusted Internet Connection (TIC).

36. Cloud Security Alerts, Advisories, and Directives

The vendor shall provide a list of their personnel, identified by name and role, with system administration, monitoring, and/or security responsibilities that are to receive security alerts, advisories, and directives. This list shall WTCHP ISSO and the NIOSH ISSO.

Appendices to Attachment #2, Information Technology Security and Privacy

Appendix A - Prospective Offeror Non-Disclosure Agreement

Appendix B - Contractor Non-Disclosure Agreement

Appendix C - WTCHP Incident Reporting Template

Appendix D – Acronyms

Prospective Offeror Non-Disclosure Agreement

NOTE: To be completed by offeror if access to sensitive information is required in order to prepare an offer.

Information Technology Systems Security
Prospective Offeror Non-Disclosure Agreement

Request For Proposal (RFP) No: _____

Project Title: _____
(Fill in Title from RFP)

_____*(Organization's name)*, intends to respond to the Government's Solicitation/Project title indicated above. The Government has determined that the solicitation requires prospective offerors to have access to sensitive information in order to prepare an offer.

I,_____*(Offeror Official name and title),*

of_____*(Organization's name)*, on this day of _____, 2016

_____, on behalf of my organization hereby request access to the sensitive information described in Section _____of the RFP sited above.

I, the undersigned, understand that the Government has determined that any individual having access to the sensitive information described in the RFP must possess a valid and current Suitability Determination at the Level identified in the RFP sited above.

I, the undersigned, do hereby affirm the following:

- I have a valid and current Suitability Determination sufficient to access the sensitive information (copy of suitability determination attached).
- I will be the corporate official solely responsible for appropriately safeguarding the sensitive information while in the possession of _____ *(Organization's name)*;
- The sensitive information will be used solely for the purpose of preparing an offer;
- I will not release, publish, or disclose the sensitive information to unauthorized personnel; and
- I will protect the sensitive information iu accordance with provisious of the following laws and any other pertinent laws and regulations governing the confidentiality of sensitive information:
    - 18 U.S.C. 641 (Criminal Code: Public Money, Property of Records)

- 18 U.S.C. 1905 (Criminal Code: Disclosure of Confidential Information)
- Public Law 96-511 (Paperwork Reduction Act)
- 5 U.S.C. 552a (Privacy Act)

Signature of Prospective Offeror Official:
_____

Name of Prospective Offeror Official:
_____

Name of Prospective Offeror:
_____

Date:_____

Signature of
Witness:_____

Name of Witness:
_____

Date:_____

Copies Retained by:

Contractor Official

Contracting Officer

Contractor Non-Disclosure Agreement

NOTE: To be completed by contractor upon award of contract.

Information Technology Systems Security
Contractor Non-Disclosure Agreement

Access to sensitive information from the files of the Centers for Disease Control and Prevention (CDC) is required in the performance of my official duties, under contract number_____between (CDC I/C Name or Component)

_____and my employer (Employer's Name)

_____. I agree that I shall not release, publish, or disclose such information to unauthorized personnel, and I shall protect such information in accordance with relevant laws and regulations available for research and review at any Law Library. Among these laws may be various provisions of:

a) 18 U.S.C. 641 (Criminal Code: Public Money, Property or Records)

b) 18 U.S.C. 1905 (Criminal Code: Disclosure of Confidential Information)

c) Public Law 96-511 (Paperwork Reduction Act)

d) 5 U.S.C. 552a (Privacy Act)

I affirm that I have received a written and/or verbal briefing by my company concerning my responsibilities under this agreement. I understand that violation of this agreement may subject me to criminal and civil penalties.

Signed: _____

Type or Print Name: _____

Date: _____

Witnessed by: _____

Date: _____

Copies are to be retained by:
Contractor Official
Contracting Officer

WTCHP Incident Reporting Template

	CDC WTC Incident Check List	
Monitoring and Treatment 9.11 WTC Health Program	**INCIDENT NO:**	
	TITLE OF INCIDENT:	
	DISCOVERY DATE:	
	BREACH DATE:	
	PREPARER:	

Type of Media: ___ Electronic ___ Paper ___ Other (Please specify)

Type of Incident: ____Matching Error____ Unauthorized Release of Information _____Other

Incident caused by: ____ Data Base Error _____ Human Error _____ Process Error _____Other

Description of Incident:_____

Estimated Number of Affected Individuals:
_____

CCE or NPN Involved:
_____

What Type of PHI is involved: ____ Certification letter(s): _____ WTCHP Member file(s) ____VCF Error

Description of PHI Involved:_____

Note: If Certification Letter(s) Include copy of letter(s) with incident report.

Mitigation:

- Letter destroyed by recipient: ___ Yes ___No ___ Don't Know
  Explain:

- Corrective Actions: Actions Taken to Prevent Recurrence:

Acronyms

ASA	Assistant Secretary for Administration
ASFR	Office of the Assistant Secretary for Financial Resources
BI	Background Investigation
CDC	Centers for Disease Control and Prevention
CIO	Chief Information Officer
CISO	Chief Information Security Officer
COR	Contracting Officers Representative
CSIRC	Computer Security Incident Response Center
CSIRT	Computer Security Incident Response Team
DA	Division of Acquisition
FAR	Federal Acquisition Regulations
FIPS	Federal Information Processing Standards
FISMA	Federal Information Security Management Act of 2002
FY	Fiscal Year
CDC	Department of Health and Human Services
CDCAR	Health and Human Services Acquisition Regulations
HIPAA	Health Insurance Portability and Accountability Act
HSPD	Homeland Security Presidential Directive
IS2P	Information Systems Security and Privacy Policy
ISSO	Information Systems Security Officer
IT	Information Technology
MBI	Minimum Background Investigation
NACI	National Agency Check and Inquiry Investigation
NDA	Non-Disclosure Agreement
NIST	National Institute of Standards and Technology
OCIO	Office of the Chief Information Officer
OGAPA	Office of Grants and Acquisition Policy and Accountability
OMB	Office of Management and Budget
OpDiv	Operating Division
OSSI	Office of Security and Strategic Information
PII	Personally Identifiable Information
PIV	Personal Identification Verification
PM	Program Manager
RBT	Role Based Training
RFP	Request for Proposal
RoB	Rules of Behavior
SCAP	Security Content Automation Protocol
SORN	System of Record Notice
SP	Special Publication
SSP	System Security Plan
STAFFDIV	Staff Division
USGCB	United States Government Configuration Baseline
USC	United States Code
WTCHP	World Trade Center Health Program

World Trade Center Health Program
Business Associate Agreement

A. Definitions

All terms used herein and not otherwise defined shall have the same meaning as in the Health Insurance Portability and Accountability Act of 1996 ("HIPAA") (Pub. L. 104–191; 42 U.S.C. § 1320d), as modified, and the corresponding implementing regulations, including the Privacy, Security, Breach Notification, and Enforcement Rules (45 C.F.R. pts. 160, 162, and 164). Non-HIPAA related provisions governing the Contractor's duties and obligations, such as those under the Privacy Act and any applicable data use agreements, are covered elsewhere in the contract and other contract-related documents.

"Business Associate" shall generally have the same meaning as the term "business associate" at 45 C.F.R. § 160.103 and, for the purpose of this agreement and any underlying contract between the parties, shall mean [Contractor], if/when the Business Associate performs functions or activities on behalf of the Covered Entity where the Business Associate creates, receives, maintains, or transmits "protected health information" or where the Business Associate provides certain services to or for the Covered Entity which require the disclosure of protected health information to the Business Associate by the Covered Entity.

"Covered Entity"[1] shall generally have the same meaning as the term "covered entity" at 45 C.F.R. § 160.103 and, for the purpose of this agreement and any underlying contract between the parties, shall mean the World Trade Center Health Program (WTC Health Program) and any other NIOSH, CDC, or HHS components to the extent that they assist in administering the WTC Health Program where protected health information is involved.

"Protected Health Information" (PHI) shall generally have the same meaning as the term "protected health information" at 45 C.F.R. § 160.103. The HIPAA regulations define PHI as individually identifiable health information (health information, including demographic information, collected from an individual and created or received by a health care provider, health plan, employer, or health care clearinghouse that relates to the individual's past, present, or future mental or physical health condition, provision of health care, or payment for care and which identifies the individual or is reasonably believed to make the individual identifiable) that is transmitted by electronic media, maintained in electronic media, or transmitted or maintained in any other form or medium. PHI excludes individually identifiable health information in certain education records, certain student medical records, employment records held by a covered entity in its role as employer, and records regarding a person who has been deceased for more than 50 years.

"Secretary" shall mean the Secretary of the Department of Health and Human Services or the Secretary's designee.

B. Obligations and Activities of Business Associate

Business Associates, as well as their agents and subcontractors, shall meet all applicable HIPAA obligations. Furthermore, they shall document in writing the policies and procedures that will be used to meet such obligations.

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[1] The term "covered entity" is used in this section for ease of understanding. However, a more precise description of the application of HIPAA to the WTC Health Program is as follows:

HHS is a hybrid entity under HIPAA, meaning HHS is a covered entity that conducts bnsiness activities, including both covered and non-covered functions, and designates "health care components" in accordance with 45 C.F.R. § 164.105(a)(2)(iii)(D). 45 C.F.R. § 164.103. As a hybrid entity, HHS must designate any component that would "meet the definition of a covered entity or business associate if it were a separate legal entity" as a health care component; a health care component also may include a component only to the extent that it performs covered functions. 45 C.F.R. § 164.105(a)(2)(iii)(D). Accordingly, the WTC Health Program is a "health care component" of the covered entity, HHS; as are any other NIOSH, CDC, or HHS components if they would meet the definition of a covered entity or business associate if they were separate legal entities and only to the extent that they perform covered functions.

These obligations include the following:

1. Business Associate agrees to not use or disclose PHI other than as permitted or required by this Contract or as required by law.

2. Business Associate agrees to prevent use or disclosure of PHI other than as provided for by this Agreement through use of appropriate safeguards and complying with Subpart C of 45 C.F.R. Part 164 with respect to electronic PHI. In accordance with 45 C.F.R. §§ 164.306 and 164.316, Business Associate shall implement written policies and procedures to:

    a. Prevent, detect, contain, and correct security violations[2] through the use of:
        i. Risk analyses (including periodic technical and nontechnical evaluations);[3]
        ii. Appropriate risk management strategies (including information system activity review);[4]
        iii. Information access procedures for approving individual's access rights to PHI (including the implementation of workforce security measures to ensure continued appropriate role-based access to PHI over time), and technical policies and procedures to ensure compliance with grants of access (including unique user identification and emergency access procedures);[5] and
        iv. The imposition of sanctions of workforce members of Business Associate for violations.[6]
    b. Limit physical access to its electronic information systems and the facility or facilities in which they are housed.[7]
    c. Limit access to PHI through workstations and other devices, including access through mobile devices.[8]
    d. Employ media controls covering the movement of devices containing PHI within or outside of the BA's facility as well as the disposal and reuse of media containing PHI.[9]

3. Business Associate agrees to report to Covered Entity any use or disclosure of PHI not provided for by this Agreement which it discovers, including breaches of unsecured PHI as required at 45 C.F.R. § 164.410, and any security incident[10] of which it becomes aware, including those of its agents and subcontractors. The Business Associate shall report any violation in use or disclosure involving PHI, any security incidents, and any breaches involving unsecured PHI to Covered Entity (designated WTC Health Program point of contact for HIPAA concerns) within ten (10) business days of discovery. Upon reporting of a potential breach by Business Associate, Covered Entity may engage Business Associate in coordinating notification actions.

4. Business Associate agrees to ensure, in accordance with 45 C.F.R. § 164.502(e)(1)(ii) and, if applicable, 45 C.F.R. § 164.308(b)(2), that any agents or subcontractors that create, receive, maintain, or transmit PHI on behalf of Business Associate agree through a written contract or other arrangement to the same restrictions, conditions, and requirements that apply to Business Associate with respect to such information.

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[2] *See* 45 C.F.R. § 164.308(a)(1)(i).

[3] *See* 45 C.F.R. § 164.308(a)(1)(ii)(A) and (B).

[4] *See* 45 C.F.R. § 164.308(a)(1)(ii)(D) and (a)(8).

[5] *See* 45 C.F.R. § 164.308(a)(3) and (4); 45 C.F.R. § 164.312(a)(2).

[6] *See* 45 C.F.R. § 164.308(a)(1)(ii)(C).

[7] *See* 45 C.F.R. § 164.310(a)(1).

[8] *See* 45 C.F.R. § 164.310(b), (c), and (d).

[9] *See* 45 C.F.R. § 164.310(d).

[10] "Security incident" is defined as the "attempted or successful unauthorized access, use, disclosure, modification, or destruction of information or interference with system operations in an information system." 45 C.F.R. § 164.304.

5. Business Associate agrees to provide access, at the request of Covered Entity, to PHI in a designated record set to Covered Entity or, as directed by Covered Entity, to an Individual or an Individual's designee in order to meet the requirements under 45 C.F.R. § 164.524.[11]

6. Business Associate agrees to make any amendment(s) to PHI in a designated record set that Covered Entity directs or agrees to pursuant to 45 C.F.R. § 164.526 npon request of Covered Entity, or take other measures as necessary to satisfy Covered Entity's obligations under 45 C.F.R. § 164.526.[12]

7. Business Associate agrees to maintain and make available any information required to provide an accounting of disclosures to Covered Entity, or to an individual identified by Covered Entity as requesting such an accounting, as necessary to satisfy Covered Entity's obligations under 45 C.F.R. § 164.528.[13]

8. Business Associate agrees that, to the extent the Business Associate carries out one or more of Covered Entity's obligation(s) under Subpart E of 45 C.F.R. Part 164, including providing access, making amendments, or providing an accounting of disclosures, as referenced in this Agreement, it will comply with the requirements of Subpart E that apply to Covered Entity in the performance of such obligation(s).

9. Business Associate agrees to make its facilities, books, records, accounts, and other sources of information, including PHI, that are pertinent to ascertaining its own or Covered Entity's compliance with the applicable HIPAA provisions, available to Covered Entity; or, in the context of an investigation or compliance review, to the Secretary for purposes of the Secretary determining Business Associate's or Covered Entity's compliance with the various rules implementing HIPAA.

10. Business Associate agrees to designate a security official who will be responsible for development and implementation of its security policies and procedures, including workforce security measures to ensure proper security awareness and training (including security incident response and reporting), and security incident procedures.

11. Business Associate agrees to use appropriate administrative, physical, and technical safeguards that reasonably and appropriately protect the confidentiality, integrity, and availability (including the use of contingency plans) of the electronic PHI, as defined in 45 C.F.R. §160.103, that it creates, receives, maintains, or transmits on behalf of the Covered Entity to prevent impermissible use or disclosure of such electronic PHI. In establishing such safeguards, Business Associate shall consider its size, complexity, and capabilities, as well as its technical infrastrncture and its hardware and software security capabilities.

12. Business Associate agrees to assess and, where appropriate, implement any addressable implementation specifications associated with applicable PHI security standards.

13. Business Associate agrees not to threaten, intimidate, coerce, harass, discriminate against, or take any other retaliatory action against any individual for the following: filing a complaint under 45 C.F.R. § 160.306; testifying, assisting, or participating in an investigation, compliance review, proceeding, or hearing under 45 C.F.R. Part 160; or opposing any act or practice that is unlawful under HIPAA, provided there is a good faith belief that the practice is unlawful, the manner of opposition is reasonable, and the opposition does not involve the disclosure of PHI in violation of subpart E of Part 164.

14. Business Associate agrees not to sell PHI other than as permitted by 45 C.F.R. § 164.508(a)(4).

15. Business Associate agrees to make reasonable efforts to limit the PHI it uses, discloses, or requests to the minimum necessary to accomplish the intended purpose of the permitted use, disclosnre, or request in accordance with 45 C.F.R. § 164.502(b).

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[11] *See* 45 C.F.R. § 164.504(e)(2)(ii)(E).

[12] *See* 45 C.F.R. § 164.504(e)(2)(ii)(F).

[13] *See* 45 C.F.R. § 164.504(e)(2)(ii)(G).

16. Business Associate may use PHI for the proper management and administration of the Business Associate or to carry out the legal responsibilities of the Business Associate.

17. Business Associate may disclose PHI for the proper management and administration of Business Associate or to carry out the legal responsibilities of Business Associate, provided the disclosures are required by law, or Business Associate obtains reasonable assurances from the person to whom the information is disclosed that the information will remain confidential and used or further disclosed only as required by law or for the purposes for which it was disclosed to the person, and the person notifies Business Associate of any instances of which it discovers that the confidentiality of the information has been breached.

C. Permitted Uses and Disclosures by Business Associate

Except as otherwise limited in this Agreement, Business Associate may use or disclose PHI on behalf of, or to provide services to, Covered Entity for purposes of the performance of this Agreement, if such use or disclosure of PHI would not violate either the HIPAA Privacy or Security Rules if done by Covered Entity or the minimum necessary policies and procedures of Covered Entity.

D. Obligations of Covered Entity

1. Covered Entity shall notify Business Associate of any limitation(s) in its notice of privacy practices of Covered Entity in accordance with 45 C.F.R. § 164.520, to the extent that such limitation may affect Business Associate's use or disclosure of PHI.

2. Covered Entity shall notify Business Associate of any changes in, or revocation of, permission by Individual to use or disclose PHI, to the extent that such changes may affect Business Associate's use or disclosure of PHI.

3. Covered Entity shall notify Business Associate of any restriction to the use or disclosure of PHI that Covered Entity has agreed to in accordance with 45 C.F.R. § 164.522, to the extent that such restriction may affect Business Associate's use or disclosure of PHI.

E. Term and Termination

1. The term of this Agreement, which is fully incorporated into the overall Contract as an Exhibit, shall be effective as of the date of contract award, and shall terminate when all of the PHI provided by Covered Entity to Business Associate, or created, received, maintained, or transmitted by Business Associate on behalf of Covered Entity, is destroyed or returned to Covered Entity or, if Business Associate needs to retain PHI for its own management and administration or to carry out its legal responsibilities, in accordance with the termination provisions in this Section.

2. Upon Covered Entity's knowledge of a material breach by Business Associate, Covered Entity shall take one of the following actions:

    a. Provide an opportunity for Business Associate to cure the breach or end the violation consistent with the termination terms of this Agreement. Covered Entity may terminate this Agreement, which would result in terminating the overall Contract with Business Associate, for default if the Business Associate does not cure the breach or end the violation within fifteen (15) business days; or,
    b. Consistent with the terms of this Agreement, terminate this Agreement, as well as the overall Contract, for default if Business Associate has breached a material term of this Agreement and cure is not possible.

3. Effect of Termination.

    a. Upon termination of this Agreement and overall Contract for any reason, Business Associate, with respect to PHI created, received, maintained, or transmitted by Business Associate on behalf of

Covered Entity, shall:

i. Retain only that PHI which is necessary for Business Associate to continue its proper management and administration or to carry out its legal responsibilities;
ii. Return to Covered Entity or destroy the remaining PHI that the Business Associate still maintains in any form;
iii. Continue to use appropriate safeguards and comply with Subpart C of 45 C.F.R. Part 164 with respect to electronic PHI to prevent use or disclosure of the PHI, other than as provided for in this Section, for as long as Business Associate retains the PHI;
iv. Not use or disclose the PHI retained by Business Associate other than for the purposes for which such PHI was retained and subject to the same conditions set out at Paragraphs b.16 and 17 above which applied prior to termination; and
v. Return to Covered Entity or destroy the PHI retained by Business Associate when it is no longer needed by Business Associate for its proper management and administration or to carry out its legal responsibilities.

b. Upon termination of this Agreement and overall Contract for any reason, Bnsiness Associate shall obtain PHI created, received, maintained, or transmitted by agents and subcontractors of Business Associate on behalf of Covered Entity.

F. Miscellaneous

1. A reference in this Agreement to a section in the Rules issued nnder HIPAA means the section as in effect or as amended.

2. PHI provided by Covered Entity to Business Associate and its contractors, subcontractors, or other agents, or gathered by them on behalf of the Covered Entity nnder this Agreement is the property of the Covered Entity.

3. The Parties agree to take such action as is necessary to amend this Contract from time to time as is necessary for Covered Entity to comply with the requirements of the Rules issued under HIPAA and any other applicable law.

4. The respective rights and obligations of Business Associate under paragraph E.3 of the section titled "Term and Termination" shall snrvive the termination of this Agreement.

5. Any ambignity in this Agreement shall be resolved to permit Covered Entity to comply with the Rules implemented nnder HIPAA.

_____ _____
Signature, On behalf of vendor Date:

Name:_____

Title:_____

Organization:_____

_____ _____
On behalf of the World Trade Center Health Program Date:
Dr. John Howard, Program Administrator

Attachment #1

C.1 Background

The attacks at the World Trade Center (WTC) on September 11, 2001 exposed rescue and recovery workers, as well as residents/non-responders, to unprecedented risks for injury, illness, and death. Funds were appropriated by Congress to the Centers for Disease Control and Prevention (CDC's) National Institute for Occupational Safety and Health (NIOSH) to provide medical screening for rescue and recovery workers beginning in fiscal year 2002. Additional funds were later appropriated for medical monitoring, diagnostic, and medical treatment services for responders for conditions related to the 9/11 attacks and initial health evaluation, diagnostic, and treatment services for residents, students, and others in the community directly affected by the 2001 WTC disaster.

On January 2, 2011, the President signed Public Law No. 111-347, the James Zadroga 9/11 Health and Compensation Act ("Zadroga Act"), providing a longer term solution to care for the WTC responders and survivors. The Zadroga Act added Title XXXIII to the Public Health Service Act, which authorized the WTC Health Program as a federal health care program for eligible responders and survivors of the 9/11 terrorist attacks. The Zadroga Act can be found at https://www.congress.gov/bill/111th-congress/house-bill/847

The Zadroga Act encompasses a myriad of responsibilities associated with the administration and performance needed to enroll, initially evaluate, medically monitor, and manage allowable health conditions for eligible individuals through diagnostic and treatment services. Responsibilities included the construction of a comprehensive medical and pharmaceutical benefit program consistent with a limited care model; similar in constraints to a worker compensation model, yet requiring the flexibility to engage with advocates and stakeholders. To align with legal requirements, the WTC Health Program promulgated regulations guiding program implementation and created member-based health benefit plans restricted to the conditions certified by the WTC Health Program Administrator for coverage. In addition, a new business processing component was developed to adjudicate and pay medical and pharmacy claims from enrolled providers for allowable services authorized by Clinical Centers of Excellence (CCEs) to manage the care of a restricted set of health conditions for eligible program members. Required administrative processes included procedures for appeals related to denials of program enrollment, health condition certification, or treatment bills. The WTC Health Program had to be operational within 6 months of the passage of the authorizing legislation - by July 1, 2011.

In the spring of 2011, the CDC awarded a contract to provide operational support services to assist NIOSH with implementation of the WTC Health Program as defined by the Zadroga Act. This contract was designed to complement NIOSH activities and a framework of contracted health care entities in the New York City metropolitan area and a large provider network for members across the United States and its territories. Services involved receipt and adjudication of claims for rendered health care, submission of processed claims to another federal entity for payment (as required by the Zadroga Act), assistance with enrollment of new members, assistance with certification (approval) of health conditions of members, and implementation of federal information technology standards for security and privacy. Member and provider support services had to be provided and tailored to the unique needs of the program. Additionally, the contractor had to be engaged with stakeholders through formalized processes related to Steering Committees established under the Zadroga Act.

Significant challenges have been navigated and lessons learned to implement this complex program, creating a key resource in the form of institutional memory.

As of December 2015, the WTC Health Program had approximately 73,800 members enrolled, including 64,000 responders and 8,500 survivors. Between 1/1/15 and 12/31/15 the WTC Health Program delivered monitoring or screening exams to an estimated 26,800 members, diagnostic services to approximately 17,900 members, and treatment services to approximately 22,600 sick or injured members. The current statistics and reports can be found at http://www.cdc.gov/wtc/reports_portal.html.

In supporting WTC Health Program, the contractor will interact with the Clinical Centers of Excellence (CCEs), the Nationwide Provider Network (NPN) contractor, the Data Centers (DCs) contractor, the Health Program Evaluation (HPE) contractor, and the Centers for Medicare and Medicaid Services (CMS) (to reimburse authorized health care services).

C.2 Project Objective

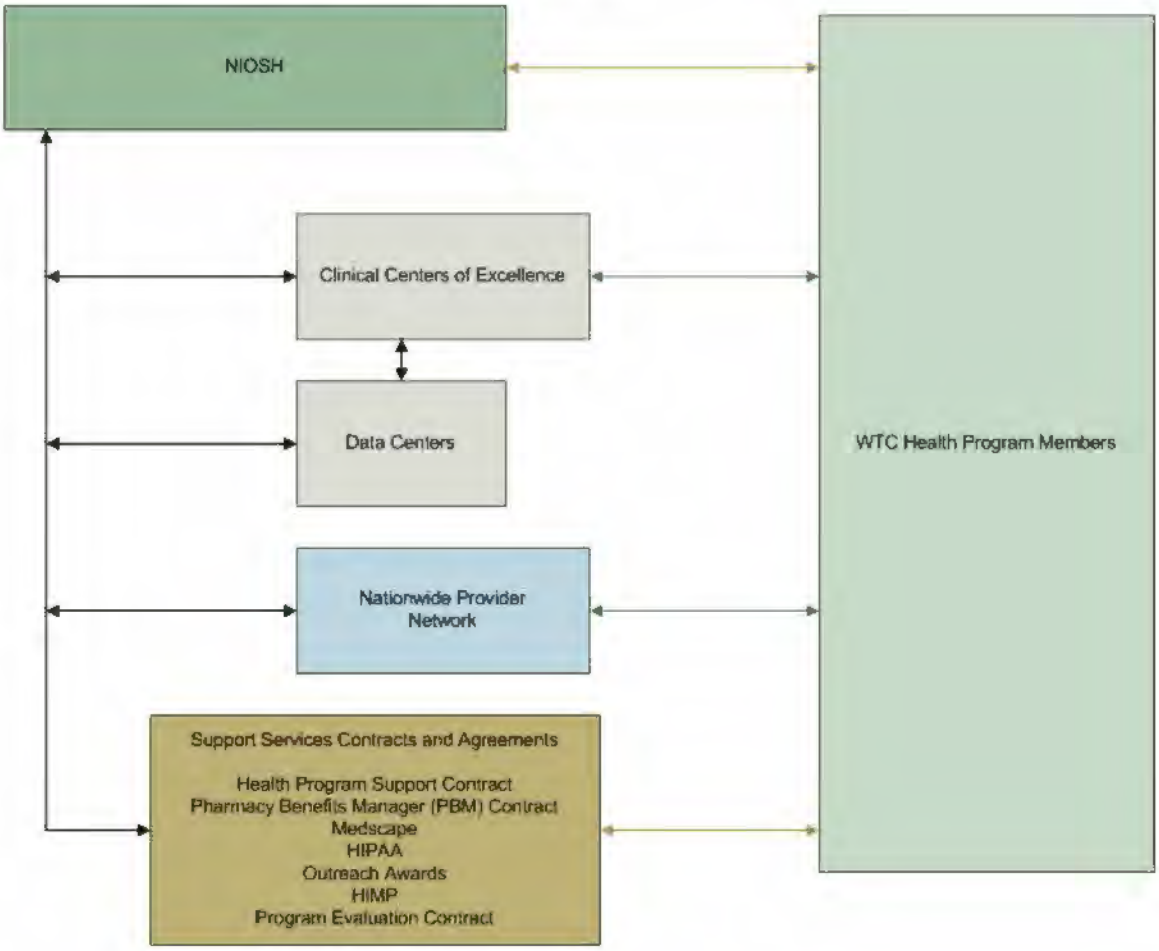
The project objective for this requirement is to provide support services to the WTC Health Program in the areas detailed in C.3 Scope of Work below. These support services will assist NIOSH in the continued operation of the WTC Health Program.

Federally Funded Program

The World Trade Center Health Program is a federally funded health benefits program and therefore must comply with all federal regulations and requirements.

Figure C.2.1 shows a diagram of the components of the WTC Health Program that are relevant to this project. In addition, the HHS Secretary delegated authority to CMS to provide payment services for WTC Health Program patient care claims.

Figure C.2.1 Components of the WTC Health Program



C.3 Scope of Work

The scope of work for this requirement includes:

- Contract Management
- Member Services and Medical Benefits Support
- Provider Network Management
- Claims Processing and Adjudication
- Case Management
- Data Management, Analytics, Decision Support and Reporting
- Fraud Waste and Abuse Compliance Plan and Reporting
- Quality Assurance and Internal Audits

C.3.1 HPS Contract Management

The contractor shall manage the contract to ensure that all subcontracts, staffing, and services, as well as any transition of those items, meet the requirements of the prime contract. The contractor shall implement project management objectives which focus on monitoring, controlling, and balancing risk related to the project's three key constraints -- scope, budget, and schedule -- on an ongoing basis.

C.3.1.1 Project Management Plan

The contractor shall develop and implement a Project Management Plan (PMP) which shall define appropriate technical and financial project management over all tasks and deliverable documents in accordance with this PWS. The PMP shall establish the staffing and procedural framework for accomplishing the work required under this PWS. The PMP shall establish a transparent management, schedule and budget, baseline from which project monitoring and quality control is routinely conducted to verify adherence to the PMP. The contractor's PMP shall include the proposed labor mix of personnel who will be responsible for implementing this requirement, including an identification of key personnel with the percentage of their time to be devoted to the contract.

A draft PMP shall be submitted with the contractor's proposal. The contractor shall submit a finalized, PMP to the WTC Health Program for approval within 30 days of the contract award according to feedback from the WTC Health Program. Once approved by the WTC Health Program, any changes to the PMP shall be updated as part of the monthly report and an updated PMP shall be submitted annually at the end of each contract year for the duration of the contract.

C.3.1.2 Operations Manual

The contractor shall develop and maintain a transferable (to the government) Operations Manual that contains a complete set of written standard operating procedures (SOPs) which guide all contract-related services, processes, and activities. The SOPs shall be used to meet the objectives outlined in the PWS and provide the requisite security and process support for the WTC Health Program to include evidentiary documentation for formal Federal audits as well as Security Certification and Accreditation of contractor-provided systems for the program. The Operations Manual shall be aligned with the Policy and Procedures Manual(s) for the WTC Health Program. The WTC Health Program Policy and Procedure Manual and Notices can be found at http://www.cdc.gov/wtc/ppm_portal.html. The contractor shall provide support of ongoing updates to the WTC Health Program Policy and Procedures Manual(s).

A draft Operations Manual shall be submitted with the contractor's proposal. The contractor shall submit a finalized Operations Manual to the WTC Health Program for approval within 60 days of the contract award according to feedback from the WTC Health Program. Once approved by the WTC Health Program, any changes to the Operations Manual shall be updated as part of the monthly report and an updated Manual shall be submitted annually for the duration of the contract.

C.3.1.3 Communications Plan

The contractor shall develop and maintain a comprehensive Communications Plan ***(post award)*** that will enable them to deliver appropriate messages or communicate effectively to all identified stakeholders in a consistent and timely manner. The Communications Plan will identify all communication activities among the contractor and the WTC Health Program, the CCEs, the DCs, the NPN contractor, other contractors to the WTC Health Program, the WTC Health Program Steering Committees, and WTC Health Program members.

As a part of the Communications Plan, the contractor shall provide administrative and meeting support to WTC Health Program including hosting conference calls, preparing agendas and action item lists, and recording issue summaries, with key decisions and action items for meeting documentation and make them available to the WTC Health Program in a timely and secure manner. The contractor shall also maintain, and when requested, provide formal program documentation to support any type of Federal Program Audits. The contractor shall provide, as requested and approved by the WTC Health Program, formatting for informational letters and materials and distribute to target audiences (e.g., members, CCEs, NPN, Data Centers); coordinate the review and approval of communication materials; coordinate the printing and mailing of communication materials, as requested and approved by the WTC Health Program; and prepare and provide educational materials on billing practices and procedures including the CCE/Provider Guide to Billing. The contractor shall implement quality assurance measures to ensure communication process efficiency and compliance with federal privacy laws and regulations by testing any automated outputs to ensure accuracy of content and addressee before materials are delivered.

A draft Communications Plan shall be submitted to the WTC Health Program for review within 60 days of contract award according to feedback from the WTC Health Program. Once approved by the WTC Health Program, any changes to the Communications Plan shall be updated as part of the monthly report and an updated Manual shall be submitted annually at the end of each contract year for the duration of the contract.

C.3.1.4 Data Security and Continuity of Operations Plan

The contractor shall develop and maintain a comprehensive Data Security and Continuity of Operations Plan ***(post award)***. The Data Security and Continuity of Operations Plan shall outline the contractor's policies and procedures for ensuring data security, continuity of operations, and the transfer of data to the WTC Health Program. The data security and continuity of operations plan should follow standard FISMA, HIPAA, and CDC guidelines. The Data Security and Continuity of Operations Plan shall follow the requirements of a certification and accreditation process and allow for continuous monitoring as outlined by CDC's Office of the Chief Information Officer (OCIO). A draft Data Security and Continuity of Operations Plan shall be submitted to the WTC Health Program for review within 60 days of contract award. As stipulated by HHSAR 352.222-70, the draft plan shall include identification of system of records and disposition as listed below:

> (a) Identify the system(s) of records and the design, development, or operation work the contractor is to perform and provide detailed description and point of contact (team or group) to the WTC Health Program;
>
> (b) Specify the disposition to be made of such records upon completion of contract performance. The contractor shall transfer all government records and all government furnished data within 30 days after the contract has ended or within the (to be) specified transition period. (Further details can be found in Attachment J-2, Information Technology Security and Privacy).

The contractor shall submit a finalized Data Security and Continuity of Operations Plan 2 weeks from receipt of comments by the WTC Health Program. Once approved, an updated Data Security and Continuity of Operations Plan shall be submitted to the WTC Health Program monthly for the duration of the contract. (See Attachment #2, Information Technology Security and Privacy and PWS Section

C.3.5.3 on Health Insurance Portability and Accountability (HIPAA) Act compliance for more information.)[1]

C.3.1.5 Risk Management Plan

The contractor shall prepare a Risk Management Plan ***(post award)*** in accordance with SEI (Software Engineering Institute) Risk Management Methods (or equivalent) and CDC policy for a continuous improvement approach to project performance. The Risk Management Plan shall include any potential program integrity risks and proposed strategies to reduce/mitigate these risks.

A draft Risk Management Plan shall be submitted to the WTC Health Program within 60 days of contract award. The contractor shall submit a finalized Risk Management Plan 2 weeks from receipt of comments by the WTC Health Program. Once approved by the WTC Health Program, any changes to the Risk Management Plan shall be updated as part of the monthly report and an updated Plan shall be submitted annually at the end of each contract year for the duration of the contract.

C.3.1.6 Fraud, Waste, and Abuse Compliance Plan

The contractor shall prepare a Fraud, Waste, and Abuse (FWA) Compliance Plan ***(post award)***. As part of the FWA Compliance Plan, the contractor shall designate executive and essential personnel to attend mandatory training in FWA detection, prevention, and reporting, and designate an officer or director in its organization with responsibility and authority for carrying out the provisions of the FWA Compliance Plan.

A draft FWA Compliance Plan shall be submitted to the WTC Health Program within 60 days of contract award. The contractor shall submit a finalized FWA Compliance Plan 2 weeks from receipt of comments by the WTC Health Program. Once approved by the WTC Health Program, any changes to the FWA Compliance Plan shall be updated as part of the monthly report and an updated Manual shall be submitted annually at the end of each contract year for the duration of the contract. (See Section C.3.6 for more details about the Fraud, Waste, and Abuse requirements.)

C.3.1.7 Quality Assurance and Internal Audit Plan

The contractor shall prepare a Quality Assurance and Internal Audit Plan. A draft Quality Assurance and Internal Audit Plan shall be submitted with the contractor's proposal. The contractor shall submit a finalized Quality Assurance and Internal Audit Plan within 60 days of contract award. Once approved by the WTC Health Program, any changes to the Quality Assurance and Internal Audit Plan shall be updated as part of the monthly report and an updated Manual shall be submitted annually at the end of each contract year for the duration of the contract. (See Section C.3.7 for more details about the Quality Assurance and Internal Audit requirements.)

C.3.1.8 Cost Tracking and Invoicing

The contractor shall develop and manage budgets for all work to be performed. Budget development and management includes tracking and analysis of costs and reporting budget-related issues to the Contracting Officer (CO).

The contractor shall submit monthly invoices with its Monthly Report which reflect adherence to budget and funding requirements. Monthly invoices shall be itemized and provide appropriate detail and backup to reflect reasonable and justifiable costs allocated to activities associated with its approved budget such as program management, member enrollment, certifications, provider enrollment, case management, claims processing, IT support, and other support duties.

[1] Please refer to FISMA compliance section for further details. IT Security and SA&A process is constantly evolving and subject to change with additional timeline and documentation. The successful award is advised to discuss the SA&A process with the WTC Health Program as soon as the award is made.

In addition to the above, the contractor shall provide quarterly a WTC Health Program invoice Reconciliation Report on the overall financial status of the contract. This report shall include detailed budget projections for the upcoming two quarters, including burn rate.

C.3.1.9 Subcontracting

The HPS contractor shall provide management and oversight of all subcontractors[2] as described in FAR Parts 42 and 44. This would include post-award conferences with subcontractors that involve the Contracting Officer and Contracting Officer's Representative.

The HPS contractor shall provide a subcontracting plan with their proposal to include all subcontracts to the Contracting Officer (CO) and Contracting Officer's Representative (COR) prior to award and any time during the period of the contract where subcontracts are added or modified. The HPS contractor shall manage any required subcontracts in a manner that best fulfills the requirements of the prime contract. The HPS contractor shall ensure its subcontractors and/or their agents who require the use of or access to individually identifiable information or protected health information under the provisions of this contract comply with WTC Health Program business associate agreements and federal law, policies and regulations, including FISMA, HIPAA, and CDC certification and authorization to operate requirements. The prime contractor will obtain a written assurance from each subcontractor to this effect. The HPS contractor shall notify the COR and CO in advance of placing any subcontract or modification, including the following information:

(i) A description of the supplies or services to be subcontracted.
(ii) Identification of the type of subcontract to be used.
(iii) Identification and selection of the proposed subcontractor.
(iv) A description of an oversight and quality assurance plan.

C.3.1.10 Change Control Process

The contractor shall manage the contract to ensure the required services are being performed and the required contract deliverables are being submitted. As part of this requirement, the contractor shall maintain a WTC Health Program-approved change control process to identify, record, assess and secure approval for changes to the project. The contractor and the WTC Health Program will follow this process to classify, prioritize, approve, or reject changes, so that implementation does not negatively impact core mission applications, functions, or live production environments. Change requests need to be defined clearly, including their cost and schedule implications, to allow the WTC Health Program to make appropriate decisions. The contractor shall receive prior authorization and approval of expenditures by the WTC Health Program before starting any work on changes. Further guidance on the Change Request process will be provided upon the award.

C.3.1.11 Steering Committees Updates

The contractor shall provide a monthly status which includes information on items such as enrollment, transfers, and medical and pharmacy claims to the WTC Health Program Steering Committees for monthly committee meetings. The contractor shall prepare the status in the form of an approved PowerPoint presentation. The contractor shall be requested to attend up to four steering committee meetings annually held in Manhattan, New York and/or Brooklyn, New York.

C.3.1.12 Technical Expert Support

The contractor shall identify and provide personnel with appropriate technical expertise to perform required

[2] Pursuant to FAR 44.101, Definitions, a "Contractor" means "the total contractor organization or a separate entity of it, such as an affiliate, division, or plant that performs its own purchasing." A "Subcontractor" means "any supplier, distributor, vendor, or firm that furnishes supplies or services to or for a prime contractor or another subcontractor."

technical advisory support to the WTC Health Program.

The contractor's technical experts shall include a cost-effective mix of specialists in Federal Health Care Delivery Policy and Procedure, End-to-End Health Care Claims Processing, Medical Billing and Coding, Medical Review and Utilization, Federal Health Information Technology, Federal Legislative/Regulatory Affairs as pertains to Federal health care system and benefit design, Provider Network Management, Education and Relations, Risk Management and Health Communication. The contractor shall have appropriate consultative and operational expertise standing up and supporting high-visibility federal healthcare programs that require exceptional end-user responsiveness. Technical experts shall be familiar with the Zadroga Act, Affordable Care Act, Federal Employees' Compensation Act, Social Security Act (Title XVIII), HIPAA, and other related federal health laws and regulations and maintain knowledge and skills needed to administer a targeted federal health care program.

The contractor shall provide technical assistance, as requested, to develop updates on medical policies and procedures upon approval by the WTC Health Program. The contractor shall conduct training on medical billing, pharmacy benefit support and other WTC Health Program procedures for the CCEs and NPN contractor. The contractor shall provide Quality Assessment/Quality Control analyses on the claims adjudication system to ensure it is functioning according to intended benefit policy. The contractor shall review, analyze, and maintain the WTC Health Program Code Books and programmed Care Suites to adjudicate claims for medical care across the WTC Health Program. The Care Suites enable benefit plans to be assigned which agree with program requirements for targeted care. The contractor shall update and maintain the WTC Health Program Code Book in three different forms – one appropriate for the claims adjudication system integration, one for the NPN contractor, and the third for the CCEs to utilize for benefit management. The contractor shall review, analyze and prepare formal recommendations for coverage of new condition codes with appropriate mapping to medical benefit plan(s) (i.e., Care Suite logic), health care service codes and procedures, durable medical equipment codes, pharmaceutical products with appropriate mapping to pharmacy benefit plan(s), and other medical services/supplies (i.e., ICD-10, ICD- 09, CPT, HCPCS, DME, RCC). The contractor shall perform medical review, service utilization reviews (claims and onsite chart review) and other medical quality assurance activities.

The contractor shall designate a position to oversee case management services. The individual serving in this function shall be experienced in practicing occupational medicine, internal medicine or family medicine (e.g., conducting occupational health screening and medical surveillance); preferably a board-certified specialist. The position may be filled by a Medical Director or Clinical Director or other suitable candidate the offers finds will appropriately execute the requirements of this effort. The HPS shall develop training to properly instruct the case management team about the potential exposures and health outcomes that have been associated with the WTC attack and the conditions and services covered by the WTC Health Program.

C.3.1.13 Program Improvement Plan

The contractor shall prepare a Program Improvement Plan ***(post award)*** which will support the design, implementation, and evaluation of a spectrum of services focusing on improving program performance. The Program Improvement Plan shall include a periodic operational assessment to evaluate total program performance and effectiveness of policies, procedures and workflows and compliance with the WTC Health Program Policies and Procedures Manual. A draft Program Improvement Plan shall be submitted to the WTC Health Program within 60 days of contract award. The contractor shall submit a finalized Program Improvement Plan 2 weeks from receipt of comments by the WTC Health Program. Once approved by the WTC Health Program, any changes to the Risk Management Plan shall be updated as part of the monthly report and an updated Plan shall be submitted annually at the end of each contract year for the duration of the contract.

The contractor shall stay abreast of evolving federal rules and industry innovation in the health care arena, and proactively engage with NIOSH to propose, facilitate and develop program improvements which address the needs of beneficiaries, the CCEs, the DCs, the Scientific/Technical Committee, Steering Committees,

members of the academic communities and other stakeholders. The contractor shall work closely with NIOSH to provide full program integrity including Fraud, Waste and Abuse considerations. The contractor shall address all the required mandates of the Zadroga Act and accompanying regulations and policies, and incorporate approved innovations or enhanced means of providing superior care and service delivery to program participants and stakeholders. The contractor's plan shall demonstrate their understanding of NIOSH's requirement to address Congress and other government organizations by providing program transparency, contractor efficiencies in performance, as well as reduced performance risk.

C.3.1.14 Reporting

The contractor shall provide communications and reports to the WTC Health Program regarding ongoing implementation and operational status as well as routine reports documenting program progress and risks related to program management and administration, including critical-success-factors and key-performance-indicators. In addition to the weekly updates of the Project Management Plan, the contractor shall submit accurate and detailed reports as required by this PWS. (See Section C.6, Deliverables for more information on reporting requirements.)

C.3.2 Member Services and Medical Benefits Support

The contractor shall provide technical and administrative support to the WTC Health Program in the areas of member services and medical benefits management. These services include, but are not limited to, member enrollment, medical condition certification, credentialed provider enrollment/relations, nurse advice line and supplemental case management upon request by a CCE/NPN, and the provision of a secure location for WTC Health Program information submission, a secure web portal, and a call center. (See Section C.3.3.2, Secure Location for WTC Health Program Information Submission and Section C.3.3.3, Secure Web-based Information Exchange System for more information.)

C.3.2.1 Medical Condition Certification

The contractor shall process certification requests from CCEs and the NPN contractor which allow a member's qualifying health condition to be treated using WTC Health Program funds. As a part of the process, the contractor shall notify the CCEs of the status of certifications (approved, denied, pended, or un-process able), update member records with health condition certification information, notify members of certification decisions and, in the case of denials of health condition certifications, notify the member with instructions about appeal rights. The contractor shall also manage member requests for appeals and coordinate with the WTC Health Program Appeals Coordinator to establish a complete case file within the WTCHP System of Record and provide weekly certification reports to the CCEs/Data Centers. An average of 700 administrative requests to certify a member's medical condition for treatment are processed each month. (See the World Trade Center Health Program website for more information: http://www.cdc.gov/wtc/index.html).

C.3.2.2 Care Coordination and Case Management

The contractor shall provide supplemental care coordination and case management for particular members as requested by CCEs or the NPN contractor. The major difference in case management of the HPS Contractor vs. that of the CCEs and NPN contractor is the HPS Contractor's capability to provide and manage case management service for severely ill and complexed cases at all phases of care continuum including post-acute care and hospitalization. The contractor's proposal should demonstrate experience providing case management services to assist with cancer care navigation, after-hours nurse advice line to support members with complex physical and mental health conditions, and technical support to providers regarding service authorization limitations.

The contractor's case management system and staff shall have the capability to provide comprehensive clinical support to members including:

- Diagnosis education, treatment options, and strategies to manage symptoms and side effects

- Access to evidence-based care (consistent with NCCN Guidelines®)
- Complication management
- End-of-life planning, hospice and psychosocial support to both patients and caregivers
- After-hours access to a nurse advice line or case management team, with documented linkage back to a member's CCE/NPN. The call center shall include access to case management service seven days per week/24 hours a day.

The HPS's Case Management staffing plan shall include the professional qualifications and skills required for clinical case management personnel, clinical coordination, appropriate recognition and handling of urgent or emergent psychiatric or medical situations, managing conflict, and administrative problem solving. The Case Management staffing plan shall be led by the Medical Director and certified registered nurse case manage at the management and team lead level with job description and required credential included in the PMP's HPS staffing plan.

The case managers should be responsible for maintaining excellent communication with the member's home CCE or NPN, and member's Patient Centered Medical Home (to notify this provider of evaluations and treatment performed through the WTC Health Program and to notify them about conditions and care that is not covered by the WTC Health Program but will need to be addressed by the Patient Centered Medical Home provider). The case managers should also have a system/process to deal with emergency/high-risk members and cases (e.g., multiple hospital readmissions, psychiatric emergencies, emergency discharge planning, emergent social issues, etc.). The case managers should also have a system/process for smoothly dealing with Hospitalizations, Skilled Nursing Facilities, Extended Care Facilities/Plans (for both physical and mental health conditions covered by the WTC Health Program), Rehabilitation Facilities, Home Health Care, Hospice Care and End of Life Care.

The HPS Case Management plan shall indicate knowledge and understanding of the WTC Health Program member population and ensure continuum of care is being coordinated from pre-acute, acute to post-acute phases once the case is referred. The HPS case management process shall ensure all referrals and referrals of prior referrals are coordinated and service summary reports are maintained. All referred rendering and billing providers shall be registered with the WTC Health Program prior to billing and must be informed of the prior approved services needed (in a non-emergency situation).[3]

C.3.3 Healthcare Provider Network Management

The contractor shall establish and maintain an adequate health care provider network on behalf of the WTC Health Program. This network shall be managed by the contractor, but the network arrangement (as constructed) shall be the property of the WTC Health Program. The contractor shall act on behalf of the WTC Program Administrator to manage the provider network and the providers shall be selected on the basis of their credentials and experience treating or diagnosing the health conditions covered by the WTC Health Program.

This network may include, but not be limited to, hospitals and ancillary service providers, specialty clinics, trauma centers, specialists, and primary care physicians, in sufficient numbers and at locations both safe and convenient to the member population.

The providers who will be conducting the standardized medical monitoring examinations shall be experienced in diagnosing and treating occupational and/or environmental disease and conducting medical surveillance; preferably board-certified physicians with a specialty in occupational, internal, or family medicine. The contractor shall work with the CCEs to identify providers to add to the network. The contractor shall also proactively identify providers to be added to the network. The contractor shall provide training to properly instruct the participating WTC Health Program providers as to the requirements of participation (i.e., fee structure, billing limitations, and authorization process), information regarding 9/11 exposure and health effects, and the limited care model of the program.

[3] Further policy guidance on post-acute care case management is forthcoming.

The contractor shall be responsible for ensuring that the health care providers who provide diagnostic and treatment services meet all requirements of the contract, including verification of appropriate credentials.

As the Provider Network Manager, the contractor shall ensure all providers are appropriately credentialed and have an approved network enrollment agreement in place. The contractor's provider verification services shall include a provider screening solution which continuously monitors all actively enrolled providers and related parties, generates alerts based on changes in risk factors, and automates the end-to-end provider screening process during all enrollment and re-enrollment periods.

The contractor shall ensure that all provider network agreements specify that this is a limited care benefit program, the fee ceiling is set by FECA (see "a" below), services must be authorized by the member's CCE/NPN before the claim will be processed, medical records shall be requested to support a claim for services, and that the balance billing of a member is not allowed. The contractor shall provide a secure means for electronic claims authorization.

The fee schedule (described below) and other requirements are described and updated in the WTC Health Program Policy and Procedures Manual that can be found at http://www.cdc.gov/wtc/ppm_portal.html

a. The Zadroga Act requires that the payment rate for treatment under the WTC Health Program be in accordance with the rates paid under the Federal Employees' Compensation Act (FECA), administered by the U.S. Department of Labor (DOL). For treatment not covered under FECA, the payment rate is pursuant to the applicable Medicare fee for service, as deemed appropriate by the Administrator. More information about FECA, including the methodologies used to calculate FECA rates, can be found at http://www.dol.gov/owcp/regs/feeschedule/fee.htm
Maximum payments under the WTC Health Program are the lesser of the calculated fee schedule amount or the billed amount.

b. The WTC Health Program also follows DOL's medical fee schedule used by the FECA program for covering inpatient hospital services. These services are subject to a reimbursement schedule based on the Medicare Inpatient Prospective Payment System (IPPS). That system assigns services to Diagnosis-Related Groups (DRGs) and adjusts rates for individual hospitals according to their specific cost index. Inpatient services not covered under the Medicare IPPS are reimbursed under a formula that is based on the cost-to-charge ratio (CCR) data tables published by CMS for rural and urban hospitals in each state. The full explanation of how fees are calculated under FECA may be found at:
http://www.dol.gov/owcp/regs/feeschedule/fee/fee12/READ_ME_FIRST_fs12_instructions.htm

Licensing and Insurance

The contractor shall ensure that all providers in the provider network are properly licensed under applicable state laws and/or regulations and are properly insured. Licensing and insurance documentation shall be reviewed at a frequency that meets industry standards.

Training Records

The contractor shall ensure that all monitoring or screening providers are properly trained in coordination with CCEs, DCs and the NPN contractor in the conduct and documentation of the standardized health exams.

The contractor shall ensure that all providers are educated regarding the limited care nature of this program, service authorization requirements, fee limitations, billing and dispute pathways, treatment or finding summaries for CCE/NPN or program staff. This includes initial and ongoing training. All training records shall be maintained and verified as part of the quality assurance plan. The contractor shall clearly describe the Provider Network Management process including automated solutions and quality assurance

plan in the Operations Manual.

C.3.3.1 Provider Enrollment

The CCEs and NPN contractor will identify medical providers based on the type of care their certified members require and will request that the contractor acquire provider enrollment information. Enrollment elements shall include, but are not limited to, the provider's National Provider Identifier (NPI), tax ID, practice address, and a signed form (e.g., Electronic Funds Transfer (EFT)/CMS-588 form) authorizing the enrollment of financial/banking information. The enrollment information shall be reviewed by the contractor prior to database entry, and if there are missing data elements or other questions, the contractor shall work closely with the CCEs, the NPN contractor, and providers to obtain the required information. The provider NPI and name shall be verified by the contractor against a NPI registry such as the National Plan & Provider Enumeration System (NPPES), administered by the Centers for Medicare and Medicaid Services (CMS). The routing number is also verified against a trusted routing directory, such as the Federal Reserve Bank Services (FRBS) repository. In addition to required data elements, the contractor shall capture information that further distinguishes providers, such as authorization to prescribe to members, special case or CCE agreement status, and the provider's medical specialty.

The contractor shall work with the CCEs and/or the NPN contractor to identify provider additions to the network. The contractor shall also proactively identify providers to be added to the network.

There are currently approximately 280 different provider specialties which include cancer care, medical and surgical specialty care, ancillary support (e.g., anesthesiology, pathology, radiology, etc.), organ transplantation services, emergency care services, skilled nursing care, durable medical equipment (DME) providers, and inpatient service facilities, among others. The number of enrolled providers currently includes approximately 16,000 unique providers.

The contractor shall produce a shared provider list of all enrolled providers and make it available for CCE and NPN use. The contractor shall employ an automated electronic solution to enhance efficiency in managing the provider network. Provider enrollment into the WTC Health Program and the management of a reliable provider network are critical to efficient claims processing. The complete and accurate enrollment of providers ensures that claims are not suspended for review of provider validity issues, and that claims may be promptly adjudicated.

Annual Confirmation of Authorized Providers

The contractor shall ensure that the list of authorized providers in the WTC Health Program Provider Network is accurate and up-to-date. At the minimum, the contractor shall annually distribute a comprehensive list of enrolled providers to each CCE and/or the NPN contractor, which the CCE and/or the NPN contractor must review and make any updates or note any discrepancies between the providers held on record by the WTC Health Program and those held on record by the CCE and/or the NPN contractor. The contractor is responsible for working with the CCE and/or the NPN contractor to resolve any discrepancies in a timely fashion.

C.3.3.2 Secure Location for WTC Health Program Information Submission

The contractor shall provide a secure location for secure submission of applications for enrollment of members, requests for certification of health conditions, appeals of decisions by the WTC Health Program, and other communications from members and providers. The location should support the X12N Electronic Data Interchange (EDI), Secure File Transfer Protocol (SFTP), a secure, web-based information exchange system using Secure Socket Layer (SSL) protocols, and other solutions that are acceptable based on the type of data being transferred. The secure location shall serve as a centralized mailing center for written WTC Health Program communications or claims (inbound & outbound mail).

C.3.3.3 Secure Web-based Information Exchange System

The contractor shall provide a secure, web-based information exchange system that shall offer role-based easy- to-navigate menus for government users, HPS support teams, the CCEs and NPN, and providers. The secure web information exchange system must be highly configurable to address the unique needs of the WTC Health Program.

The information exchange system shall support all standard browsers (Microsoft Edge, Internet Explorer, Firefox, Safari, Chrome). Access shall be encrypted through 128/256-bit HTTPS/SSL certificates and authorized through the system's configurable HIPAA-compliant role and group-based security interface. CCEs and the NPN contractor shall be able to view the status of their claims and approve claims for services rendered on their behalf by other providers, which may entail information processing across CCE/NPN-customized authorization systems.

The information exchange system shall comply with all sections of the Americans with Disabilities Act (ADA), accessibility standards established under section 508 of the Rehabilitation Act (standards that provide greater accessibility for individuals with disabilities), the Older Americans Act, the Web Content Accessibility Guidelines WCAG 2.0, FISMA, and federal civil rights laws

The secure, self-service web information exchange system shall, at a minimum, address the needs of the CCEs, the NPN contractor, providers, and the WTC Health Program Administration Office (NIOSH).

Provider Web Information Exchange System

The purpose of the provider web information exchange system is to provide a self-guided user interface to enter or track claims/authorizations and download program content, such as Code Books. The information exchange system shall also provide self-administration screens that allow providers to change information, such as mailing addresses.

The provider web information exchange system shall offer a single point of entry for all provider-related functions. Information captured in the system's provider database shall be capable of fully supporting the WTC Health Program in monitoring and maintaining its enrolled providers to ensure that WTC responders and survivors have access to high-quality, cost-effective, and effectively managed health care. The secure provider web information exchange system shall have the capability for the online entry, verification, and update of provider information to ensure that only qualified providers who comply with program rules and regulations are reimbursed for services rendered to eligible program beneficiaries.

The secure provider web information exchange system shall be available 24/7 from any computer and allow access to self-service features including verification of WTC Health Program member eligibility, online entry of claims, inquiry of claim and payment status, electronic funds transfer (EFT) capabilities, 1099 download capabilities, secure inbox-messaging and computer-based training (CBT). The provider web information exchange system shall also facilitate the WTC Health Program's ability to communicate with providers and to receive valuable feedback that can be used to improve services.

The secure provider web information exchange system shall integrate user-configurable and detailed rules, processes, and procedures for the maintenance of all required provider information. Real-time entry, verification, and updating of provider-related information ensure that it is accurate and readily available for provider inquiry and system processes such as claims adjudication, claims payment, and reporting. This information can also assist the WTC Health Program in monitoring and maintaining its provider network to ensure that all program beneficiaries have access to high-quality, cost-effective, and effectively managed health care.

Program Administration Web Information Exchange System

The program administration web information exchange system shall provide a dashboard view into the overall system that includes system information such as claims, enrollments, and certifications on all

members. This provides insight to NIOSH staff about internal operations and will play an important role in decision making. WTC Health Program-authorized users shall be able to run standard or customized reports and download or print report results. The secure web information exchange system's reporting capability must be highly configurable to address the unique needs of the WTC Health Program.

C.3.3.4 Call Center

The contractor shall maintain a call center to receive and respond to inquiries from potential enrollees, members, and providers. The contractor shall also provide escalation as needed for properly responding to inquiries. The WTC Health Program call center currently receives approximately 100 calls per day. The contractor's proposal shall define the hours of operation and other operation details (for example, translation services) about the call center.

The contractor's call center shall have the ability to capture a broad range of data to track and measure inquiry disposition, including the capture and retention of various program-specific data. These data include caller name, applicant name, date and time of the contact, results of the call, inquiry status, inquiry type (Enrollees, Members, and Providers), method of inquiry receipt, method of inquiry response, follow-up assignments and related transfers and escalations, follow-up and closure details, and FAQs. This data needs to be protected to be fully compliant with required HIPAA and FISMA security standards. The call center shall have the ability to link duplicate inquiries made to the call center in a seamless manner to support call flow. Whether the customer contact is a new or repeat inquiry, the call center systems shall enable call center personnel to establish any necessary follow-up, escalation, or callback activities.

Call center personnel shall have the aptitude to respond directly to contacts in both English and Spanish. For other languages, the use of a company that provides translation and interpretation services for inbound and outbonnd communications, both written and verbal, is acceptable.

C.3.4 Claims Processing

The contractor shall provide a complete array of claims-related services including claims receipt, claims adjudication, submitting claims to another entity for payment, and assistance with claims reconciliation. The contractor shall assess and provide a high volume of technical experts that may be necessary during the transition phase to be operationalized within a timely manner. The contractor shall work closely with the WTC Health Program, CMS, the CCEs, the NPN contractor, and other stakeholders of the program to ensure interoperability of the current, diverse and unique systems being used by the incumbent contractors to avoid major development work; enhance capacities and capabilities; and minimize cost while maximizing efficiency.

The contractor shall conduct ongoing reviews of claims to detect and resolve fraud, waste, and abuse (FWA) incidents before and after claims adjudication, taking into account the return on investment of the FWA activities. See Section C.3.6 for more information on Fraud, Waste, and Abuse activities.

The contractor's FWA activities shall provide a proactive deterrent to FWA which provides for greater containment of health care costs through the recovery of payments for erroneous billings and deterrence of fraudulent behaviors. The contractor's FWA solution shall be fully integrated with their claims processing solution and use current and historical data from that system for FWA detection analysis. The FWA system should be able to conduct advanced analytics, mine the data, and learn from the data in order to find new patterns of FWA.

C.3.4.1 Electronic Data Exchange (EDI) and Paper Claims

The contractor shall receive ANSI-X12 837 files for ICD-10 electronic data interchange (EDI) claims from CCEs and the NPN contractor, as well as either EDI or paper claims (on an approved exception basis) from the network of providers that are external to the CCEs. The contractor shall create EDI claims from paper claims when submitted.

The contractor's system shall fully comply with HIPAA Privacy and Security standards and related ANSI-X12 transactions standards for electronic Protected Health Information (ePHI) transmission.

C.3.4.2 Authorization of Services by CCE or NPN

The contractor shall ensure that all medical claims are appropriately authorized by the assigned program CCE or NPN (based on member assignment) before service claims are further adjudicated.

C.3.4.3 Medical Claims Adjudication

The contractor shall adjudicate medical claims using a system that meets the needs of the WTC Health Program, covering the following:

- Professional and Institutional
- Limited set of health conditions that are covered and certified (approved) by the WTC Health Program and certified (approved) for members
- Limited durable medical equipment
- Plan service limits and pre-authorizations for certain procedures
- Pricing at a maximum of Federal Employees' Compensation Act (FECA) rates and some pricing at Medicare rates, including Diagnosis Related Group (DRG) pricing
- Pricing for limited dental coverage using 85% of the American Dental Association fee structure
- National Correct Coding Initiative (NCCI) edits
- National Comprehensive Cancer Network Guidelines http://www.nccn.org/professionals/physician_gls/f_guidelines.asp
- Milliman Healthcare Guidelines http://www.milliman.com/solutions/healthcare/

The contractor shall provide real-time entry and adjudication processing for all claim types, including dental claims. This shall allow the WTC Health Program and providers' access to claims information via the Web portal and downloadable reports.

Claims shall be subjected to a number of pre-payment audits, including duplicate checking, utilization review, and editing to ensure claims meet National Correct Coding Initiative (NCCI) edits, OWCP/FECA reimbursement, DRG pricing for inpatient institutional claims, member and provider enrollment status, and member certification conditions.

The contractor claims adjudication system shall use care suites or benefit plan packages and can be configured to determine covered services, service locations, covered providers, service limitations, and rate information for WTC Health Program beneficiaries. The system shall maintain the benefit plan structure online and allows authorized personnel to define any number of plans or programs.

Each care suite is associated in the system with services that are covered under the plan, subject to applicable conditions established by WTC Health Program policies. The services eligible for coverage are associated with one or more specific care suites. Each service-care suite association is governed by rules maintained in the system based on WTC Health Program policy.

The contractor shall submit a transition plan that includes how the Medical Claims Adjudication process with the incumbent contractor will be transferred or rebuilt to ensure optimized operations.

C.3.4.4 Medical and Pharmacy Claims Payment

The contractor shall electronically submit medical and pharmacy claim files to a contractor of the Centers for Medicare & Medicaid Services (CMS) for provider payment. Alternatively, pharmacy claims shall not be routed through the contractor, and shall be routed directly to the CMS contractor. Upon the award, the WTC Health Program will provide guidance on how pharmacy claims will be handled.

Each week, the contractor shall submit two data files to a CMS contractor. One file contains payment information and one file contains claims detail. Using these data, the CMS contractor pays the providers and provides Remittance Advice. The CMS contractor then returns a file to the contractor that provides payment details such as payment confirmation and EFT check numbers.

This process shall be supported by numerous quality assurance, reconciliation, and exception reporting activities, including the following:

- Secure file transmission in both directions with validation of receipt
- Weekly status meetings with the payment contractor
- Weekly payment reconciliation
- Monthly data quality reporting
- Monthly budgetary account reconciliation
- Prompt Pay Interest payment
- Weekly payment integrity process to prevent improper payments
- Expenditure forecasting by budgetary fiscal year
- Payment reissue processing
- Remittance advice troubleshooting and replacement
- Monthly expenditure reporting

C.3.4.5 Claims Reconciliation

The contractor shall support the CCEs, their external providers, and the NPN contractor on processed claims to facilitate reconciliation with submitted claims, corrective actions on denied claims, and dispute resolutions.

The CCEs and the NPN contractor can choose between electronic or a paper remittance advice that provides claim line detail to facilitate bill reconciliation. The CCEs and NPN can choose to receive an electronic payment file in the ANSI 835 format or structured data file format to reconcile claim payment for all their members, including both internal and external providers. The WTC Health Program, the CCEs and NPN receive monthly claims dashboards that provide lists of denied claims and summary charts that show the number and reasons for claims denials. Appeals of denied claims received by the contractor will be forwarded to the designated WTC Health Program representative with supporting documentation as requested to facilitate the Program's decision.

The contractor shall provide dedicated WTC Health Program-trained customer service representatives available to CCEs, the NPN contractor, and external providers to answer questions about their claims payments and denials through meetings, as needed.

The contractor shall provide a secure web-based information exchange system accessible to the WTC Health Program, the CCEs, the NPN contractor, and their providers. The web information exchange system shall provide all of the information necessary to resolve claims issues.

C.3.4.6 Coordination of Benefit and Recovery

The contractor shall prepare a Coordination of Benefit and Recovery (COB&R) Plan. The COB&R Plan shall establish and maintain a robust operationalized system and rules for COB&R functions for all claims involving COB with private and public insurers which shall pay before the WTC Health Program for survivor members and for members who have workers' compensation benefits. The plan shall outline which entity pays first using a variety of methods and programs to identify situations in which the WTC Health Program beneficiaries have other insurance that is primary to WTC Health Program. The contractor's (or its subcontractor's) COB&R activities shall include the collection, management, and reporting of other insurance coverage for beneficiaries and recovery of paid claims which were paid incorrectly or should have been paid by other insurances or coverages.

A draft COB&R Plan shall be submitted with the contractor's proposal. The contractor shall submit a finalized COB&R Plan to the WTC Health Program for approval within 60 days of the contract award. Once approved, an updated COB&R Plan shall be submitted to the WTC Health Program monthly for the duration of the contract.

C.3.5 Data Management, Analytics, and Reporting

In addition to the core claims processing system, the contractor shall develop a consolidated data repository for program reporting, along with a business analytics and reporting capability. This repository shall contain all data on claims processed through the WTC Health Program, allowing for detailed analysis of program operations and health care delivery. In addition, this repository shall be a key tool for program analysis.

This data repository shall also contain data from the current WTC Health Program to allow for various trends and comparisons. As part of this task, the contractor shall convert existing data into the data repository and integrate any other required data sources to provide consolidated reporting for the overall program.
This conversion process and the design of the data repository shall be developed in compliance with FISMA requirements, fully respecting patient privacy and information security.

In addition to the data repository, the contractor shall develop an analytics and reporting capability which will allow the government and other stakeholders (as authorized) to report on program metrics. Key standardized program management reports shall also be developed which will provide a management dashboard for the core operational metrics for the WTC Health Program, including the per-member, per- month (PMPM) costs, the medical loss ratio, and other gauges of the financial health of the program and its overall efficiency and efficacy.

The contractor shall provide the following services related to analytics, decision support, and reporting:

- Scrub Data for completeness and accuracy
- Collect, Transform, and Load Analytics Data
- De-identify data for certain reports
- Develop Analytics Reports for NIOSH Use
- Replicate and transport complete and clean datasets that can be concatenated by unique member ID and CCE for certification (includes WTC Health Program-related health conditions and the health conditions medically associated with the WTC Health Program-related condition), medical (includes medical coding for condition and service/procedure /product, date of service and place of service) and pharmacy service utilization (includes plan cost paid and supply details) on a routine and timely basis to NIOSH.

The contractor shall manage all data to meet a security and privacy framework that supports FISMA requirements and can scale as necessary to support changes in statutes (HIPAA, HITECH, ACA) and associated regulatory and policy guidance.

The contractor shall collect, de-identify, and aggregate data from multiple sources (providers, PBM, etc.) into a data repository to allow NIOSH the ability to generate the necessary reports for analysis and research.

The contractor shall develop analytics reports for NIOSH use. Both analytical and ad hoc query data reports shall be designed to support NIOSH needs in WTC Health Program-related health effects/exposure analysis. The contractor shall provide additional support to NIOSH to support data management and research requirements arising from the program such as:

- Program reporting
- Reporting on program efficiency, efficacy, and program integrity

C.3.5.1 Records Management

The contractor shall develop, implement, and administer a comprehensive records management program. This program shall ensure that accurate WTC Health Program records are maintained in a manner such that the information is secure and readily accessible to be sent to NIOSH. This program shall be approved by the WTC Program Administrator and implemented by the contractor within 1 month of the approval. The contractor's Operations Manual shall provide the details of the records management program.

C.3.5.2 Data Warehouse

The contractor shall create and maintain a data warehouse of all member information, including certified health conditions, and claims for medical and pharmacy benefits provided. The data warehouse shall be compliant with Federal Information Security Management Act (FISMA) and Health Insurance Portability and Accountability Act (HIPAA) requirements.

The contractor data warehouse shall have the capability for ad-hoc reporting, data analyses, graphical data presentation, parameter-driven report generation, and standard reporting. In addition to standardized reports, users may design their own queries and reports.

The contractor's data warehouse shall be fully compliant with HIPAA and the data warehouse security shall incorporate the processes and procedures that ensure the required physical and operational security is in accordance with HIPAA, HITECH, FISMA, and other applicable security and privacy standards.

The contractor shall establish and maintain the data warehouse during the base year of the contract while the WTC Health Program is building its internal capacities that may alter where the contractor obtains source data in subsequent years. The contractor shall ensure continuity of operations that include claims adjudication and other services. The contractor will work closely with the program to tailor processes to establish the system of records as the program transitions data warehousing solutions.

C.3.5.3 HIPAA Compliance and Business Associate Agreement

The WTC Health Program, including any other NIOSH, CDC, or HHS components to the extent that they assist in administering the WTC Health Program involving protected health information (PHI), is a Covered Entity for purposes of the Health Insurance Portability and Accountability Act of 1996 (HIPAA). The contractor is a business associate of the WTC Health Program as that term is defined in the HIPAA Privacy, Security, Breach Notification, and Enforcement Rules (45 C.F.R. pts. 160, 162, and 164). To the extent that the contractor performs functions or activities on behalf of, or provides certain services to, the WTC Health Program where the contractor creates, receives, maintains, or transmits "protected health information" (PHI), the following "HIPAA provisions" apply: the Health Insurance Portability and Accountability Act of 1996 (HIPAA) (Pub. L. 104–191; 42 U.S.C. § 1320d); the Health Information Technology for Economic and Clinical Health (HITECH) Act (Pub. L. 111-5; 42 U.S.C. §§ 300jj et seq.); the HIPAA Privacy, Security, Breach Notification, and Enforcement Rules (45 C.F.R. pts. 160, 162, and 164); and HHS HIPAA policies.

The contractor shall require its providers and any and all agents and subcontractors to comply with all applicable provisions of HIPAA and shall document in writing the policies and procedures that will be used to meet such obligations. The contractor shall provide the WTC Program Administrator with a copy of their Notice of Use and Disclosures of Protected Health Information required by the HIPAA Privacy rule for review and approval and ensure in that notice that the members are aware the WTC Program Administrator will have access to the member's protected health information for purposes of the treatment provided by and administration of the WTC Health Program established under the James Zadroga 9/11 Health and Compensation Act. Furthermore, the contractor's Operations Manual shall provide the details of how the contractor will meet the obligations and activities of a Business Associate of the WTC Health Program.

At all times throughout the duration of this contract and until the contractor fulfills its obligations under this contract and the Business Associate Agreement, the contractor is subject to and shall comply with all applicable HIPAA provisions regarding business associates, as well as any updates to those provisions.

As a Business Associate of the WTC Health Program, the contractor is directly liable under HIPAA for the following violations:

- Impermissible uses and disclosures of PHI
- Failure to provide breach notification to the Covered Entity
- Failure to provide access to a copy of electronic PHI to either the Covered Entity, the individual, or the individual's designee, as specified in the Business Associate Agreement
- Failure to disclose PHI where required by the Secretary of HHS to investigate or determine the Business Associate's compliance with HIPAA
- Failure to provide an accounting of disclosures
- Failure to comply with the requirements of the Security Rule

C.3.5.4 Transfer of Data and Information

At the end of the contract or at a time determined by the WTC Program Administrator, the contractor shall transfer to the WTC Program Administrator or a subsequent contractor all data and information necessary to transition operations, including: medical records, data and reference tables; WTC-specific data, configurations, process documents, and specifications that are available at the completion of the contract; documentation relating to software and interfaces; functional business process flows; provider network data and agreements; and operational information, including correspondence, documentation of ongoing or outstanding issues, operations support documentation, and operational information regarding subcontractors. For purposes of this provision, "documentation" means all operations, technical and user manuals used in conjunction with the software, services and deliverables, in whole or in part, that the WTC Program Administrator determines are necessary to view and extract application data in a proper format. The contractor shall provide the documentation in the formats in which such documentation exists at the expiration or completion of the contract.

The contractor shall provide to the WTC Program Administrator all data, information, and services necessary and sufficient to enable the WTC Program Administrator to map all data from the contractor's system(s) to the replacement system(s) of the WTC Program Administrator or a successor contractor, including a comprehensive data dictionary that gives the names and definitions of all data fields in the database.

The contractor shall provide to the WTC Program Administrator all necessary data, information, and services in the format defined by the WTC Program Administrator, and must be HIPAA compliant. All of the data, information and services mentioned in this section shall be provided and performed in a manner by the contractor using its best efforts to ensure the efficient administration of the WTC Health Program. The data and information must be supplied in a medium and format specified by the WTC Program Administrator and according to the schedule approved by the WTC Program Administrator. All relevant data and information must be received and verified by the WTC Program Administrator. The contractor's Operations Manual shall provide the details of the transfer of data and information and turnover-of-services procedure.

C.3.6 Fraud, Waste, and Abuse Compliance Plan and Reporting

The contractor's Fraud, Waste, and Abuse Compliance Plan shall address how the contractor will mitigate, detect, respond to and prevent Fraud, Waste and Abuse (FWA) incidents.

1. Reporting requirements for the FWA Compliance Plan are as follows:
   The contractor shall notify the CO and COR within ten (10) business days of the discovery of

possible or actual FWA incidents related to covered services that warrant investigation.

a. If a possible FWA incident is determined not to be an actual incident, the contractor shall submit a final report to the CO and COR within ten (10) business days of that determination date but no later than forty (40) business days from the incident discovery date.

b. If a possible FWA incident is determined to be an actual case, the contractor shall provide a preliminary FWA report of their actions to the CO and COR within ten (10) business days of that determination. The preliminary report shall contain a brief synopsis of the situation and actions the contractor has taken and will take to address the matter, and to prevent it from recurring.

c. For an actual incident, the contractor shall provide a full report of their actions to the CO and COR within forty (40) business days of the determination date. Such report shall be reviewed and signed by the key staff or designee and contain at least the following information:

1) The name of the individual or entity that committed the fraud, waste, or abuse;
2) The source or surveillance method that identified the fraud, waste, or abuse;
3) The type of provider, entity or organization that committed the fraud, waste, or abuse;
4) A description of the fraud, waste, or abuse;
5) The approximate dollar amount of the fraud, waste, or abuse if applicable;
6) The legal and administrative disposition of the case including actions taken by law enforcement officials to whom the case has been referred; and
7) Other data/information deemed pertinent by the contractor or as prescribed by the WTC Program Administrator.

2. A monthly report shall include summaries of all open possible and actual FWA incidents and the status of the contractor's actions on each incident.

3. FWA requirements apply to the full scope of contract efforts/activities.

4. FWA definitions:

Fraud is defined as an intentional deception or misrepresentation made by a person with the knowledge that the deception could result in some unauthorized benefit to him or some other person. It includes any act that constitutes fraud under applicable Federal or State law.[1]

Examples include but are not limited to:

- Billing or filing a claim for services that were not rendered
- Altering claim forms to receive a higher payment rate
- Receiving services by nonmembers using a legitimate member's enrollment card[2]
- Prescribing medications for member conditions where the member is not certified for that particular condition

Waste is defined as failure to control costs or regulated payment associated with federal program funding. Furthermore, waste results in taxpayers not receiving reasonable value for their money in connection with any Government-funded activities due to an inappropriate act or omission by payers with control over or access to Government resources. Importantly, waste relates primarily to mismanagement, inappropriate actions, or inadequate oversight."[3]

Examples include but are not limited to:

- Use of higher-priced services that have negligible or no health benefits over less-

expensive alternatives
- Administrative complexity[4]
- Writing high-cost prescriptions over less expensive bnt equivalent generic prescriptions
- Failures of care delivery or of care coordination
- Overtreatment

Abuse is defined as practices that are inconsistent with professional standards of care; medical necessity; or sound fiscal, business, or medical practices.[5 6]

Examples included but are not limited to:
- Billing for services that were not medically necessary
- Charging excessively for services or supplies[7]
- Coding and billing for 99215 Office or other outpatient visit Evaluation and Management (E/M) for one hour when the visit was actually only 15 minutes.

Importantly, intent is the key distinction between Fraud and Abuse. An allegation of waste and abuse can escalate into a fraud investigation if a pattern of intent is determined.

C.3.7 Quality Assurance and Internal Audits

The contractor's administrative procedures shall include a Quality Assurance component. Responsibilities for Quality Assurance shall include all scope of work items including but not limited to:
- Program Management
- Member Services
- Claims Processing
- Data Management, Analytics, Decisiou Support, and Reporting
- Fraud Waste and Abuse Compliance Plan and Reporting

The contractor shall be responsible for developing and documenting its approach to Quality Assurance. The contractor shall perform all required services in accordance with their established Quality Assurance Program. The contractor shall develop a project-specific Quality Assurance Plan (QAP) which describes the Quality Assurance Program and the procedures which will be used to monitor and improve all project activities including reporting.

The g Quality Assurance Program shall include a customer satisfaction survey of the CCEs, the NPN contractor, and NIOSH to be conducted at least annually.

As a part of their Quality Assurance Program, the contractor shall conduct internal audits of each of the parts of the WTC Health Program listed below. The contractor shall submit a summary audit report to the WTC Program Administrator within 45 days of the end of each quarter. These audits shall include all scope of work items, including but not ~~be~~ limited to:

- Member Enrollment Processing
- Certification Processing
- Provider Enrollment
- Call Center Activities
- Claims Processing
- Data Management
- Project Management Activities

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[1] Definitions, 42 C.F.R. § 455.2 (2010). Retrieved August 20, 2014 from http://www.gpo.gov/fdsys/pkg/CFR-2010-title42-vol4/xml/CFR-2010-title42-vol4-sec455-2.xml

[2] U.S. Department of Health and Human Services, Centers for Medicare and Medicaid Services. (2013). *2013 national training program: Module 10: Medicare and Medicaid fraud prevention workbook.* Retrieved August 20, 2014 from

http://www.cms.gov/Outreach-and-Education/Training/CMSNationalTrainingProgram/Downloads/2013-Fraud-and-Abuse-Prevention-Workbook.pdf

[3] Walker, D. (2007). *Transforming government, management, and accountability*. (GAO-07-1214CG). Retrieved August 20, 2014 from http://www.gao.gov/cghome/d071214cg.pdf

[4] Lallemand, N. C. (2012). Health policy brief: Reducing waste in health care. *Health Affairs*. Retrieved August 20, 2014 from http://www.healthaffairs.org/healthpolicybriefs/brief.php?brief_id=82

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[6] U.S. Department of Health and Human Services, Centers for Medicare and Medicaid Services. (2013). *2013 national training program: Module 10: Medicare and Medicaid fraud prevention workbook*. Retrieved August 20, 2014 from http://www.cms.gov/Outreach-and-Education/Training/CMSNationalTrainingProgram/Downloads/2013-Fraud-and-Abuse-Prevention-Workbook.pdf

[7] U.S. Department of Health and Human Services, Centers for Medicare and Medicaid Services. (2012, November). Medicare fraud & abuse: prevention, detection, and reporting. (ICN 006827). Retrieved August 20, 2014 from http://www.cms.gov/Outreach-and-Education/Medicare-Learning-Network-MLN/MLNProducts/downloads/Fraud_and_Abuse.pdf

Contractor Metrics/Quality Assurance Surveillance Plan (QASP)

The contractor shall establish and continuously operate an internal quality management/quality improvement program covering every aspect of its operation, both clinically and administratively. The contractor shall provide a quarterly written report and briefing in person or via video teleconference to the COR and NIOSH staff on its ongoing internal quality improvement program. The contractor shall submit a draft Quality Assurance Surveillance Plan based on the following suggested metrics:

Performance Objectives	Performance Standard References	Performance Standard	Acceptable Quality Levels (AQL)	Methods of Surveillance	Incentives
1. Submit deliverables and monthly project management report of scheduled tasks, completed tasks, outstanding tasks, and any additional administrative reports	C.3.1 HPS Contract Management C.6.1 Deliverables Required by the PWS C.6.2 Monthly Reports C.6.3 Additional Administrative Reports	Submitted within 10 days after the end of the previous month	No more than 2 submissions are delivered greater than 10 - 20 days after the end of the previous month No submissions are delivered greater than 20 days after the end of the previous month	Document reviews, periodic and random inspections, observations, and customer complaints	Failure to meet 100% AQL in two consecutive months will require a corrective action plan and result in a lower performance rating
2. Develop and implement an on-going Quality Assurance Program (QAP) which includes conducting internal audits, management of subcontractors, resolving complaints, and conducting customer satisfaction surveys	C.3.1 HPS Contract Management C.3.1.7 Quality Assurance and Internal Audit Plan C.3.1.9 Subcontracting C.3.7 Quality Assurance and Internal Audits C.3.6 FWA Compliance Plan and Reporting	Submissions depict accurate findings All components of the QAP are addressed in each report with sufficient detail to provide a clear understanding of any corrective actions Submitted within 45 business days after the end of the previous quarter	No submissions are delivered greater than 45 days after the end of the previous quarter	Document reviews, periodic and random inspections, observations, and customer complaints	Failure to meet 100% AQL in two consecutive quarters will require a corrective action plan and result in a lower performance rating

Performance Objectives	Performance Standard References	Performance Standard	Acceptable Quality Levels (AQL)	Methods of Surveillance	Incentives
3. Ensure customer satisfaction (customer refers to NIOSH and NIOSH contractors)	C.3.1 HPS Contract Management C.3.2 Member Services and Medical Benefit Support	All customer complaints are reported monthly All valid customer complaints are resolved within a timely manner A quarterly satisfaction survey is conducted and reported as part of the Quarterly Quality Assurance Internal Audits	No more than 2 valid complaints per month or 3 valid complaints per quarter	Document reviews, periodic and random inspections, observations, and customer complaints	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating
4. Provide member services including eligibility, certification, and appeal processes	C.3.2 Member Services and Medical Benefits Support C.3.2.1 Medical Condition Certification C.6.1 Deliverables Required by the PWS	Work flow is sufficient to provide time-sensitive services to accommodate an average of 350 applications, 400 certification requests, and 25 appeals monthly	95% of all services are completed within the required time periods as defined in C.6.1, Deliverables Required by the PWS	Document reviews, periodic and random inspections, observations, and customer complaints	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating
5. Maintain a call center operation to receive and respond to inquiries from potential enrollees, members, and providers	C.3.3.4 Call Center	Work flow is sufficient to accommodate at least 100 calls per day on average in English and at least 15 'other languages' Use of a third-party translation and interpretation services must be available for 'other languages'	Volume shall never drop below 90% of thresholds Zero occurence of consecutive months in which the requirements are not met	Document reviews, periodic and random inspections, observations, and customer complaints Integration test with claim processing system, member database and application system	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating

Performance Objectives	Performance Standard References	Performance Standard	Acceptable Quality Levels (AQL)	Methods of Surveillance	Incentives
6. Establish and maintain a credentialed provider network	C.3.2 Member Services and Medical Benefits Support C.3.3 Healthcare Provider Network Management C.3.2.2 Case Management	Development and maintenance of a network of credentialed providers with access to specialties which are sufficient to treat WTCHP disease conditions	100% of provider availability to meet the needs of the Program monthly No more than 5 providers recruited monthly as a result of lack of provider coverage to meet the needs of the Program	Document reviews, periodic and random inspections, observations, and customer complaints Integration test with other automated systems Validation of providers network (to include: cancer care systems, medical and surgical specialists, ancillary support services, CMS-approved organ transplantation, emergency care, same-day surgery, inpatient facilities, and mental health services)	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating
7. Provide a comprehensive case management service specific to WTCHP members for either cancer or complex non-cancer conditions	C.3.2 Member Services and Medical Benefits Support C.3.2.2 Case Management	Case management services with after-hours nurse advice line ensuring quality and continuity of care	No more than 2 valid member complaints per month	Document reviews, periodic and random inspections, observations, and customer complaints	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating

Performance Objectives	Performance Standard References	Performance Standard	Acceptable Quality Levels (AQL)	Methods of Surveillance	Incentives
8. Provide an interactive and secure self-service portal for government users, case managers, other contractors and providers	C.3.3.2 Secure location for WTC Health Program Information Submission C.3.3.3 Secure Web-based Information Exchange System	The secure web portal must meet federal government and CDC's standards with zero deviation The portal must be highly configurable to address the unique needs of the WTCHP for federal users, providers, case managers, and other contractors	No more than 2 valid user complaints per month	Document reviews, periodic and random inspections, observations, and customer complaints Integration test Validation Audit report Program Administration Web Portal Test	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating.
9. Provide a comprehensive and secure claim processing, adjudication and reconciliation system through a web-based information exchange system	C.3.4 Claims Processing C.3.4.3 Medical Claims Adjudication C.3.4.4 Medical and Pharmacy Claims Payment C.3.4.5 Claims Reconciliation	Claims System shall accommodate at least 500,000 claims annually While, over 95% of claims are submitted electronically, there must be a capability to process paper claims within 5 days of receipt Adequate industry-standard information shall be provided for claims reconciliation (when requested by CCEs and the NPN contractor)	Volume shall never drop below 90% of thresholds Zero occurrence of consecutive months in which the requirements are not met	Document reviews, periodic and random inspections, observations, and customer complaints Integration test Validation Audit report	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating

Performance Objectives	Performance Standard References	Performance Standard	Acceptable Quality Levels (AQL)	Methods of Surveillance	Incentives
10. Develop a consolidated data repository for program reporting, along with an analytics and reporting capability	C.3.5 Data Management, Analytics, and Reporting C.3.5.1 Records Management C.3.5.2 Data Warehouse C.3.5.3 HIPAA Compliance C.3.5.4 Transfer of Data and Information C.6.3 Additional Administrative Reports Attachment #2 Information Technology Security and Privacy	Standardized, accurate, and secure database, clean datasets, analytics and reports are available and accessible for authorized users within time parameters established in a Data Access Plan Ensure data management meets federal security standards with scalable framework necessary to support changes in statutes nd associated regulatory and policy guidance	Zero occurrence of consecutive months in which the requirements are not met Zero occurrence in which timeliness requirements are more than 10 days late	Document reviews, periodic and random inspections, observations, and customer complaints Integration test Validation Audit report Analytical products, reports and program metrics Data System Security Plan Review Operations Manual that includes details of the records management program	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating
11. Implement Fraud, Waste, and Abuse Program according to federal law, regulations, policies and the FWA Compliance Plan, and report monthly	C.3.6 Fraud, Waste, and Abuse Compliance Plan and Reporting	All suspected, investigated and or confirmed FWA incidents are timely reported to NIOSH according to FWA guidance 100% of the time	100% of contractor's availability to meet the needs of the Program monthly	Document reviews, periodic and random inspections, observations, and customer complaints Validate monthly report, address approaches and changes to practice in quarterly report	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating
12. Operationalize the Coordination of Benefit and Recovery Program	C.3.4.6 Coordination of Benefit and Recovery	The claims processing and adjudication system shall process COB&R functions for all members under the survivor category	100% of contractor's availability to meet the needs of the Program monthly	Document reviews, periodic and random inspections, observations, and customer complaints Validate monthly report, address approaches and changes to practice in quarterly report	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating

C.4 Background and Experience

The contractor shall have experience in providing healthcare services of a type similar or greater in scope, size, and complexity to those required under the WTC Health Program. Relevant experience shall include providing all items in this PWS, including but not limited to:

- Program Management
- Member Services and Case Management
- Claims Processing
- Data Management, Analytics, Decision Support, and Reporting
- Fraud Waste and Abuse Compliance Plan and Reporting
- Quality Assurance and Internal Audits

C.5 Personnel

The contractor shall have personnel assigned to this project that have the experience, educational background, and record of past accomplishment appropriate to the scope of the effort. The contractor's Staffing Plan shall provide the qualifications and experience of the personnel who will perform duties under the contract and which services they will perform.

C.5.1 Key Personnel

The contractor shall list all personnel designated as key personnel for this contract along with their years of relevant experience and the functions they will perform on this contract. The key personnel shall not be removed from the contract or replaced without the prior approval of the CO.

At a minimum, key personnel shall include a Program Manager who is highly qualified and have experience in managing and administering similar scope and scale of the federal health program.

The Program Manager shall have overall responsibility for this contract and shall serve as the contractor's primary point of contact with the WTC Health Program on all issues, including non-medical issues. The contractor's Program Manager shall be highly knowledgeable of all aspects of the WTC Health Program. (Refer to HHSAR 352.273-75 Key Personnel.)

C.6 Deliverables

All deliverables required under this contract shall be packaged, marked, and shipped in accordance with the guidance found in this section. The contractor shall guarantee that all required materials shall be delivered in immediately usable and acceptable condition.

Reports submitted under the contract shall reference and cite the contract number and identify HHS as the sponsoring agency. All contract deliverables must be reviewed and approved by the CO and the COR. Satisfactory reporting performance under the contract shall be deemed to occur upon delivery and acceptance by the CO, or a duly authorized representative.

The data and analyses to be reported to the WTC Program Administrator shall include all information required to effectively administer and manage this effort. Minimum required data are listed below. The contractor shall provide a listing and short description of standard reports that will provide these data and analyses of these data, as well as any other data and analyses that the contractor considers to be valuable to the WTC Program Administrator. All reports shall be submitted monthly unless otherwise specified. Based on the contractor's best knowledge and belief, the contractor shall attest to the accuracy, completeness, and truthfulness of the reports being submitted. The contractor shall ensure that the WTC Program Administrator does not receive any personal data, including protected health information, on the members (names or other identifiers) in these reports.

The contractor shall base their proposed reporting formats and content on their experience with other similar programs. The contractor shall analyze the data collected during each reporting period and ensure

that all reports provide summaries which highlight significant trends. All reports shall include a narrative companion text document that summarizes the report data and explains any unusual occurrences or trends.

C.6.1 Deliverables Required by the PWS

PWS J-1 Reference	Deliverable Title	Date Due
C.3.1	Project Management Plan	Draft with proposal; finalized version within 30 days of contract award; updates to be reported in the Monthly Reports.
C.3.1	Operations Manual	Draft with proposal; finalized version within 60 days of contract award; updates to be reported in the Monthly Reports. Substantial re-write of this manual to be provided annually.
C.3.1	Communications Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates to be reported in the Monthly Reports. Substantial re-write of this plan to be provided annually.
C.3.1	Data Security and Continuity of Operations Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates to be reported in the Monthly Reports. Substantial re-write of this plan to be provided annually.
C.3.1	Risk Management Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates to be reported in the Monthly Reports. Substantial re-write of this plan to be provided annually.
C.3.1	Fraud, Waste, and Abuse Compliance Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates to be reported in the Monthly Reports. Substantial re-write of this plan to be provided annually.
C.3.1	Quality Assurance and Internal Audit Plan	Draft with proposal; finalized version within 60 days of contract award; updates to be reported in the Monthly Reports. Substantial re-write of this plan to be provided annually.
C.3.1	WTC Health Program Reconciliation Report on the overall Financial Status of the contract.	The 10th day of the month for the previous quarter's information
C.3.1	Subcontracting	Draft prior to award
C.3.1	Program Improvement Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates monthly
C.3.4	Coordination of Benefit and Recovery Plan	Draft with proposal; finalized version within 60 days of contract award; updates monthly
C.3.7	Quality Assurance and Internal Audits	Summary report due 45 days after the end of each quarter
C.3.2.3	Transition Plan for the Medical Claims Adjudication Process with the Incumbent Contractor	Draft with proposal; finalized version within 30 days of contract award; updates to be reported in the Monthly Reports.
C.6.2	Monthly Reports	The 10th day of the month for the previous month's information
-	Annual Reports	The 10th day of the month following the contract year end
C.6.3	Additional Administrative Reports	As requested
IT J-2 Reference	**Deliverable Title**	**Date Due**
7.3	PII Security Plan	Draft due with proposal; finalized version due within 45 days of contract award
7.3.3	PTA/PIA	Due within 30 days of contract award
17.3-17.5	FIPS 199; SA&A; SSP; SAR; POA&M; Contingency Plan	Due within 45 days of contract award

C.6.2 Monthly Reports

The contractor shall develop and submit Monthly Reports to the COR and CO. The Monthly Reports shall include a summary of activities for the month, issues, and goals for the next month. The Monthly Reports shall summarize the contractor's accomplishments during the previous month, planned work during the upcoming month and any significant problems or issues requiring resolution, along with proposed corrective actions. It shall also include a description of any substantive changes to any manuals and plans detailed in C.3. The Monthly Reports shall include all scope of work items including but not limited to, the following information:

- Contract Number
- Program Management Operations and Activities
- Member Services Support including case management services
- Call Center Operations
- Medical Benefits Support
- Provider Network Support
- Claims Processing Activities
- IT Architecture, Security, and Implementation Testing
- Fraud, Waste, and Abuse Activities
- Quality Assurance Activities
- Cost Alerts

Format

Except as otherwise specified herein, the contractor shall prepare and submit to NIOSH the reports required under this section in an agreed-upon media format on the 10th day of the month for the monthly reports. All reports including protected health information or other personal data of members must be securely transmitted in compliance with HIPAA and CDC, NIOSH, and WTC Health Program protocols.

C.6.3 Additional Administrative Reports

Upon request by the CO, the contractor shall prepare and submit additional administrative reports. The contractor shall prepare reports to track applicants, members, certifications, and claims; review paid, pended, and denied claims; and conduct utilization review for quality assurance and appropriate health care. The contractor's reporting capabilities shall be based on the entire set of WTC Health Program data including, but not limited to, members' demographic and enrollment information, members' certification information, medical claims information, pharmacy claims information, and provider data. Such requests will be limited to situations in which the desired data is considered essential and cannot be obtained through existing reports. Whenever possible, the contractor will be provided with a 90-day notice and the opportunity to discuss and comment on the proposed requirements before work is begun. However, the COR on behalf of the Program Administrator reserves the right to give a 5-day notice in circumstances where time is of the essence. The contractor shall prepare and submit to the CO and Program Administrator, through the COR, any other data reports in e-mail format.

The CO may extend due dates or modify report requirements or formats upon a written request by the contractor to the CO, where the contractor has demonstrated a good and compelling reason for the extension or modification. The determination to grant a modification or extension of time shall be made by the CO.

C.7 Period of Performance

The anticipated Period of Performance for this requirement is:

Base Period:	November 15, 2016 – May 14, 2017
Option Period 1:	May 15, 2017 – May 14, 2018
Option Period 2:	May 15, 2018 – May 14, 2019
Option Period 3:	May 15, 2019 – May 14, 2020
Option Period 4:	May 15, 2020 – May 14, 2021

AWARD/CONTRACT

1. THIS CONTRACT IS A RATED ORDER UNDER DPAS (15 CFR 350)	RATING	PAGE OF PAGES
▶		1 | 42

2. CONTRACT (Proc. Inst. Ident.) NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQUEST/PROJECT NO.
200-2016-90996	11/28/2016	0000HCCP-2017-03654

5. ISSUED BY	CODE	6. ADMINISTERED BY (If other than Item 5)	CODE
Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070	3635	Ctrs for Disease Control & Prevention PGH Office of Acquisition Services, Branch 4 PO Box 18070, 626 Cochrans Mill Rd Pittsburgh, PA 15236-0070 Approved as to Form and Legality: _____	3635

7. NAME AND ADDRESS OF CONTRACTOR (No., street, county, State and ZIP Code)	8. DELIVERY	9. DISCOUNT FOR PROMPT PAYMENT	10. SUBMIT INVOICES (4 copies unless otherwise specified) TO THE ADDRESS SHOWN IN:	ITEM
KARNA LLC 2987 CLAIRMONT RD STE 205 ATLANTA, GA 30329-1687	☐ FOB ORIGIN ☒ OTHER (See below)	Net 30	▶	

CODE	828268503	FACILITY CODE	

11. SHIP TO/MARK FOR	CODE	12. PAYMENT WILL BE MADE BY	CODE
		Centers for Disease Control and Prevention (FMO) PO Box 15580 404-718-8100 Atlanta, GA 30333-0080	434

13. AUTHORITY FOR OTHER THAN FULL AND OPEN COMPETITION:	14. ACCOUNTING AND APPROPRIATION DATA
☐ 10 U.S.C. 2304 (c)( ) ☐ 41 U.S.C. 253 (c)( )	939ZNJH 2512 2017 75-X-0946 W614111601 Total Funded (b)(4)

15A. ITEM NO.	15B. SUPPLIES/SERVICES	15C. QUANTITY	15D. UNIT	15E. UNIT PRICE	15F. AMOUNT
	"See Continuation Page"				

Total Value of Contract including all unexercised option periods (54 months) 15G. TOTAL AMOUNT OF CONTRACT ➔ (b)(4)

16. TABLE OF CONTENTS

(√)	SEC.	DESCRIPTION	PAGE(S)	(√)	SEC.	DESCRIPTION	PAGE(S)
		PART I – THE SCHEDULE				PART II – CONTRACT CLAUSES	
X	A	SOLICITATION/CONTRACT FORM	1	X	I	CONTRACT CLAUSES	32
X	B	SUPPLIES OR SERVICES AND PRICES/COSTS	2			PART III - LIST OF DOCUMENTS, EXHIBITS AND OTHER ATTACH.	
X	C	DESCRIPTION/SPECS./WORK STATEMENT	12	X	J	LIST OF ATTACHMENTS	42
X	D	PACKAGING AND MARKING	13			PART IV - REPRESENTATIONS AND INSTRUCTIONS	
X	E	INSPECTION AND ACCEPTANCE	14		K	REPRESENTATIONS, CERTIFICATIONS AND OTHER STATEMENTS OF CONTRACTORS	
X	F	DELIVERIES OR PERFORMANCE	15				
X	G	CONTRACT ADMINISTRATION DATA	18		L	INSTRS., CONDS., AND NOTICES TO	
X	H	SPECIAL CONTRACT REQUIREMENTS	25		M	EVALUATION FACTORS FOR AWARD	

CONTRACTING OFFICER WILL COMPLETE ITEM 17 OR 18 AS APPLICABLE

17. ☒ CONTRACTOR'S NEGOTIATED AGREEMENT (Contractor is required to sign this document and return 1 copies to issuing office.)
Contractor agrees to furnish and deliver all items or perform all the services set forth or otherwise identified above and on any continuation sheets for the consideration stated herein. The rights and obligations of the parties to this contract shall be subject to and governed by the following documents: (a) this award/contract, (b) the solicitation, if any, and (c) such provisions, representations, certifications, and specifications, as are attached or incorporated by reference herein. (Attachments are listed herein.)

18. ☐ AWARD (Contractor is not required to sign this document.)
Your offer on Solicitation Number 2016-N-17955 including the additions or changes made by you which additions or changes are set forth in full above, is hereby accepted as to the items listed above and on any continuation sheets. This award consummates the contract which consists of the following documents: (a) the Government's solicitation and your offer, and (b) this award/contract. No further contractual document is necessary.

19A. NAME AND TITLE OF SIGNER (Type or print)	20A. NAME OF CONTRACTING OFFICER
AT	Abbie Jemmott

19B. NAME OF CONTRACTOR	19C. DATE SIGNED	20B. UNITED STATES OF AMERICA	20C. DATE SIGNED
B (b)(6) (Signature of person authorized to sign)	11/30/2016	BY Abbie G. Jemmott -S (Signature of Contracting Officer) Digitally signed by Abbie G. Jemmott -S Date: 2016.11.30 15:36:32 -05'00'	

Section B - Supplies Or Services and Prices/Costs

Base Year Items:

ITEM	SUPPLIES / SERVICES	EST. COST	FIXED FEE	TOTAL EST. CPFF
0001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 12/01/2016 - 05/31/2017 **Line Of Accounting:** **939ZNJH 2512 2017 75-X-0946** **W614111601** (b)(4)	(b)(4)		
0002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 12/01/2016 - 05/31/2017 **Line Of Accounting:** **939ZNJH 2512 2017 75-X-0946** **W614111601** (b)(4)	(b)(4)		
0003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 12/01/2016 - 05/31/2017 **Line Of Accounting:** **939ZNJH 2512 2017 75-X-0946** **W614111601** (b)(4)	(b)(4)		

0004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 12/01/2016 - 05/31/2017 **Line Of Accounting:** **939ZNJH 2512 2017 75-X-0946** **W614111601** (b)(4)	(b)(4)
0005	Finance and Accounting Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 12/01/2016 - 05/31/2017 **Line Of Accounting:** **939ZNJH 2512 2017 75-X-0946** **W614111601** (b)(4)	(b)(4)
0006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 12/01/2016 - 05/31/2017 **Line Of Accounting:** **939ZNJH 2512 2017 75-X-0946** **W614111601** (b)(4)	(b)(4)

ITEM	SUPPLIES / SERVICES	UNIT	EXTENDED PRICE
0007	Call Center All work to be performed in accordance with the Performance Work Statement. This is a Firm Fixed Price CLIN. Period of Performance: 12/01/2016 - 05/31/2017 **Line Of Accounting:** **939ZNJH 2512 2017 75-X-0946** **W614111601** (b)(4)	1 Each	(b)(4)

Option Period 1 Items:

ITEM	SUPPLIES / SERVICES	EST. COST	FIXED FEE	TOTAL EST. CPFF
1001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 1 Period of Performance: 06/01/2017 - 05/31/2018	(b)(4)		
1002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 1 Period of Performance: 06/01/2017 - 05/31/2018	(b)(4)		
1003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 1 Period of Performance: 06/01/2017 - 05/31/2018	(b)(4)		
1004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 1 Period of Performance: 06/01/2017 - 05/31/2018	(b)(4)		
1005	Finance and Accounting All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 1 Period of Performance: 06/01/2017 - 05/31/2018	(b)(4)		

1006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 1 Period of Performance: 06/01/2017 - 05/31/2018	(b)(4)

ITEM	SUPPLIES / SERVICES	UNIT	EXTENDED PRICE
1007	Call Center All work to be performed in accordance with the Performance Work Statement. This is a Firm Fixed Price CLIN. Option Period 1 Period of Performance: 06/01/2017 - 05/31/2018	1 Each	(b)(4)

Option Period 2 Items:

ITEM	SUPPLIES / SERVICES	EST. COST	FIXED FEE	TOTAL EST. CPFF
2001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 2 Period of Performance: 06/01/2018 - 05/31/2019	(b)(4)		
2002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 2 Period of Performance: 06/01/2018 - 05/31/2019	(b)(4)		

2003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 2 Period of Performance: 06/01/2018 - 05/31/2019	(b)(4)
2004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 2 Period of Performance: 06/01/2018 - 05/31/2019	(b)(4)
2005	Finance and Accounting Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 2 Period of Performance: 06/01/2018 - 05/31/2019	(b)(4)
2006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 2 Period of Performance: 06/01/2018 - 05/31/2019	(b)(4)

ITEM	SUPPLIES / SERVICES	UNIT	EXTENDED PRICE
2007	Call Center All work to be performed in accordance with the Performance Work Statement. This is a Firm Fixed Price CLIN. Option Period 2 Period of Performance: 06/01/2018 - 05/31/2019	1 Each	(b)(4)

Option Period 3 Items:

ITEM	SUPPLIES / SERVICES	EST. COST	FIXED FEE	TOTAL EST. CPFF
3001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 3 Period of Performance: 06/01/2019 - 05/31/2020	(b)(4)		
3002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 3 Period of Performance: 06/01/2019 - 05/31/2020	(b)(4)		
3003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 3 Period of Performance: 06/01/2019 - 05/31/2020	(b)(4)		
3004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 3 Period of Performance: 06/01/2019 - 05/31/2020	(b)(4)		
3005	Finance and Accounting Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 3 Period of Performance: 06/01/2019 - 05/31/2020	(b)(4)		

3006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 3 Period of Performance: 06/01/2019 - 05/31/2020	(b)(4)

ITEM	SUPPLIES / SERVICES	UNIT	EXTENDED PRICE
3007	Call Center All work to be performed in accordance with the Performance Work Statement. This is a Firm Fixed Price CLIN. Option Period 3 Period of Performance: 06/01/2019 - 05/31/2020	1 Each	(b)(4)

Option Period 4 Items:

ITEM	SUPPLIES / SERVICES	EST. COST	FIXED FEE	TOTAL EST. CPFF
4001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 4 Period of Performance: 06/01/2020 - 05/31/2021	(b)(4)		
4002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 4 Period of Performance: 06/01/2020 - 05/31/2021	(b)(4)		

4003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 4 Period of Performance: 06/01/2020 - 05/31/2021	(b)(4)
4004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 4 Period of Performance: 06/01/2020 - 05/31/2021	(b)(4)
4005	Finance and Accounting Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 4 Period of Performance: 06/01/2020 - 05/31/2021	(b)(4)
4006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 4 Period of Performance: 06/01/2020 - 05/31/2021	(b)(4)

ITEM	SUPPLIES / SERVICES	UNIT	EXTENDED PRICE
4007	Call Center All work to be performed in accordance with the Performance Work Statement. This is a Firm Fixed Price CLIN. Option Period 4 Period of Performance: 06/01/2020 - 05/31/2021	1 Each	(b)(4)

TOTAL CONTRACT COST ALL CPFF & FFP CLINS $64,913,896.11

B.1 DETAILED CLIN DESCRIPTIONS

CLIN 1: Program Support/Data Management
The contractor shall manage the health care network for the Clinical Centers of Excellence in the New York/New Jersey area and to process the monitoring, medical treatment and pharmacy claims for the entire program. The contractor shall be responsible for maintaining a data warehouse that includes all member information and claims data.

CLIN 2: Member Services Benefits Management
The contractor shall be responsible for receiving and recording the certifications of all members. Based on the certifications, the contractor shall adjudicate claims, record and respond to requests for new codes for treatment, and submit requests to NIOSH for pricing exceptions for care if codes submitted are not covered by FECA, DRG or other approved pricing mechanisms. The contractor shall communicate to WTCHP members based on the Program's direction. Case management actvities will be conducted under this CLIN.

CLIN 3: Technical Architecture
The contractor shall be responsible for establishing and maintaining the IT infrastructure necessary for secure data storage and transmission that meets FISMA standards. Technical architecture activities are largely involved with claims processing system, data transfer and IT security.

CLIN 4: Software Architecture
The contractor shall maintain all software systems and licenses required for the secured collection and processing of program information, member information, claims, code books and formularies, and the provider network. Where necessary, the contractor shall modify software to meet the program needs. Solutions architecture activities are largely data submission and warehousing solutions referred to in PWS sections C.3.1.2 and C.3.3.2.

CLIN 5: Finance and Accounting Management
The contractor shall be responsible for tracking, analyzing and controlling costs of the Health Care Program. The contractor shall determine areas of cost savings (whenever possible) and notify the Program of trade-offs prior to implementation. The contractor shall provide a complete array of claims related services including claims adjudication for all types of standard claims, submission of claim files and vendor files to CMS for payment of claims to vendors, claims information to assist CCEs/NPN in knowing the status of claims and in dealing with denied claims, claims review and evaluation to help resolve denials and appeals, and adjustments to payments along with documentation of the reasons for the adjustments.

CLIN 6: Claims Adjudication Management
The contractor shall provide a complete array of claims-related services including claims receipt, claims adjudication, submitting claims to another entity for payment, and assistance with claims reconciliation. The contractor shall assess and provide a high volume of technical experts that may be necessary during the transition phase to be operationalized within a timely manner. The contractor shall work closely with the WTC Health Program, Centers for Medicare Medicaid Services, the Clinical Centers of Excellence, and the Nationwide Provider Network Contractor and other stakeholders of the program to ensure interoperability of the current, diverse and unique systems being used by the incumbent contractors to avoid major development work; enhance capacities and capabilities; and minimize cost while maximizing efficiency.

CLIN 7: Call Center
The contractor shall maintain a call center to receive and respond to inquiries from potential enrollees, members, and providers. The contractor shall also provide escalation as needed for properly responding to inquiries.

B.2 LABORY CATEGORY CROSSWALK TO CLINS

Possible Labor Categories	CLIN support
Management, Project Lead	0001
Management, Team Lead	0001
QA/QC Specialist	0002
Medical Director / Clinical Director	0002
Case Manager	0002
System Administrator	0003
System Analyst	0003
Enterprise System Architect	0003
Computer Programmer	0003
Business Process Consultant II	0004
Business Process Consultant III	0004
Financial Professional I	0005
Financial Professional II	0005
Financial Professional III	0005
Adjudicator	0006
Claims processor	0006
QA/QC Specialist	0006
User Support Specialist I	0007
User Support Specialist II	0007
User Support Specialist III	0007

B.3 TYPE OF CONTRACT

This contract is a Cost-Plus-Fixed-Fee type completion form contract with one Fixed Price CLIN as defined in Federal Acquisition Regulation Section 16.306 and 16.202 respectively.

Section C - Description/Specification/Work Statement

C.1 PERFORMANCE OF INHERENTLY GOVERNMENTAL, CRITICAL, OR OTHER FUNCTIONS

"The services contemplated under this contract are considered to be an Other Function as defined by the Office of Federal Procurement Policy in OFPP letter 11-01 and by HHS in Acquisition Policy Memorandum 2012-01."

Please see Section J for the Performance Work Statement for the World Trade Center Health Program, Health Program Support.

Section D - Packaging And Marking

There are no clauses/provisions included in this section.

Section E - Inspection And Acceptance

FAR SOURCE	TITLE AND DATE
52.246-4 (applies to all FFP CLINs)	Inspection of Services – Fixed Price (Aug 1996)
52.246-5 (applies to all CPFF CLINs)	Inspection of Services – Cost Reimbursement (Apr 1984)

E.1 INSPECTION AND ACCEPTANCE (JUL 1999)

Inspection and acceptance of the articles, services, and documentation called for herein shall be accomplished by the Contracting Officer, or his duly authorized representative (who for the purposes of this contract shall be the Project Officer) at the destination of the articles, services or documentation.

(End of Clause)

Section F - Deliveries Or Performance

FAR SOURCE	TITLE AND DATE
52.242-15	Stop-Work Order (Aug 1989)

F.1 PERIOD OF PERFORMANCE (JUL 1999 REVISED)

The period of performance for this contract is as follows:

Base Period:	December 01, 2016 – May 31, 2017
Option Period 1:	June 01, 2017 – May 31, 2018
Option Period 2:	June 01, 2018 – May 31, 2019
Option Period 3:	June 01, 2019 – May 31, 2020
Option Period 4:	June 01, 2020 – May 31, 2021

(End of Clause)

F.2 DELIVERABLE DOCUMENTATION (JUL 1999 REVISED)

(a) Documentation
The following is a list of required documentation to be delivered to the Government as a part of contract performance.

PWS J-1 Reference	Deliverable Title	Date Due
C.3.1	Project Management Plan	Draft with proposal; finalized version within 30 days of contract award; updates weekly
C.3.1	Operations Manual	Draft with proposal; finalized version within 60 days of contract award; updates monthly
C.3.1	Communications Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates monthly
C.3.1	Data Security and Continuity of Operations Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates monthly
C.3.1	Risk Management Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates monthly
C.3.1	Fraud, Waste, and Abuse Compliance Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates monthly
C.3.1	Quality Assurance and Internal Audit Plan	Draft with proposal; finalized version within 60 days of contract award; updates monthly
C.3.1	WTC Health Program Reconciliation Report on the overall Financial Status of the contract	The 10th day of the month for the previous quarter's information
C.3.1	Program Improvement Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates monthly
C.3.4	Coordination of Benefit and Recovery Plan	Draft with proposal; finalized version within 60 days of contract award; updates monthly
C.3.7	Quality Assurance and Internal Audits	Summary report due 45 days after the end of each quarter
C.3.2.3	Transition Plan for the Medical Claims Adjudication Process with the Incumbent Contractor	Draft with proposal; finalized version within 30 days of contract award; updates to be reported in the Monthly Reports.

C.6.2	Monthly Reports	The 10th day of the month for the previous month's information
-	Annual Reports	The 10th day of the month following the contract year end
C.6.3	Additional Administrative Reports	As requested
IT J-2 Reference	**Deliverable Title**	**Date Due**
7.3	PII Security Plan	Draft due with proposal; finalized version due within 45 days of contract award
7.3.3	PTA/PIA	Due within 30 days of contract award
17.3-17.5	FIPS 199; SA&A; SSP; SAR; POA&M; Contingency Plan	Due within 45 days of contract award

(1) **Monthly Technical/Status Report**

(i) **Quantity**: One (1) electronic copy per month.
(ii) **Due Date**: On or before ten (10) days after the end of each month.

(b) Report Content

(1) Monthly Technical/Status Report

The Contractor shall submit monthly status reports on all activities performed for the WTC Health Program. The status reports shall provide sufficient detail for the WTC Program Administrator to track all significant program activities and progress. The reports shall summarize the activities and accomplishments of the past month (including the associated cost for these services invoiced to the government) and identification of any potential risks to continued program success (along with the plans to mitigate the risk).

The Contractor shall develop and submit Monthly Reports to the Contracting Officer's Representative (COR) and Contracting Officer (CO). The Monthly Reports shall include a summary of activities for the month, issues, and goals for the next month. The Monthly Reports shall summarize the Contractor's accomplishments during the previous month, planned work during the upcoming month and any significant problems or issues requiring resolution, along with proposed corrective actions. The Monthly Reports shall include, at a minimum, the following sections:

1. Operations Manual and Project Management Plan (including updates and revisions)
2. Complaints
3. HIPAA compliance: Any violation in use or disclosure involving PHI, any security incident, and any breaches involving unsecured PHI
4. Fraud Waste and Abuse Report: Cases investigated including resolution
5. Cost Alerts: The Contractor must advise the CO and COR when 75% of the approved budget has been reached.
6. Budget projections to include burn rate of current obligations.

Upon request by the CO, the Contractor shall prepare and submit other operational data reports. Such requests will be limited to situations in which the desired data is considered essential and cannot be obtained through existing reports. Whenever possible, the Contractor will be provided with ninety (90) days' notice and the opportunity to discuss and comment on the proposed requirements before work is begun. However, the COR on behalf of the Program Administrator reserves the right to give five (5) days' notice in circumstances where time is of the essence. The Contractor shall prepare and submit to the CO and Program Administrator, through the COR, the other operational data reports in e-mail format.

Format - Except as otherwise specified herein, the Contractor shall prepare and submit to NIOSH the reports required under this section in an agreed media format on the 10th day of the month for the monthly reports.

The CO may extend due dates or modify report requirements or formats upon a written request by the Contractor to the CO, where the Contractor has demonstrated a good and compelling reason for the extension or modification. The determination to grant a modification or extension of time shall be made by the CO.

(c) Report Approvals

(1) Draft Reports

The Government shall be allowed **fourteen (14) days** to review the draft and notify the Contractor in writing of approval or of recommended changes to be made in the final copy. If the Government does not approve or recommend changes within **fourteen (14) days** of receipt of the draft report, the report shall be deemed **approved.**

(2) Final Reports

Within **fourteen (14) days** of receipt of a notice of approval from the **Government** of any contract related document, the Contractor shall furnish the Government with 508 compliant Portable Document Format (PDF) file versions of the reports and additionally a reproducible master hard copy of the instructor's guides in final form.

(d) Reporting Period

(1) All monthly reporting periods shall end on the last day of a calendar month.

For contracts awarded (i.e., effective date) on or before the fifteenth (15th) of a calendar month, the initial reporting period shall end on the last day of the calendar month during which the contract became effective.

For contracts awarded (i.e., effective date) on or after the sixteenth (16th) of a calendar month, the initial reporting period shall end on the last day of the month immediately following the month during which the contract became effective.

(2) All quarterly, Semi-Annually, and annual reporting periods shall end on the last day of the calendar month of the previous quarter.

(e) Delivery of Reports

The contractor shall submit 1 hard copy and 1 electronic copy of each of the reports, unless designated otherwise, as required by the contract to:

Centers for Disease Control and Prevention
Attn: Abbie Jemmott, Contracting Officer
Office of Acquisition Services
626 Cochrans Mill Road
Pittsburgh, PA 15236

Abbie Jemmott – ygf3@cdc.gov

All remaining copies of deliverables/reports shall be delivered as an email attachment concurrent with shipping hard copies, prepaid, as required by the contract to the Contracting Officer's Representative (COR) to invoice_deliverable@cdc.gov .

(End of Clause)

Section G - Contract Administration Data

G.1 CONTRACTING OFFICER (JUL 1999)

(a) The Contracting Officer is the only individual who can legally commit the Government to the expenditnre of pnblic fnnds. No person other than the Contracting Officer can make any changes to the terms, conditions, general provisions, or other stipulations of this contract.

(b) No information, other than that which may be contained in an authorized modification to this contract, duly issued by the Contracting Officer, which may be received from any person employed by the United States Government, or otherwise, shall be considered grounds for deviation from any stipulation of this contract.

(End of Clause)

G.2 CONTRACTING OFFICER'S REPRESENTATIVE (COR) TECHNICAL GUIDANCE (SEP 2009 REVISED)

Performance of the work hereunder shall be subject to the technical directions of the designated COTR for this contract. **For purposes of this contract, any reference to a Project Officer (PO) or Contracting Officer's Technical Representative (COTR), shall mean the COR.**

As used herein, technical directions are directions to the Contractor which fill in details, suggests possible lines of inquiry, or otherwise completes the general scope of work set forth herein. These technical directions must be within the general scope of work, and may not alter the scope of work or cause changes of such a nature as to justify an adjustment in the stated contract price/cost, or any stated limitation thereof. In the event that the Contractor feels that full implementation of any of these directions may exceed the scope of the contract, he or she shall notify the originator of the technical direction and the Contracting Officer in a letter separate of any required report(s) within two (2) weeks of the date of receipt of the technical direction and no action shall be taken pursuant to the direction. If the Contractor fails to provide the required notification within the said two (2) week period that any technical direction exceeds the scope of the contract, then it shall be deemed for purposes of this contract that the technical direction was within the scope. No technical direction, nor its fulfillment, shall alter or abrogate the rights and obligations fixed in this contract.

The Government **COR** is not authorized to change any of the terms and conditions of this contract. Changes shall be made only by the Contracting Officer by properly written modification(s) to the contract.

The Government will provide the Contractor with a copy of the delegation memorandum for the **COR**. Any changes in **COR** delegation will be made by the Contracting Officer in writing with a copy being furnished to the Contractor.

(End of Clause)

G.3 CONTRACT COMMUNICATIONS/CORRESPONDENCE (JUL 1999)

The Contractor shall identify all correspondence, reports, and other data pertinent to this contract by imprinting thereon the contract number from Page 1 of the contract.

(End of Clause)

G.4 PAYMENT BY ELECTRONIC FUNDS TRANSFER (DEC 2005) (REVISED)

(a) The Government shall use electronic funds transfer to the maximum extent possible when making payments under this contract. FAR 52.232-33, Payment by Electronic Funds Transfer -- **System For Award Management**, in

Section I, requires the contractor to designate in writing a financial institution for receipt of electronic funds transfer payments.

(b) In addition to Central Contractor Registration, the contractor shall make the designation by submitting the form titled "ACH Contractor/Miscellaneous Payment Enrollment Form" to the address indicated below. **Note:** The form is either attached to this contract (see Section J, List of Attachments) or may be obtained by contacting the Contracting Officer or the CDC Financial Management Office at **(404)-718-8100**.

(c) In cases where the contractor has previously provided such designation, i.e., pursuant to a prior contract/order, and been enrolled in the program, the form is not required unless the designated financial institution has changed.

(d) The completed form shall be mailed after award, but no later than 14 days before an invoice is submitted, to the following address:

The Centers for Disease Control and Prevention
Financial Management Office (FMO)
P.O. Box 15580
Atlanta, GA 30333
Or – Fax copy to: 404-638-5342

(End of Clause)

G.5 PAYMENT OF FIXED FEE (JAN 2000)

The fixed fee for each performance period stated in the contract will be paid in equal monthly or bi-monthly amounts up to 85% of the fee for that period. In accordance with FAR 52.216-8, Fixed Fee, incorporated by reference in Section I, the Contracting Officer may withhold the final 15% of the fee for each period, or $100,000 whichever is less, until audited final rates are determined or until a closeout agreement is negotiated.

(End of Clause)

G.6 NON-PERSONAL SERVICES (APRIL 2013)

(a) Personal services shall not be performed under this contract. Although the Government may provide sporadic or occasional instructions within the scope of the contract, the Contractor is responsible for control and supervision of its employees. If the Contractor (including its employees) believes any Government action or communication has been given that would create a personal services relationship between the Government and any Contractor employee, the Contractor shall promptly notify the Contracting Officer of this communication or action.

(b) The contractor shall comply with, and ensure their employees and subcontractors comply with, CDC Policy titled "Identification of Contractors' Employees and Safeguarding Government Information." No Contractor employee shall hold him or herself out to be a Government employee, agent, or representative. No Contractor employee shall state orally or in writing at any time that he or she is acting on behalf of the Government. In all communications with third parties in connection with this contract, Contractor employees shall identify themselves as Contractor employees and specify the name of the company for which they work. . The contractor is limited to performing the services identified in the contract statement of work and shall not interpret any communication with anyone as a permissible change in contract scope or as authorization to perform work not described in the contract. All contract changes will be incorporated by a modification signed by the Contracting Officer.

(c) The Contractor shall ensure that all of its employees and subcontractor employees working on this contract are informed of the substance of this clause. The Contractor agrees that this is a non-personal services contract; and that for all the purposes of the contract, the Contractor is not, nor shall it hold itself out to be an agent or partner of, or joint venture with, the Government. The Contractor shall notify its employees that they shall neither supervise nor accept supervision from Government employees. The substance of this clause shall be included in all subcontracts at any tier.

(d) Nothing in this clause shall limit the Government's rights in any way under any other provision of the contract, including those related to the Government's right to inspect and accept or reject the services performed under this contract.

(End of Clause)

G.7 BILLING INSTRUCTIONS FOR NEGOTIATED COST-TYPE CONTRACTS (MAR 2006)

Centers for Disease Control and Prevention

Introduction

Reimbursement procedures related to negotiated cost-type contracts require that Contractors submit to the Government adequately prepared claims. The instructions that follow are provided for Contractors' use in the preparation and submission of invoices or vouchers requesting reimbursement for work performed. The preparation of invoices or vouchers as outlined below will aid in the review and approval of claims and enable prompt payment to the Contractor.

1. Forms to Be Used

In requesting reimbursement, Contractors may use the regular Government voucher form, Standard Form 1034, "Public Voucher for Purchases and Services Other Than Personal," and Standard Form 1035, "Continuation Sheet," or the Contractor's own invoice form. If the Contractor desires to use the Government's standard forms, a request for the forms should be submitted to the Contracting Officer. If the Contractor uses his own invoice, the billing must conform with the instructions set forth herein.

2. Submission of Invoices or Vouchers

Invoices or vouchers shall be submitted per Section G of the contract. All original invoices or vouchers shall also be submitted via email to the following:

Financial Management Office (FMO): FMOAPINV@CDC.GOV
Contracting Officer: ygf3@cdc.gov
Contracting Officer's Representative: To Be Determined (TBD)

REMINDER: *Failure to submit vouchers in the proper format will delay your payment.*

3. Preparation of Invoices or Vouchers

a. Summary of All Costs

As shown on Standard Form 1034, a summary of all current costs ***must*** be shown. This summary consists of a list identifying the general categories and the amounts incurred during the period covered by the billing, together with the portion of fixed fee (if any) payable for that period. The reimbursable costs incurred and the dates of the period for which the charges are claimed must fall within the period specified in the contract.

b. Details of Costs Claimed

As shown on Standard Form 1035, a detailed breakdown ***must*** be provided to substantiate the categories shown on the summary of costs. The following describes some of the categories that might appear on your billings:

(1) Direct Labor

Direct Labor costs consist of salaries and wages paid for scientific, technical, and other work performed directly for the contract and pursuant to the contract terms. Labor costs, excluding fringe benefits and overtime premium pay, will be billed as follows:

List the titles and amounts for employees whose salaries or wages, or portions thereof, were charged to the contract; show the rate (or hours) worked, and amount for each individual. The cost of direct labor, which is charged directly to the contract, must be supported by time records maintained in the contractor's office.

(2) Fringe Benefits

If it is the Contractor's established practice to treat fringe benefits as a direct cost, such costs should be billed separately as a single item.

NOTE: Fringe benefits, bonuses, etc., are usually treated as indirect costs for inclusion in the overhead pool; however, they may be treated as direct labor costs or as an "Other Direct Charge" if such treatment is in accordance with the Contractor's established accounting procedures.

(3) Premium Pay

Premium pay is the difference between the rates and amounts paid for overtime or shift work and amount normally paid on a straight time basis. Generally such pay is not included in the direct labor base and ***should not be included*** in the billing for "direct labor" unless the Contractor has consistently followed this practice in the past as a matter of policy. Premium pay of any kind unless provided for in the contract ***must*** be authorized by the Contracting Officer ***in advance***. Billings for unauthorized premium pays have caused frequent delays in payment due to suspensions and exchange of correspondence. Citations of authorization for premium pay will avoid delays in payment. Authorized premium pay may be shown as a single item on the summary of costs. However, it must be separately itemized for each position, or job category, showing the amount, and a citation of the Contracting Officer's letter of authorization on the continuation sheet of the invoice or voucher.

(4) Materials and Supplies

Only those items, which the Contractor normally treats as "direct costs", should be claimed under this heading. Major classifications of material ***only*** should be billed separately under appropriate classification. Items costing less than $25.00 may be listed by category of materials or supplies. Show the description and dollar amount of individual classifications. All such charges ***must be supported*** by the Contractor's office records.

(5) Travel

When authorized in the contract as a direct cost, travel costs that are directly related to specific contract performance may be billed as a direct cost. Travel cost detail should show:

(a) Name of traveler and official title,

(b) Purpose of trip,

(c) Dates of departure and return to starting point (station or airport),

(d) Transportation costs, identified as to rail, air, private automobile (including mileage and rate) and taxi.

(e) If claim for subsistence is on per diem basis, show number of days, rate and amount, as authorized in contract.[1] If claim is based on actual cost of subsistence, show, on a daily basis, the amounts claimed for lodging and meals separately.

(f) Reference to Contracting Officer's letter of authorization if required by contract.

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[1] For purposes of computing per diem charges in lieu of actual subsistence charges, unless otherwise provided in the contract, a day is divided into four quarters that begin at 12 midnight, 6:00 AM, 12 noon, and 6:00 PM. For example, at an authorized per diem rate of $35.00 per day, a traveler who departed at 9:15 AM on July 15 and returned at 6:45 PM on July 18 would be entitled to $131.25.

(6) Consultant Fees

Identify the consultant by name, number of days utilized, and amount of fee.

(7) Equipment

Nonexpendable personal property ***must*** be specifically approved in writing by the Contracting Officer or authorized by the terms of the contract. Billing data should include a description of item, make model, quantity, unit cost, total cost, and date approved by the Contracting Officer, if applicable. A copy of the contractor's bill may be snbmitted in lieu of the identifying information.

(8) Burden

Pending establishment of final contract indirect cost rates for each of the Contractor's fiscal years, the Contractor will be reimbursed based on his submittal of provisional rates as set forth in the contract. The contract may provide for more than one type indirect cost rate, such as overhead rate, and general and administrative expense rate, in which case the direct cost bases (e.g., direct labor, total direct cost, etc.)

(9) Fixed Fee

Ordinarily the fixed fee is stated in the contract as a lump sum and may be billed in the ratio of incurred costs to total estimated cost as set forth in the contract, with the final 15 percent to be billed on the final invoice or voucher. Contract terms govern the method of payments.

(10) Cumulative Amount Claimed

As shown on Standard Form 1035, the Contractor must show the cumulative amounts claimed by categories from the contract award date through the date of the current invoice or voucher, as well as the estimated cost to complete per category.

(11) Remaining Balance

Remaining balance shall be shown as part of the summary of the expenses for each cost-reimburseable CLIN.

QUICK CHECKLIST FOR INVOICE SUBMISSION:

- Standard Forms 1034 and 1035 recommended. If submitting own forms, statement must conform to billing instructions
- Quarterly billing as a minimum
- Invoice or Voucher must contain the minimum requirements per Voucher/Invoice Submission in Section G of the contract
- Vouchers should be collated
- Detail of Cost Claimed
- All invoices shall be submitted in conjunction with the monthly report.

(End of Clause)

G.8 EVALUATION OF CONTRACTOR PERFORMANCE UTILIZING CPARS (APRIL 2013)

In accordance with FAR 42.15, the Centers for Disease Control and Prevention (CDC) will review and evaluate contract performance. FAR 42.1502 and 42.1503 requires agencies to prepare evaluations of contractor performance and submit them to the Past Performance Information Retrieval System (PPIRS). The CDC utilizes the Department of Defense (DOD) web-based Contractor Performance Assessment Reporting System (CPARS) to prepare and report these contractor performance evaluations. All information contained in these assessments may be used by the Government, within the limitations of FAR 42.15, for future source selections in accordance with FAR 15.304 where past performance is an evaluation factor.

The CPARS system requires a contractor representative to be assigned so that the contractor has appropriate input into the performance evaluation process. The CPARS contractor representative will be given access to CPARS and will be given the opportunity to concur or not-concur with performance evaluations before the evaluations are

complete. The CPARS contractor representative will also have the opportunity to add comments to performance evaluations.

The assessment is not subject to the Disputes clause of the contract, nor is it subject to appeal beyond the review and comment procedures described in the guides on the CPARS website. Refer to: www.cpars.gov for details and additional information related to CPARS, CPARS user access, how contract performance assessments are conducted, and how Contractors participate. Access and training for all persons responsible for the preparation and review of performance assessments is also available at the CPARS website.

The contractor must provide the CDC contracting office with the name, e-mail address, and phone number of their designated CPARS representative who will be responsible for logging into CPARS and reviewing and commenting on performance evaluations. The contractor must maintain a current representative to serve as the contractor representative in CPARS. It is the contractor's responsibility to notify the CDC contracting office, in writing (letter or email), when their CPARS representative information needs to be changed or updated. Failure to maintain current CPARS contractor representative information will result in the loss of an opportunity to review and comment on performance evaluations.

Contractor to provide CPARS representative contact information:

Name: _____
Title:_____
E-Mail Address:_____
Phone Number:_____

(End of Clause)

G.9 NON-DISCLOSURE AGREEMENT FOR CONTRACTOR AND CONTRACTOR EMPLOYEES (MAY 2009)

(a) The contractor shall prepare and submit a Non-Disclosure Agreement (NDA) to the Contracting Officer prior to access of government information or the commencement of work at CDC.

(b) The NDA made part of this clause, Attachment #2 Appendix B, is required in service contracts where positions and/or functions proposed to be filled by contractor's employees will have access to non-public and procurement-sensitive information. The NDA also requires contractor's employees properly identify themselves as employees of a contractor when communicating or interacting with CDC employees, employees of other governmental entities (when communication or interaction relates to the contractor's work with the CDC), and members of the public. The Federal Acquisition Regulation (FAR) 37.114 (c), states "All contractor personnel attending meetings, answering Government telephones, and working in other situations where their contractor status is not obvious to third parties are required to identify themselves as such to avoid creating an impression in the minds of members of the public or Congress that they are Government officials, unless, in the judgment of the agency, no harm can come from failing to identify themselves. They must also ensure that all documents or reports produced by contractors are suitably marked as contractor products or that contractor participation is appropriately disclosed."

(c) The Contractor shall inform employees of the identification requirements by which they must abide and monitor employee compliance with the identification requirements.

(d) During the contract performance period, the Contractor is responsible to ensure that all additional or replacement contractors' employees sign a NDA and it is submitted to the Contracting Officer prior to commencement of their work with the CDC.

(e) Contractor employees in designated positions or functions that have not signed the appropriate NDA shall not have access to any non-public, procurement sensitive information or participate in government meeting where sensitive information maybe discussed.

(f) The Contractor shall prepare and maintain a current list of employees working under NDA's and submit to the Contracting Officer upon request during the contract period of performance. The list should at a minimum include: contract number, employee's name, position, date of hire and NDA requirement.

(End of Clause)

G.10 BILLING RATES (MAY 2013)
(applies to all CPFF CLINs)

Billing Rates (May 2013)

(a) As defined in FAR Subpart 42.7, Billing Rate means an indirect cost rate established temporarily for interim reimbursement of incurred indirect costs. Billing rates may be adjusted as necessary pending establishment of final indirect rates

(b) Notwithstanding the provisions of the clause entitled Allowable Cost and Payment (FAR 52.216-7), allowable indirect costs under this contract shall be determined by applying the following provisional billing rates to the bases specified below:

TYPE	EFFECTIVE PERIOD (FROM - TO)	RATE	APPLICABLE TO
FRINGE	**FY 2016**	(b)(4)	**Direct Labor**
OVERHEAD	**FY 2016**		**Direct Labor, Fringe**
G&A	**FY 2016**		**Direct Labor, Fringe, Overhead, Subcontractor Costs, Travel, ODCs**

Bases: (1)

(c) The above rates are provisional billing rates only and shall apply from the date of award until such time as the rates are revised by modification. Any modification to change the above rates will also state the effective period covered by the revised rates.

(d) Billing rates may be prospectively or retroactively revised by mutual agreement of the Contracting Officer and the contractor at either party's request. When agreement cannot be reached, the billing rates may be unilaterally determined by the Contracting Officer as set forth in FAR 42.704.

(e) Final indirect rates shall be established in accordance with FAR Clause 52.216-7, Allowable Cost and Payment. As required by that clause, the contractor shall submit an adequate final indirect cost rate proposal to the Contracting Officer and auditor within the 6 month period following the expiration of each of its fiscal years. Upon receipt of the final indirect cost rate proposal and pending the establishment of final indirect cost rates, the contractor and the Government may mutually agree to revise the provisional billing rates to reflect the proposed indirect cost rates until the proposal has been audited and settled.

(End of Clause)

Section H - Special Contract Requirements

H.1 PROHIBITION ON THE USE OF APPROPRIATED FUNDS FOR LOBBYING ACTIVITIES (JUL 1999)

The contractor is hereby notified of the restrictions on the use of Department of Health and Human Service's funding for lobbying of Federal, State and Local legislative bodies.

Section 1352 of Title 10, United Stated Code (Public Law 101-121, effective 12/23/89), among other things, prohibits a recipient (and their subcontractors) of a Federal contract, grant, loan, or cooperative agreement from using appropriated funds (other than profits from a federal contract) to pay any person for influencing or attempting to influence an officer or employee of any agency, a Member of Congress, an officer or employee of Congress, or an employee of a Member of Congress in connection with any of the following covered Federal actions; the awarding of any Federal contract; the making of any Federal grant; the making of any Federal loan; the entering into of any cooperative agreement; or the modification of any Federal contract, grant, loan, or cooperative agreement. For additional information of prohibitions against lobbying activities, see FAR Subpart 3.8 and FAR Clause 52.203-12.

In addition, the current Department of Health and Human Services Appropriations Act provides that no part of any appropriation contained in this Act shall be used, other than for normal and recognized executive-legislative relationships, for publicity or propaganda purposes, for the preparation, distribution, or use of any kit, pamphlet, booklet, publication, radio, television, or video presentation designed to support, or defeat legislation pending before the Congress, or any State or Local legislature except in presentation to the Congress, or any State or Local legislative body itself.

The current Department of Health and Human Services Appropriations Act also provides that no part of any appropriation contained in this Act shall be used to pay the salary or expenses of any contract or grant recipient, or agent acting for such recipient, related to any activity designed to influence legislation or appropriations pending before the Congress, or any State or Local legislature.

(End of Clause)

H.2 IDENTIFICATION OF DATA (MAY 1998)

The Contractor shall identify the technical data delivered to the Government as required by this contract with the number of the contract and the name and address of the Contractor or subcontractor that generated the data.

(End of Clause)

H.3 DISSEMINATION OF INFORMATION (MAY 1998)

No information related to data obtained under this contract shall be released or publicized without the prior written consent of the COTR.

(End of Clause)

H.4 RECORD RETENTION

The Contractor is responsible for records retention and CDC/NIOSH is not implementing a document destruction program authorizing the destruction of medical documents. However, in an effort to reduce associated costs for storing medical documents, the Department of Justice (DOJ) has agreed that electronic imaging is an acceptable method of storage. This provides contractors the opportunity to destroy the paper documents, as long as the following conditions are met:

(1) Contractors must certify the image is absolutely an identical replication of the paper document in every way;

(2) The scanned image becomes the record keeping copy and is verified and documented as an identical replication of the paper document; and
(3) Contractors must maintain accessibility and the ability to read the document in accordance with changes in technology.
Under no circumstances are contractors to destroy the only copy of any information, data, or files that CDC/NIOSH, DOJ, DHHS' Office of General Counsel or the OIG have identified as relating to a current investigation or litigation/negotiation, ongoing Workers' Compensation set aside arrangements, or documents which prompt suspicions of fraud or abuse of over utilization of services. Contractors can either retain the image copy or the hardcopy. This will satisfy evidentiary needs and discovery obligations critical to the agency's litigation interest.

(End of Clause)

H.5 USE OF GOVERNMENT DATA (REPORTS/FILES/COMPUTER TAPES OR DISCS)

Any data given to the contractor by the Government shall be used only for the performance of the contract unless the Contracting Officer specifically permits another use, in writing. Should the Contracting Officer permit the contractor the use of Government-supplied data for a purpose other than solely for performance of this contract and if such use could result in a commercially viable product, the Contracting Officer and the contractor must negotiate a financial benefit to the Government. This benefit should most often be in the form of a reduction in the price of the contract; however, the Contracting Officer may negotiate any other benefits he/she determines are adequate compensation for the use of these data.

Upon the request of the Contracting Officer or the expiration date of this contract, whichever shall come first, the contractor shall, upon instructions from the Contracting Officer, return or destroy all data given to the contractor by the Government. However, the Contracting Officer may direct that the data be retained by the contractor for a specific period of time, which period shall be subject to agreement by the contractor. Whether the data are returned, retained, or destroyed shall be the decision of the Contracting Officer with the exception that the contractor may refuse to retain the data. The contractor shall retain no data, copies of data, or parts thereof, in any form, when the Contracting Officer directs that the data be returned or destroyed. If the data are to be destroyed, the contractor shall directly furnish evidence of such destruction in a form the Contracting Officer shall determine is adequate.

(End of Clause)

H.6 PRIVACY ACT (SEP 2009)

(a) Notification is hereby given that the Contractor and its employees are subject to criminal penalties for violation of the Privacy Act to the same extent as employees of the Government. The Contractor shall assure that each of its employees knows the prescribed rules of conduct and that each is aware that he or she can be subjected to criminal penalty for violation of the Act. A copy of 45 CFR Part 5b, Privacy Act Regulations, may be obtained at http://ecfr.gpoaccess.gov.

(b) The Contracting Officer's Technical Representative (COTR) is hereby designated as the official who is responsible for monitoring contractor compliance with the Privacy Act.

(End of Clause)

H.7 HEALTH INSURANCE PORTABILITY & ACCOUNTABILITY ACT OF 1996 (SEP 2008)

Pursuant to the Standards for Privacy of Individually Identifiable Health Information promulgated under the Health Insurance Portability and Accountability Act (HIPAA)(45 CFR Parts 160 and 164), covered entities may disclose protected health information to public health authorities "...anthorized by law to collect or receive such information for the pnrpose of preventing or controlling disease, injury, or disability, including, but not limited to, the reporting of disease, injury, vital events such as birth or death, and the conduct of public health surveillance, public health investigations, and public health interventions..." The definition of a public health authority includes "...a person or entity acting under a grant of authority from or contract with such public agency..." The **Karna** is acting under

contract with the CDC to carry out **Health Program Support** which is authorized by **H.R. 5503 (IH) James Zadroga 9/11 Health and Compensation Reauthorization Act** and therefore may be considered a public health authority under the Privacy Rule for purposes of this project. Further, CDC consider this to be a **public health activity** for which disclosure of protected health information by covered entities is authorized by section 164.512(b) of the Privacy Rule (45 CFR 164.512(b)).

(End of Clause)

H.8 HEALTH INFORMATION TECHNOLOGY (JANUARY 2012)

Executive Order 13410: Promoting Quality and Efficient Health Care in Federal Government Administered or Sponsored Health Care Programs promotes efficient delivery of quality health care through the use of health information technology, transparency regarding health care quality and price, and incentives to promote the widespread adoption of health information technology and quality of care. To support this objective the awardee shall adhere to the following interoperability standards and requirements. For purposes of this contract clause, interoperability is defined as the ability of two or more systems or components to exchange information and to use the information that has been exchanged.

The Health Information Technology Clause (January 2012) shall be inserted in Section H of all contracts that involve (1) the exchange of public health data between public health entities, CDC, and other federal agencies, or (2) the exchange of public health data within the clinical setting, such as hospitals and providers, or any combination of (1) and (2), and covers both situations. The inclusion of this clause is a Contracting Officer decision made with the advice and in coordination with the appropriate program officials from the requesting office.

Interoperability of Health Information Technology

This section of the clause applies to contracts that involve the exchange of public health data between public health entities, CDC, and other federal agencies. This supports the development and use of interoperable information systems for public health functions such as biosurveillance, situational awareness, laboratory science, or environmental health.

- In addition to complying with HHS Standards for Security Configurations (HHSAR Clause 352.239-70), HHS Standards for Encryption Language (HHSAR Clause 352.239-71) and Federal Laws, Regulations and Standards related to IT Security (HHSAR Clause 352.239-72), the awardee is required to meet recognized interoperability standards as follows:

    - The awardee shall use nationally adopted Public Health Information Network (PHIN) standards (www.cdc.gov/phin) at the time of the system implementation, acquisition, or upgrade, in all relevant information technology systems supported, in whole or in part, through this contract. Where no PHIN standards exist, the awardee shall utilize recognized standards established by the Office of the National Coordinator for Health Information Technology (ONC) as described in their Nationwide Health Information Network (NwHIN) standards (www.hhs.gov/healthit/standards/background).

    - The awardee shall use PHIN policies, standards, practices, and services in the research and implementation of efficient and effective public health information systems, to facilitate interoperability (the ability of two or more systems or components to exchange information and to use the information that has been exchanged) with public health organizations and networks, including NwHIN.

    - Where the awardee supports or participates in health information or data exchange with disparate entities, the awardee must have an architecture that is compatible with the architectnre of PHIN, where appropriate. If PHIN standards do not exist, the awardee shall refer to NwHIN standards (www.hhs.gov/healthit/standards/background).

Public Health Data within Clinical Healthcare

This section of the clause applies to contracts that involve the exchange of public health data within the clinical setting, such as hospitals and providers. This supports the exchange of public health information, including laboratory exchange data, between public health entities and healthcare systems.

- The awardee shall use health IT investments, such as electronic health records (EHRs), personalized health records, and network components through which they operate and share information, that are certified by a recognized certification board and/or validated and tested with PHIN (www.cdc.gov/phin) or NwHIN (http://healthit.hhs.gov/portal/server.pt?open=512&objID=1153&mode=2) certification to ensure a minimum level of interoperability or compatibility of health IT products.

- The awardee may use services available from the Agency for Healthcare Research and Quality (AHRQ) at www.healthit.ahrq.gov. This link provides technical assistance, identifies challenges to health IT adoption and use, and identifies solutions and best practices that have the potential to transform clinical practice through the best and most effective use of IT.

- Where feasible, the awardee shall collaborate with other healthcare entities, local quality improvement organizations, and/or local medical societies to promote the adoption of certified EHRs in clinical provider settings.

- Where feasible, the awardee shall collaborate with other healthcare entities and/or local hospital associations to promote the adoption of certified EHRs in hospital settings.

- The awardee shall identify public health entities that have a stake in the award and demonstrate interactions with them to implement and promote the adoption of PHIN and/or NwHIN data exchange standards in support of federal health initiatives (e.g., situational awareness, EHR Meaningful Use).

- The awardee shall report on public health information exchange activities that include partnerships with health information exchanges, other public health partners, or any other stakeholders within program reports, progress reports, or as requested by CDC.

(End of Clause)

H.9 SECTION 508 COMPLIANCE

ELECTRONIC AND INFORMATION TECHNOLOGY ACCESSIBILITY NOTICE

(a) Section 508 of the Rehabilitation Act of 1973 (29 U.S.C. 794d), as amended by the Workforce Investment Act of 1998 and the Architectural and Transportation Barriers Compliance Board Electronic and Information (EIT) Accessibility Standards (36 CFR part 1194), require that when Federal agencies develop, procure, maintain, or use electronic and information technology, Federal employees with disabilities have access to and use of information and data that is comparable to the access and use by Federal employees who are not individuals with disabilities, unless an undue burden would be imposed on the agency. Section 508 also requires that individuals with disabilities, who are members of the public seeking information or services from a Federal agency, have access to and use of information and data that is comparable to that provided to the public who are not individuals with disabilities, unless an undue burden would be imposed on the agency.

(b) Accordingly, any contractor responding to this solicitation must comply with established HHS EIT accessibility standards. Information about Section 508 is available at http://www.hhs.gov/web/508. The complete text of the Section 508 Final Provisions can be accessed at http://www.access-board.gov/sec508/standards.htm.

(c) The Section 508 accessibility standards applicable to this contract are: 1194.21, .22, .31, and .41.
In order to facilitate the Government's determination whether proposed EIT supplies meet applicable Section 508 accessibility standards, contractors must submit an HHS Section 508 Product Assessment Template, in accordance

with its completion instructions. The purpose of the template is to assist HHS acqnisition and program officials in determining whether proposed EIT snpplies conform to applicable Section 508 accessibility standards. The template allows contractors or developers to self-evalnate their supplies and documentation detail - whether they conform to a specific Section 508 accessibility standard, and any underway remediation efforts addressing conformance issues. Instructions for preparing the HHS Section 508 Evaluation Template are available under Section 508 policy on the HHS Web site http://hhs.gov/web/508.

In order to facilitate the Government's determination whether proposed EIT services meet applicable Section 508 accessibility standards, contractors must provide enough information to assist the Government in determining that the EIT services conform to Section 508 accessibility standards, including any underway remediation efforts addressing conformance issues.

(d) Respondents to this solicitation must identify any exception to Section 508 requirements. If a contractor claims its supplies or services meet applicable Section 508 accessibility standards, and it is later determined by the Government, i.e., after award of a contract or order, that supplies or services delivered do not conform to the described accessibility standards, remediation of the supplies or services to the level of conformance specified in the contract will be the responsibility of the Contractor at its expense.

(End of provision)

H.10 SUBCONTRACTING PLAN (JUL 1999 REVISED)

The Contractor's subcontracting plan, as negotiated, submitted in response to this acquisition's solicitation is hereby incorporated by reference.

(End of clause)

H.11 HHSAR 352.273-75 KEY PERSONNEL (DEC 2015)

The key personnel specified in this contract are considered to be essential to work performance. At least 30 days prior to the contractor voluntarily diverting any of the specified individuals to other programs or contracts the Contractor shall notify the Contracting Officer and shall submit a justification for the diversion or replacement and a request to replace the individual. The request must identify the proposed replacement and provide an explanation of how the replacement's skills, experience, and credentials meet or exceed the requirements of the contract (including, when applicable, Human Subjects Testing requirements). If the employee of the contractor is terminated for cause or separates from the contractor voluntarily with less than thirty days notice, the Contractor shall provide the maximum notice practicable under the circumstances. The Contractor shall not divert, replace, or announce any such change to key personnel without the written consent of the Contracting Officer. The contract will be modified to add or delete key personnel as necessary to reflect the agreement of the parties.

Name	Labor Category
(b)(6)	Program Manager

(End of clause)

H.12 CONDITIONS OF PERFORMANCE

In addition to the performance requirements of this contract set forth under Section C, Description/Specifications, Work Statement, the Contractor may be required to comply with the requirements of any revisions in legislation or regulations, which may be enacted or implemented during the period of performance of this contract, and are directly applicable to the performance requirements of this contract. In the event that revisions in legislation or regulations are enacted and do impact the performance requirements of this contract, the Contractor will have an opportunity to assess the cost and schedule impacts of such revisions and will, when applicable, be provided an equitable adjustment subject to the negotiated indirect cost ceiling(s) established in the contract.

(End of Clause)

H.13 HOMELAND SECURITY PRESIDENTIAL DIRECTIVE – 12(HSPD-12) REQUIREMENTS (NOVEMBER 2011)

(a) To perform the work specified herein, contractor personnel are expected to have routine 1) physical access to an HHS-controlled facility; 2) logical access to an HHS controlled information systems; or 3) access to sensitive HHS data or information, whether in an HHS controlled information system or in hard copy. This contract/order will entail the following position sensitivity level(s): **Public Trust – Level 5.**

(b) To gain routine physical access to an HHS facility, logical access to an HHS controlled information system, and or access to sensitive data or information, the contractor and its employees shall comply with Homeland Security Presidential Directive 12, Policy for a Common Identification Standard for Federal Employees and Contractors; Office of Management and Budget Memorandum (M-05-24); Federal Information Processing Standards Publication Number 201; and with the personal identity verification and investigation procedures contained in the following documents:

(1) HHS Information Security Program Policy

(2) HHS Office of Security and Drug Testing, Personnel Security/Suitability Handbook, dated February 1, 2005

(3) CDC Policy: "ISSUANCE AND UTILIZATION OF PIV CREDENTIALS"

(4) CDC Policy: "IN- AND OUT-PROCESSING OF CDC FTEs, PSCs, CONTRACTORS, AND OTHER NON-FTEs"

(5) CDC Policy: "PHYSICAL ACCESS TO CDC FACILITIES"

(6) CDC Policy: "NATIONAL AGENCY CHECK AND INQUIRY PROCEDURES"

(c) The personnel investigation procedures for Contractor personnel require that the Contractor prepare and submit background check/investigation forms based on the type of investigation required. The minimum Government investigation for a non-sensitive position is a National Agency Check and Inquiries (with fingerprinting). More restricted positions, above non-sensitive, require more extensive documentation and investigation. The Contractor shall notify the Contracting Officer in advance when any new personnel, who are subject to a background check/investigation, will work under the contract and if they have previously been the subject of national agency checks or background investigations.

(d) Investigations are expensive and may delay performance, regardless of the outcome of the investigation. Delays associated with rejections and consequent re-investigations may not be excusable in accordance with the FAR clause, Excusable Delays--see FAR 52.249-14. Accordingly, the Contractor shall ensure that any additional employees whose names it submits for work under this contract have a reasonable chance for approval.

(e) Multiple investigations for the same position may, at the Contracting Officer's discretion, justify reduction(s) in the contract price of no more than the cost of the extra investigation(s).

(f) Language similar to this Security section shall be included in any subcontracts which require subcontractor personnel to have the same frequency and duration of (1) physical access to an HHS-controlled facility; (2) logical access to an HHS-controlled information system; or (3) access to sensitive HHS data/information, whether in an HHS-controlled information system or in hard copy; access to an information system, access to sensitive data, regular or prolonged access to an HHS-controlled facility.

(g) Inquiries on matters that affect contract compliance or terms and conditions should be directed to the Contracting Officer or designee.

(h) Within seven (7) calendar days after final acceptance of the work specified herein, the contractor shall return all identification badges in accordance with CDC's Policy titled "In- And Out-Processing of CDC FTEs, PSCs, Contractors, and Other Non-FTEs Policy". Contractor employees who separate from service under the contract prior to final acceptance shall be out processed in accordance with that same policy.

(End of Clause)

Section I - Contract Clauses

Section I-1 - Clauses Incorporated By Reference

I.1 52.252-2 CLAUSES INCORPORATED BY REFERENCE (FEB 1998)

This contract incorporates one or more clauses by reference, with the same force and effect as if they were given in full text. Upon request, the Contracting Officer will make their full text available. Also, the full text of a clause may be accessed electronically at this/these address(es):

http://www.acqnet.gov
http://farsite.hill.af.mil/

(End of Clause)

I.2 FAR 52.252-6 AUTHORIZED DEVIATIONS IN CLAUSES (APR 1984)

(a) The use in this solicitation or contract of any Federal Acquisition Regulation (48 CFR Chapter 1) clause with an authorized deviation is indicated by the addition of "(DEVIATION)" after the date of the clause.

(b) The use in this solicitation or contract of any **Health and Human Services Acquisition Regulation** (48 CFR **Chapter 3**) clause with an authorized deviation is indicated by the addition of "(DEVIATION)" after the name of the regulation.

(End of Clause)

FAR SOURCE	TITLE AND DATE
52.202-1	Definitions (Nov 2013)(Deviation)(352.202-1)(Jan 2006)
52.203-3	Gratuities (Apr 1984)
52.203-5	Covenant against Contingent Fees (May 2014)
52.203-6	Restrictions on Subcontractor Sales to the Government (Sep 2006)
52.203-7	Anti-Kickback Procedures (May 2014)
52.203-8	Cancellation, Rescission, and Recovery of Funds for Illegal or Improper Activity (May 2014)
52.203-10	Price or Fee Adjustment for Illegal or Improper Activity (May 2014)
52.203-12	Limitation on Payments to Influence Certain Federal Transactions (Oct 2010)
52.203-13	Contractor Code of Business Ethics and Conduct (Oct 2015)
52.203-17	Contractor Employee Whistleblower Rights and Requirement To Inform Employees of Whistleblower Rights (Apr 2014)
52.204-4	Printed or Copied Double-Sided on Recycled Paper (May 2011)
52.204-10	Reporting Executive Compensation and First-Tier Subcontract Awards (Jul 2013)
52.204-13	System for Award Management Maintenance (Jul 2013)
52.204-14	Service Contract Reporting Requirements (Jan 2014)
52.204-19	Incorporation by Reference of Representations and Certifications (Dec 2014)
52.209-6	Protecting the Government's Interest When Subcontracting with Contractors Debarred, Suspended, or Proposed for Debarment (Oct 2015)

52.209-9	Updates of Publicly Available Information Regarding Responsibility Matters (Jul 2013)
52.209-10	Prohibition on Contracting With Inverted Domestic Corporations (Nov 2015)
52.215-2	Audit and Records - Negotiation (Oct 2010)
52.215-8	Order of Precedence - Uniform Contract Format (Oct 1997)
52.215-10	Price Reduction for Defective Cost or Pricing Data (Aug 2011)
52.215-11	Price Reduction for Defective Cost or Pricing Data - Modifications (Aug 2011)
52.215-12	Snbcontractor Cost or Pricing Data (Oct 2010)
52.215-13	Subcontractor Cost or Pricing Data - Modifications (Oct 2010)
52.215-14	Integrity of Unit Prices (Oct 2010)
52.215-15	Pension Adjustments and Asset Reversions (Oct 2010)
52.215-18	Reversion or Adjnstment of Plans for Postretirement Benefits (PRB) Other Than Pensions (Jul 2005)
52.215-21	Requirements for Cost or Pricing Data or Information Other Than Cost or Pricing Data - Modifications (Oct 2010)
52.215-23	Limitations on Pass-Through Charges (Oct 2009)
52.216-7	Allowable Cost and Payment (Jun 2013)
(applies to all CPFF CLINs)	
52.216-8	Fixed Fee (Jun 2011)
(applies to all CPFF CLINs)	
52.219-6	Notice of Total Small Business Set-Aside (Nov 2011)
52.219-14	Limitations on Subcontracting (Nov 2011)
52.219-28	Post-Award Small Business Program Rerepresentation (Jul 2013)
52.222-3	Convict Labor (Jun 2003)
52.222-17	Non-displacement of Qualified Workers (May 2014)
52.222-21	Prohibition of Segregated Facilities (Apr 2015)
52.222-26	Equal Opportunity (Apr 2015)
52.222-35	Equal Opportunity for Veterans (Oct 2015)
52.222-36	Equal Opportunities for Workers With Disabilities (Jul 2014)
52.222-37	Employment Reports on Veterans (Oct 2015)
52.222-40	Notification of Employee Rights Under the National Labor Relations Act (Dec 2010)
52.222-50	Combating Trafficking in Persons (Oct 2015)
52.222-54	Employment Eligibility Verification (Oct 2015)
52.223-6	Drug-Free Workplace (May 2001)
52.223-18	Encouraging Contractor Policies to Ban Text Messaging While Driving (Aug 2011)
52.224-1	Privacy Act Notification (Apr 1984)

52.224-2	Privacy Act (Apr 1984)
52.225-13	Restrictions on Certain Foreign Purchases (Jun 2008)
52.227-1	Authorization and Consent (Dec 2007)
52.227-2	Notice and Assistance Regarding Patent and Copyright Infringement (Dec 2007)
52.227-11	Patent Rights -- Ownership by the Contractor (May 2014)
52.227-14	Rights in Data - General (May 2014)
52.227-17	Rights in Data – Special Works (Dec 2007)
52.227-23	Right to Proposal Data (Technical) (Jun 1987)
52.228-7	Insurance -- Liability to Third Persons (Mar 1996)
52.232-1 (applies to all FFP CLINs)	Payments (Apr 1984)
52.232-9	Limitation on Withholding of Payments (Apr 1984)
52.232-17	Interest (May 2014)
52.232-20 (applies to all CPFF CLINs)	Limitation of Cost (Apr 1984)
52.232-22	Limitation of Funds (Apr 1984)
52.232-23	Assignment of Claims (May 2014)
52.232-25	Prompt Payment (Jul 2013) - Alternate I (Feb 2002)
52.232-33	Payment by Electronic Funds Transfer—System for Award Management (Jul 2013)
52.233-1	Disputes (May 2014)
52.233-3	Protest After Award (Aug 1996) – Alternate I (Jun 1985)
52.233-4	Applicable Law for Breach of Contract Claim (Oct 2004)
52.237-3	Continuity Of Services (Jan 1991)
52.237-7	Indemnification and Medical Liability Insurance (Jan 1997)
52.239-1	Privacy or Security Safeguards (Aug 1996)
52.242-1 (applies to all CPFF CLINs)	Notice of Intent to Disallow Costs (Apr 1984)
52.242-3 (applies to all CPFF CLINs)	Penalties for Unallowable Costs (May 2014)
52.242-4 (applies to all CPFF CLINs)	Certification of Final Indirect Costs (Jan 1997)
52.242-13	Bankruptcy (Jul 1995)
52.243-1 (applies to all FFP	Changes – Fixed Price (Aug 1987)

CLINs)	
52.243-1 Alternate III	Changes – Fixed Price – Alternate III (Apr 1984)
(applies to all FFP CLINs)	
52.243-2	Changes - Cost-Reimbursement (Aug 1987) – Alternate I (Apr 1984)
(applies to all CPFF CLINs)	
52.244-5	Competition in Subcontracting (Dec 1996)
52.244-6	Subcontracts for Commercial Items (Oct 2015)
52.245-1	Government Property (Apr 2012)
52.245-9	Use and Charges (Apr 2012)
52.246-5	Inspection of Services – Cost Reimbursement (Apr 1984)
52.246-25	Limitation of Liability - Services (Feb 1997)
52.248-1	Value Engineering (Oct 2010)
52.249-2	Termination for Convenience of the Government (Fixed-Price) (May 2004)
(applies to all FFP CLINs)	
52.249-6	Termination (Cost Reimbursement) (May 2004)
(applies to all CPFF CLINs)	
52.249-8	Default (Fixed-Price Supply and Service)(Apr 1984)
(applies to all FFP CLINs)	
52.249-14	Excusable Delays (Apr 1984)
52.253-1	Computer Generated Forms (Jan 1991)
HHSAR SOURCE	**TITLE AND DATE**
352.216-70	Additional Cost Principles (Dec 2015)
(applies to all CPFF CLINs)	
352.219-70	Mentor-Protégé Program (Dec 2015)
352.222-70	Contractor Cooperation in Equal Employment Opportunity Investigations (Dec 2015)
352.223-70	Safety and Health (Dec 2015)
352.224-70	Privacy Act (Dec 2015)
352.227-70	Publications and Publicity (Dec 2015)
352.232-70	Incremental Funding (Dec 2015)
352.233-71	Litigation and Claims (Dec 2015)
352.239-73	Electronic Information and Technology Accessibility (Dec 2015)

Section I-2 - Clauses Incorporated In Full Text

I.3. FAR 52.203-14 – DISPLAY OF HOTLINE POSTER(S) (OCT 2015)

(a) *Definition.*
"United States," as used in this clause, means the 50 States, the District of Columbia, and outlying areas.
(b) *Display of fraud hotline poster(s).* Except as provided in paragraph (c)—

(1) During contract performance in the United States, the Contractor shall prominently display in common work areas within business segments performing work under this contract and at contract work sites—

(i) Any agency fraud hotline poster or Department of Homeland Security (DHS) fraud hotline poster identified in paragraph (b)(3) of this clause; and

(ii) Any DHS fraud hotline poster subsequently identified by the Contracting Officer.

(2) Additionally, if the Contractor maintains a company website as a method of providing information to employees, the Contractor shall display an electronic version of the poster(s) at the website.

(3) Any required posters may be obtained as follows:

Poster(s)	Obtain from

(Contracting Officer shall insert—

(i) Appropriate agency name(s) and/or title of applicable Department of Homeland Security fraud hotline poster); and

(ii) The website(s) or other contact information for obtaining the poster(s).)

(c) If the Contractor has implemented a business ethics and conduct awareness program, including a reporting mechanism, such as a hotline poster, then the Contractor need not display any agency fraud hotline posters as required in paragraph (b) of this clause, other than any required DHS posters.
(d) *Subcontracts.* The Contractor shall include the substance of this clause, including this paragraph (d), in all subcontracts that exceed $5.5 million, except when the subcontract—

(1) Is for the acquisition of a commercial item; or

(2) Is performed entirely outside the United States.

(End of clause)

I.4 FAR 52.215-19 -- NOTIFICATION OF OWNERSHIP CHANGES (OCT 1997)

(a) The Contractor shall make the following notifications in writing:

(1) When the Contractor becomes aware that a change in its ownership has occurred, or is certain to occur, that could result in changes in the valuation of its capitalized assets in the accounting records, the Contractor shall notify the Administrative Contracting Officer (ACO) within 30 days.

(2) The Contractor shall also notify the ACO within 30 days whenever changes to asset valuations or any other cost changes have occurred or are certain to occur as a result of a change in ownership.

(b) The Contractor shall –

(1) Maintain current, accurate, and complete inventory records of assets and their costs;

(2) Provide the ACO or designated representative ready access to the records upon request;

(3) Ensure that all individual and grouped assets, their capitalized values, accumulated depreciation or amortization, and remaining useful lives are identified accurately before and after each of the Contractor's ownership changes; and

(4) Retain and continue to maintain depreciation and amortization schedules based on the asset records maintained before each Contractor ownership change.

(c) The Contractor shall include the substance of this clause in all subcontracts under this contract that meet the applicability requirement of FAR 15.408(k).

(End of Clause)

I.5 FAR 52.217-8 OPTION TO EXTEND SERVICES (NOV 1999)

The Government may require continued performance of any services within the limits and at the rates specified in the contract. These rates may be adjusted only as a result of revisions to prevailing labor rates provided by the Secretary of Labor. The option provision may be exercised more than once, but the total extension of performance hereunder shall not exceed 6 months. The Contracting Officer may exercise the option by written notice to the Contractor within **any time prior to expiration of the contract or any exercised option periods.**

(End of Clause)

I.6 FAR 52.217-9 OPTION TO EXTEND THE TERM OF THE CONTRACT (MAR 2000)

(a) The Government may extend the term of this contract by written notice to the Contractor within **any time prior to expiration of the contract or any exercised option periods**; provided, that the Government gives the Contractor a preliminary written notice of its intent to extend at least **thirty (30)** days before the contract expires. The preliminary notice does not commit the Government to an extension.

(b) If the Government exercises this option, the extended contract shall be considered to include this option clause.

(c) The total duration of this contract, including the exercise of any options under this clause, shall not exceed **fifty-four (54) months**.

(End of Clause)

I.7 FAR 52.219-28 POST-AWARD SMALL BUSINESS PROGRAM REPRESENTATION (JUL 2013)

(a) *Definitions*. As used in this clause--

Long-term contract means a contract of more than five years in duration, including options. However, the term does not include contracts that exceed five years in duration because the period of performance has been extended for a cumulative period not to exceed six months under the clause at 52.217-8, Option to Extend Services, or other appropriate authority.

Small business concern means a concern, including its affiliates, that is independently owned and operated, not dominant in the field of operation in which it is bidding on Government contracts, and qualified as a small business under the criteria in 13 CFR part 121 and the size standard in paragraph (c) of this clause. Such a concern is "not dominant in its field of operation" when it does not exercise a controlling or major influence on a national basis in a kind of business activity in which a number of business concerns are primarily engaged. In determining whether dominance exists, consideration shall be given to all appropriate factors, including volume of business, number of employees, financial resources, competitive status or position, ownership or control of materials, processes, patents, license agreements, facilities, sales territory, and nature of business activity.

(b) If the Contractor represented that it was a small business concern prior to award of this contract, the Contractor shall rerepresent its size status according to paragraph (e) of this clause or, if applicable, paragraph (g) of this clause, upon the occurrence of any of the following:

(1) Within 30 days after execution of a novation agreement or within 30 days after modification of the contract to include this clause, if the novation agreement was executed prior to inclusion of this clause in the contract.

(2) Within 30 days after a merger or acquisition that does not require a novation or within 30 days after modification of the contract to include this clause, if the merger or acquisition occurred prior to inclusion of this clause in the contract.

(3) For long-term contracts—

(i) Within 60 to 120 days prior to the end of the fifth year of the contract; and

(ii) Within 60 to 120 days prior to the date specified in the contract for exercising any option thereafter.

(c) The Contractor shall rerepresent its size status in accordance with the size standard in effect at the time of this rerepresentation that corresponds to the North American Industry Classification System (NAICS) code assigned to this contract. The small business size standard corresponding to this NAICS code can be found at http://www.sba.gov/content/table-small-business-size-standards .

(d) The small business size standard for a Contractor providing a product which it does not manufacture itself, for a contract other than a construction or service contract, is 500 employees.

(e) Except as provided in paragraph (g) of this clause, the Contractor shall make the representation required by paragraph (b) of this clause by validating or updating all its representations in the Representations and Certifications section of the System for Award Management (SAM) and its other data in SAM, as necessary, to ensure that they reflect the Contractor's current statns. The Contractor shall notify the contracting office in writing within the timeframes specified in paragraph (b) of this clause that the data have been validated or updated, and provide the date of the validation or update.

(f) If the Contractor represented that it was other than a small business concern prior to award of this contract, the Contractor may, but is not required to, take the actions required by paragraphs (e) or (g) of this clause.

(g) If the Contractor does not have representations and certifications in SAM, or does not have a representation in SAM for the NAICS code applicable to this contract, the Contractor is required to complete the following rerepresentation and submit it to the contracting office, along with the contract number and the date on which the rerepresentation was completed:

The Contractor represents that it [ ] is, [ ] is not a small business concern under NAICS Code _____ assigned to contract number _____.[*Contractor to sign and date and insert authorized signer's name and title*].

(End of clause)

I.8 FAR 52.222-2 PAYMENT FOR OVERTIME PREMIUMS (JUL 1990)

(a) The use of overtime is authorized under this contract if the overtime premium does not exceed **(to be negotiated)** or the overtime premium is paid for work –

(1) Necessary to cope with emergencies such as those resulting from accidents, natural disasters, breakdowns of production equipment, or occasional production bottlenecks of a sporadic nature;

(2) By indirect-labor employees such as those performing duties in connection with administration, protection, transportation, maintenance, standby plant protection, operation of utilities, or accounting;

(3) To perform tests, industrial processes, laboratory procedures, loading or unloading of transportation conveyances, and operations in flight or afloat that are continuous in nature and cannot reasonably be interrupted or completed otherwise; or

(4) That will result in lower overall costs to the Government.

(b) Any request for estimated overtime premiums that exceeds the amount specified above shall include all estimated overtime for contract completion and shall –

(1) Identify the work unit; e.g., department or section in which the requested overtime will be used, together with present workload, staffing, and other data of the affected unit sufficient to permit the Contracting Officer to evaluate the necessity for the overtime;

(2) Demonstrate the effect that denial of the request will have on the contract delivery or performance schedule;

(3) Identify the extent to which approval of overtime would affect the performance or payments in connection with other Government contracts, together with identification of each affected contract; and

(4) Provide reasons why the required work cannot be performed by using multishift operations or by employing additional personnel.

(End of Clause)

I.9 FAR 52.222-42 STATEMENT OF EQUIVALENT RATES FOR FEDERAL HIRES (MAY 2014)

In compliance with the Service Contract Labor Standards statute and the regulations of the Secretary of Labor (29 CFR part 4), this clause identifies the classes of service employees expected to be employed under the contract and states the wages and fringe benefits payable to each if they were employed by the contracting agency subject to the provisions of 5 U.S.C. 5341 or 5332.

This Statement is for Information Only:

Employee Class	Monetary Wage—Fringe Benefits
01041 – Customer Service Representative I	(b)(4)
01111 – General Clerk I	
14041 – Computer Operator I	
24550 – Case Manager	

(End of Clause)

I.10 FAR 52.244-2 SUBCONTRACTS (OCT 2010)

(a) Definitions. As used in this clause—

"Approved purchasing system" means a Contractor's purchasing system that has been reviewed and approved in accordance with Part 44 of the Federal Acquisition Regulation (FAR).

"Consent to subcontract" means the Contracting Officer's written consent for the Contractor to enter into a particular subcontract.

"Subcontract" means any contract, as defined in FAR Subpart 2.1, entered into by a subcontractor to furnish supplies or services for performance of the prime contract or a subcontract. It includes, but is not limited to, purchase orders, and changes and modifications to purchase orders.

(b) When this clause is included in a fixed-price type contract, consent to snbcontract is reqnired only on unpriced contract actions (including unpriced modifications or unpriced delivery orders), and only if required in accordance with paragraph (c) or (d) of this clause.

(c) If the Contractor does not have an approved purchasing system, consent to subcontract is required for any subcontract that—

(1) Is of the cost-reimbursement, time-and-materials, or labor-hour type; or

(2) Is fixed-price and exceeds—

(i) For a contract awarded by the Department of Defense, the Coast Guard, or the National Aeronautics and Space Administration, the greater of the simplified acquisition threshold or 5 percent of the total estimated cost of the contract; or

(ii) For a contract awarded by a civilian agency other than the Coast Guard and the National Aeronautics and Space Administration, either the simplified acquisition threshold or 5 percent of the total estimated cost of the contract.

(d) If the Contractor has an approved purchasing system, the Contractor nevertheless shall obtain the Contracting Officer's written consent before placing the following subcontracts: **Any action exceeding the simplified acquisition threshold or 5 percent of the total estimated cost of the contract.**

(e)(1) The Contractor shall notify the Contracting Officer reasonably in advance of placing any subcontract or modification thereof for which consent is required under paragraph (b), (c), or (d) of this clause, including the following information:

(i) A description of the supplies or services to be subcontracted.

(ii) Identification of the type of subcontract to be used.

(iii) Identification of the proposed subcontractor.

(iv) The proposed subcontract price.

(v) The subcontractor's current, complete, and accurate certified cost or pricing data and Certificate of Current Cost or Pricing Data, if required by other contract provisions.

(vi) The subcontractor's Disclosure Statement or Certificate relating to Cost Accounting Standards when such data are required by other provisions of this contract.

(vii) A negotiation memorandum reflecting—

(A) The principal elements of the snbcontract price negotiations;

(B) The most significant considerations controlling establishment of initial or revised prices;

(C) The reason certified cost or pricing data were or were not required;

(D) The extent, if any, to which the Contractor did not rely on the subcontractor's certified cost or pricing data in determining the price objective and in negotiating the final price;

(E) The extent to which it was recognized in the negotiation that the subcontractor's certified cost or pricing data were not accurate, complete, or current; the action taken by the Contractor and the subcontractor; and the effect of any such defective data on the total price negotiated;

(F) The reasons for any significant difference between the Contractor's price objective and the price negotiated; and

(G) A complete explanation of the incentive fee or profit plan when incentives are used. The explanation shall identify each critical performance element, management decisions used to quantify each incentive element, reasons for the incentives, and a snmmary of all trade-off possibilities considered.

(2) The Contractor is not required to notify the Contracting Officer in advance of entering into any subcontract for which consent is not required under paragraph (b), (c), or (d) of this clause.

(f) Unless the consent or approval specifically provides otherwise, neither consent by the Contracting Officer to any subcontract nor approval of the Contractor's purchasing system shall constitute a determination—

(1) Of the acceptability of any subcontract terms or conditions;

(2) Of the allowability of any cost under this contract; or

(3) To relieve the Contractor of any responsibility for performing this contract.

(g) No subcontract or modification thereof placed under this contract shall provide for payment on a cost-plus-a-percentage-of-cost basis, and any fee payable under cost-reimbursement type subcontracts shall not exceed the fee limitations in FAR 15.404-4(c)(4)(i).

(h) The Contractor shall give the Contracting Officer immediate written notice of any action or suit filed and prompt notice of any claim made against the Contractor by any subcontractor or vendor that, in the opinion of the Contractor, may result in litigation related in any way to this contract, with respect to which the Contractor may be entitled to reimbursement from the Government.

(i) The Government reserves the right to review the Contractor's purchasing system as set forth in FAR Subpart 44.3.

(j) Paragraphs (c) and (e) of this clause do not apply to the following subcontracts, which were evaluated during negotiations:

CSRA, LLC

(End of clause)

Section J - List Of Attachments

Attachment J-1	Health Program Support Performance Work Statement
Attachment J-2	Information Technology Security and Privacy
Attachment J-3	World Trade Center Health Program Business Associate Agreement

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE OF PAGES
	1 | 5

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00008	06/01/2018	0000HCCP-2018-23434	

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Centers for Disease Control and Prevention (CDC) Office of Acquisition Services (OAS) 2920 Brandywine Road Atlanta, GA 30341-5539	8219		

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2970 CLAIRMONT RD STE 720 ATLANTA, GA 30329-1638		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
		10B. DATED *(See Item 13)* 11/28/2016
CODE 828268503	FACILITY CODE	

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods:

(a) By completing Items 8 and 15, and returning _____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA *(If required)*

939ZNJH 2512 2018 75-X-0946 W614111601 Increase (b)(4)

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES *(such as changes in paying office, appropriation date, etc.)* SET FORTH IN ITEM 14. PURSUANT TO THE AUTHORITY OF FAR 43.103(b).
	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:
X	D. OTHER *(Specify type of modification and authority)* 52.217-9 Option to Extend the Term of the Contract.

E. IMPORTANT: Contractor ☐ is not, ☒ is required to sign this document and return __1__ copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*

See page 2.

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*		16A. NAME OF CONTRACTING OFFICER	
(b)(4)		Sherrie N Randall	
15B. CONTRACTOR/OFFEROR	15C. DATE SIGNED	16B. UNITED STATES OF AMERICA	16C. DATE SIGNED
(Signature of person authorized to sign)	5/31/2018	BY Sherrie N. Randall *(Signature of Contracting Officer)*	05/31/2018

This modification is issued to

1. Exercise Option Period II, CLINs 2001, 2002, 2002, 2004, 2005, 2006, and 2007 with a period of performace of 06/01/2018 – 05/31/2019
2. Fully fund CLIN 2001 in the amount of (b)(4)
3. Fully fund CLIN 2002 in the amount of
4. Fully fund CLIN 2003 in the amount of
5. Fully fund CLIN 2004 in the amount of
6. Fully fund CLIN 2005 in the amount of
7. Fully fund CLIN 2006 in the amount of
8. Fully fund CLIN 2007 in the amount of
9. Increase the total funded amount of this Contract by (b)(4) from (b)(4) to (b)(4)

All other terms and conditions reamin the same

Section B - Supplies Or Services and Prices/Costs

ITEM	SUPPLIES / SERVICES	QTY / UNIT	ESTIMATED COST	FIXED FEE	TOTAL COST PLUS FIXED FEE
2001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 2 Period of Performance: 06/01/2018 - 05/30/2019	1 Each	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)				

ITEM	SUPPLIES / SERVICES	QTY / UNIT	ESTIMATED COST	FIXED FEE	TOTAL COST PLUS FIXED FEE
2002	Member Svs Medical Benefits Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 2 Period of Performance: 06/01/2018 - 05/30/2019	1 Each	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)				

ITEM	SUPPLIES / SERVICES	QTY / UNIT	ESTIMATED COST	FIXED FEE	TOTAL COST PLUS FIXED FEE
2003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 2 Period of Performance: 06/01/2018 - 05/30/2019	1 Each	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)				

ITEM	SUPPLIES / SERVICES	QTY / UNIT	ESTIMATED COST	FIXED FEE	TOTAL COST PLUS FIXED FEE
2004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 2 Period of Performance: 06/01/2018 - 05/30/2019	1 Each	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)				

ITEM	SUPPLIES / SERVICES	QTY / UNIT	ESTIMATED COST	FIXED FEE	TOTAL COST PLUS FIXED FEE
2005	Finance and Accounting Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2018 - 05/30/2019	1 Each	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)				

ITEM	SUPPLIES / SERVICES	QTY / UNIT	ESTIMATED COST	FIXED FEE	TOTAL COST PLUS FIXED FEE
2006	Claims Adjudications Management All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Period of Performance: 06/01/2018 - 05/30/2019	1 Each	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)				

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	NOT TO EXCEED
2007	Call Center All work to be performed in accordance with the Performance Work Statement. This is a Firm Fixed Price CLIN. Period of Performance: 06/01/2018 - 05/30/2019	1 Job	(b)(4)	
	939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)			

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE	OF PAGES
	1	1

2 AMENDMENT/MODIFICATION NO	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO	5 PROJECT NO *(If applicable)*
00009	06/13/2018		

6 ISSUED BY	CODE	7 ADMINISTERED BY *(If other than Item 6)*	CODE
Centers for Disease Control and Prevention (CDC) Office of Acquisition Services (OAS) 2920 Brandywine Road Atlanta, GA 30341-5539	8219		

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO
KARNA LLC 2970 CLAIRMONT RD STE 720 ATLANTA, GA 30329-1638		9B DATED *(See Item 11)*
	X	10A MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
CODE 828268503 FACILITY CODE		10B. DATED *(See Item 13)* 11/28/2016

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended.
Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods:

(a) By completing Items 8 and 15, and returning ____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12 ACCOUNTING AND APPROPRIATION DATA *(If required)*
N/A

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	
X	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO IN ITEM 10A.
	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES *(such as changes in paying office, appropriation date, etc.)* SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43 103(b).
	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF
	D. OTHER *(Specify type of modification and authority)*

E. IMPORTANT: Contractor ☒ is not, ☐ is required to sign this document and return ____ copies to the issuing office.

14 DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*
This modification is issued to change the Contracting Officer's Representative from:

Carol Lavrich

To:

Andrew Weathers
Phone Number - 404-498-6539
Email Address - aqw5@cdc.gov

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A NAME AND TITLE OF SIGNER *(Type or print)*	16A NAME OF CONTRACTING OFFICER
	Sherrie N Randall

15B. CONTRACTOR/OFFEROR	15C. DATE SIGNED	16B. UNITED STATES OF AMERICA	16C. DATE SIGNED
(Signature of person authorized to sign)		BY Sherrie N. Randall *(Signature of Contracting Officer)*	6/13/18

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE OF PAGES
	1 | 4

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00010	07/12/2018	0000HCCP-2018-24906	

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Centers for Disease Control and Prevention (CDC) Office of Acquisition Services (OAS) 2920 Brandywine Road Atlanta, GA 30341-5539	8219		

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2970 CLAIRMONT RD STE 720 ATLANTA, GA 30329-1638		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
		10B. DATED *(See Item 13)* 11/28/2016

CODE 828268503 FACILITY CODE

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods:

(a) By completing Items 8 and 15, and returning _____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA *(If required)*

See Section B

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES *(such as changes in paying office, appropriation date, etc.)* SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).
X	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF: FAR 52.232-22, Limitation of Funds
	D. OTHER *(Specify type of modification and authority)*

E. IMPORTANT: Contractor ☐ is not, ☒ is required to sign this document and return 1 copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*

Contracting Officer: Sherrie N. Randall, srandall@cdc.gov, 770-488-2866
Contracting Officer's Representative: Andy Weathers, agw5@cdc.gov, 404-498-6539
Contractor Point of Contact: (b)(6)

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*	16A. NAME OF CONTRACTING OFFICER
(b)(6)	Sherrie N Randall

15B.	15C. DATE SIGNED	16B. UNITED STATES OF AMERICA	16C. DATE SIGNED
Paula Monaville	7/16/2018	BY Sherrie N. Randall -S Digitally signed by Sherrie N. Randall -S Date: 2018.07.16 17:15:53 -04'00' *(Signature of Contracting Officer)*	07/16/2018

This modification is issued to:

1. Adjust the cost ceiling for CLINs 1001, 1002, 1004, 1005 and 1006 in Option Period 1 to account for the contractor's estimate to complete the period of performance.
2. De-obligate cost and funding from CLIN 1001 in the amount of (b)(4) to be reallocated to CLINs 1002, 1004, 1005 and 1006. Cost and funding for CLIN 1001 has been decreased by (b)(4) from (b)(4) to (b)(4)
3. Cost and funding for CLIN 1002 has been increased by (b)(4) from (b)(4) to (b)(4)
4. Cost and funding for CLIN 1004 has been increased by (b)(4) from (b)(4) to (b)(4)
5. Cost and funding for CLIN 1005 has been increased by (b)(4) from (b)(4) to (b)(4)
6. Cost and funding for CLIN 1006 has been increased by (b)(4) from (b)(4) to (b)(4)
7. There will be no change in the total cost or funded amount for this contract.

All other terms and conditions remain the same

Section B - Supplies Or Services and Prices/Costs

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
1001	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)			

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
1002	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601 (b)(4)			

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
1004	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601(b)(4)			

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
1005	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601(b)(4)			

ITEM	SUPPLIES / SERVICES	QTY / UNIT	UNIT PRICE	EXTENDED PRICE
1006	Program Support/Data Management All work to be performed in accordance with the Performance Work Statement. This CLIN effort is Cost Plus Fixed Fee. The Fixed Fee for this CLIN is at CLIN 1008. Period of Performance: 06/01/2017 - 05/30/2018	1 Each	(b)(4)	
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601(b)(4)			

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT

1. CONTRACT ID CODE	PAGE OF PAGES
	1 | 3

2. AMENDMENT/MODIFICATION NO.	3. EFFECTIVE DATE	4. REQUISITION/PURCHASE REQ. NO.	5. PROJECT NO. *(If applicable)*
00011	08/31/2018	0000HCCP-2018-28273	

6. ISSUED BY	CODE	7. ADMINISTERED BY *(If other than Item 6)*	CODE
Centers for Disease Control and Prevention (CDC) Office of Acquisition Services (OAS) 2920 Brandywine Road Atlanta, GA 30341-5539	8219		

8. NAME AND ADDRESS OF CONTRACTOR *(No., street, county, State and ZIP Code)*	(√)	9A. AMENDMENT OF SOLICITATION NO.
KARNA LLC 2970 CLAIRMONT RD STE 720 ATLANTA, GA 30329-1638		9B. DATED *(See Item 11)*
	X	10A. MODIFICATION OF CONTRACT/ORDER NO. 200-2016-90996
CODE 828268503 FACILITY CODE		10B. DATED *(See Item 13)* 11/28/2016

11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS

☐ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers __ is extended, __ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods:

(a) By completing Items 8 and 15, and returning _____ copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or telegram which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by telegram or letter, provided each telegram or letter makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified.

12. ACCOUNTING AND APPROPRIATION DATA *(If required)*
939ZNJH 2512 2018 75-X-0946 W614111601

13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.

(√)	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: *(Specify authority)* THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.
	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES *(such as changes in paying office, appropriation date, etc.)* SET FORTH IN ITEM 14. PURSUANT TO THE AUTHORITY OF FAR 43.103(b).
X	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF: FAR 43.103(a) Mutual Agreement of the Parties
	D. OTHER *(Specify type of modification and authority)*

E. IMPORTANT: Contractor ☐ is not, ☒ is required to sign this document and return 1 copies to the issuing office.

14. DESCRIPTION OF AMENDMENT/MODIFICATION *(Organized by UCF section headings, including solicitation/contract subject matter where feasible.)*
See the Summary of Changes on Page 2.

Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.

15A. NAME AND TITLE OF SIGNER *(Type or print)*	16A. NAME OF CONTRACTING OFFICER
(b)(6)	Sherrie N Randall

15B. CONTRACTOR/OFFEROR	15C. DATE SIGNED	16B. UNITED STATES OF AMERICA	16C. DATE SIGNED
(Signature of person authorized to sign)	8/31/2018	BY Sherrie N. Randall -S *(Signature of Contracting Officer)* Digitally signed by Sherrie N. Randall -S Date: 2018.08.31 16:25:17 -04'00'	08/31/2018

Summary of Changes

This modification is issued to deobligate cost and funding from CLIN 2004 to CLIN 2003 to correct the error in the Contractor's business proposal for Operation and Maintenance Support under CLIN 2003 and incorproate the following changes:

1. Decrease the estmated cost of CLIN 2004 by (b)(4) from (b)(4) to $(b)(4). The fixed fee amount of (b)(4) for CLIN 2004 remains the same. Total value for CLIN 2004 has been decreased by (b)(4) from (b)(4) to (b)(4)

2. Increase the estmated cost of CLIN 2003 by (b)(4) from (b)(4) to $(b)(4). The fixed fee amount of (b)(4) for CLIN 2003 remains the same. Total value for CLIN 2003 has been increased by (b)(4) from (b)(4) to (b)(4)

All other terms and conditions remain the same.

Section B - Supplies Or Services and Prices/Costs

ITEM	SUPPLIES / SERVICES	QTY / UNIT	ESTIMATED COST	FIXED FEE	TOTAL COST PLUS FIXED FEE
2003	Technical Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 2 Period of Performance: 06/01/2018 - 05/30/2019	1 Each	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601(b)(4)				

ITEM	SUPPLIES / SERVICES	QTY / UNIT	ESTIMATED COST	FIXED FEE	TOTAL COST PLUS FIXED FEE
2004	Solutions Architecture All work to be performed in accordance with the Performance Work Statement. This is a Cost Plus Fixed Fee CLIN. Option Period 2 Period of Performance: 06/01/2018 - 05/30/2019	1 Each	(b)(4)		
	Line(s) Of Accounting: 939ZNJH 2512 2018 75-X-0946 W614111601(b)(4)				

Attachment #1

C.1 Background

The attacks at the World Trade Center (WTC) on September 11, 2001 exposed rescue and recovery workers, as well as residents/non-responders, to unprecedented risks for injury, illness, and death. Funds were appropriated by Congress to the Centers for Disease Control and Prevention (CDC's) National Institute for Occupational Safety and Health (NIOSH) to provide medical screening for rescue and recovery workers beginning in fiscal year 2002. Additional funds were later appropriated for medical monitoring, diagnostic, and medical treatment services for responders for conditions related to the 9/11 attacks and initial health evaluation, diagnostic, and treatment services for residents, students, and others in the community directly affected by the 2001 WTC disaster.

On January 2, 2011, the President signed Public Law No. 111-347, the James Zadroga 9/11 Health and Compensation Act ("Zadroga Act"), providing a longer term solution to care for the WTC responders and survivors. The Zadroga Act added Title XXXIII to the Public Health Service Act, which authorized the WTC Health Program as a federal health care program for eligible responders and survivors of the 9/11 terrorist attacks. The Zadroga Act can be found at https://www.congress.gov/bill/111th-congress/house-bill/847

The Zadroga Act encompasses a myriad of responsibilities associated with the administration and performance needed to enroll, initially evaluate, medically monitor, and manage allowable health conditions for eligible individuals through diagnostic and treatment services. Responsibilities included the construction of a comprehensive medical and pharmaceutical benefit program consistent with a limited care model; similar in constraints to a worker compensation model, yet requiring the flexibility to engage with advocates and stakeholders. To align with legal requirements, the WTC Health Program promulgated regulations guiding program implementation and created member-based health benefit plans restricted to the conditions certified by the WTC Health Program Administrator for coverage. In addition, a new business processing component was developed to adjudicate and pay medical and pharmacy claims from enrolled providers for allowable services authorized by Clinical Centers of Excellence (CCEs) to manage the care of a restricted set of health conditions for eligible program members. Required administrative processes included procedures for appeals related to denials of program enrollment, health condition certification, or treatment bills. The WTC Health Program had to be operational within 6 months of the passage of the authorizing legislation - by July 1, 2011.

In the spring of 2011, the CDC awarded a contract to provide operational support services to assist NIOSH with implementation of the WTC Health Program as defined by the Zadroga Act. This contract was designed to complement NIOSH activities and a framework of contracted health care entities in the New York City metropolitan area and a large provider network for members across the United States and its territories. Services involved receipt and adjudication of claims for rendered health care, submission of processed claims to another federal entity for payment (as required by the Zadroga Act), assistance with enrollment of new members, assistance with certification (approval) of health conditions of members, and implementation of federal information technology standards for security and privacy. Member and provider support services had to be provided and tailored to the unique needs of the program. Additionally, thecontractor had to be engaged with stakeholders through formalized processes related to Steering Committees established under the Zadroga Act.

Significant challenges have been navigated and lessons learned to implement this complex program, creating a key resource in the form of institutional memory.

As of September 2018, the WTC Health Program had approximately 90,600 members enrolled, including 73,000 responders and 17,600 survivors. The rate at which new enrollees are joining the WTCHP has continued to increase over the life of Karna's HPS contract as shown in Figure 1 below.

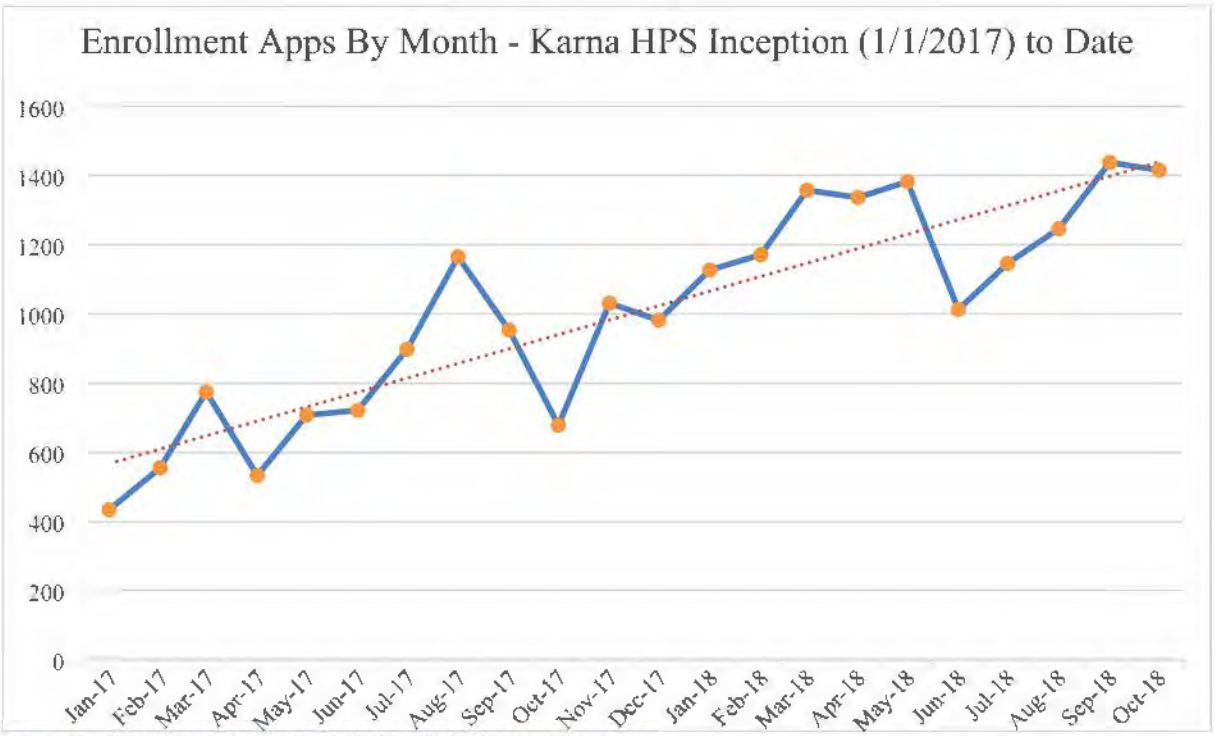


Figure 1 - Enrollment Applications by Month Continue to Increase

Between 1/1/15 and 12/31/15 the WTC Health Program delivered monitoring or screening exams to an estimated 26,800 members, diagnostic services to approximately 17,900 members, and treatment services to approximately 22,600 sick or injured members. The current statistics and reports can be found at http://www.cdc.gov/wtc/reports_portal.html.

In supporting WTC Health Program, the contractor will interact with the Clinical Centers of Excellence (CCEs), the Nationwide Provider Network (NPN) contractor, the Data Centers (DCs) contractor, the Health Program Evaluation (HPE) contractor, and the Centers for Medicare and Medicaid Services (CMS) (to reimburse authorized health care services).

C.2 Project Objective

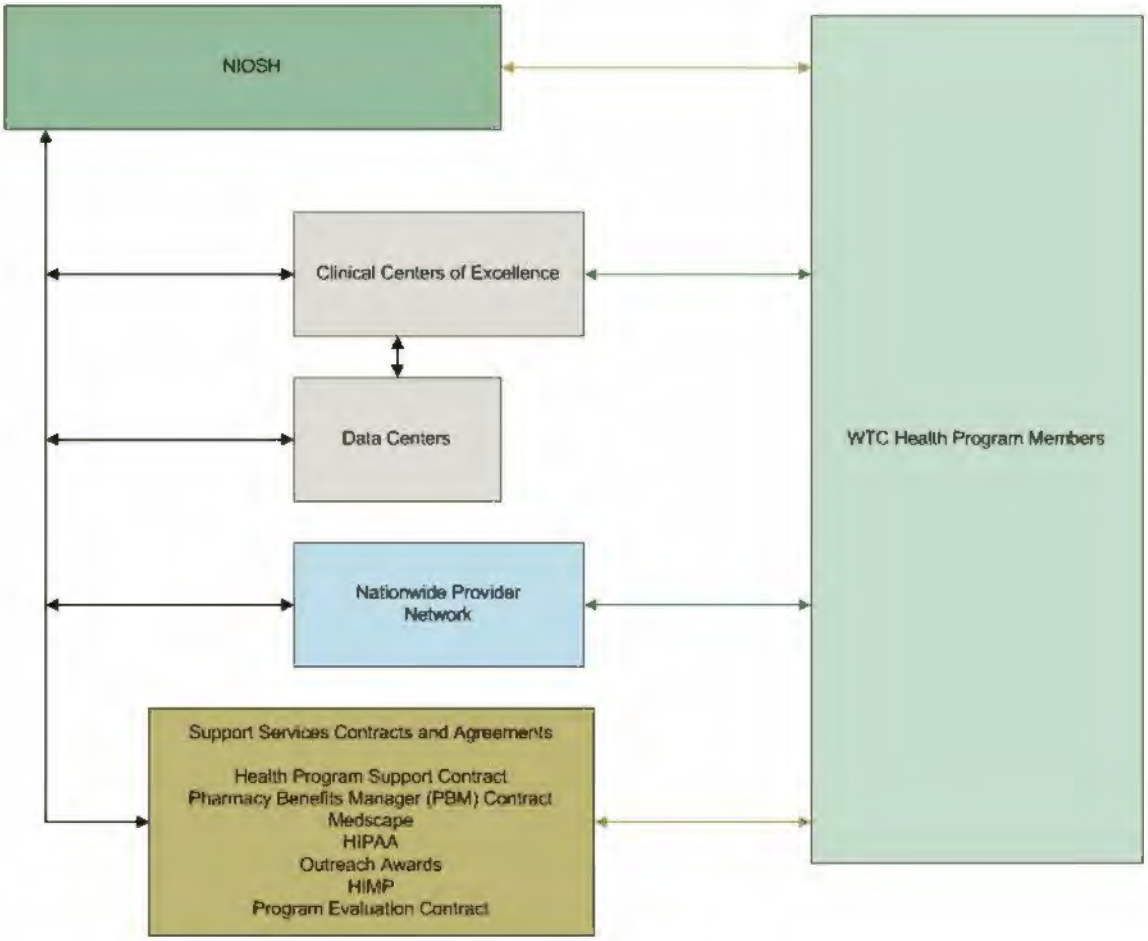
The project objective for this requirement is to provide support services to the WTC Health Program in the areas detailed in C.3 Scope of Work below. These support services will assist NIOSH in the continued operation of the WTC Health Program.

Federally Funded Program

The World Trade Center Health Program is a federally funded health benefits program and therefore must comply with all federal regulations and requirements.

Figure C.2.1 shows a diagram of the components of the WTC Health Program that are relevant to this project. In addition, the HHS Secretary delegated authority to CMS to provide payment services for WTC Health Program patient care claims.

Figure C.2.1 Components of the WTC Health Program


NIOSH
Clinical Centers of Excellence
Data Centers
Nationwide Provider Network
Support Services Contracts and Agreements
Health Program Support Contract
Pharmacy Benefits Manager (PBM) Contract
Medscape
HIPAA
Outreach Awards
HIMP
Program Evaluation Contract
WTC Health Program Members

C.3 Scope of Work

The scope of work for this requirement includes:

- Contract Management
- Member Services and Medical Benefits Support
- Provider Network Management
- Claims Processing and Adjudication
- Case Management
- Data Management, Analytics, Decision Support and Reporting
- Fraud Waste and Abuse Compliance Plan and Reporting
- Quality Assurance and Internal Audits

C.3.1 HPS Contract Management

The contractor shall manage the contract to ensure that all subcontracts, staffing, and services, as well as any transition of those items, meet the requirements of the prime contract. The contractor shall implement project management objectives which focus on monitoring, controlling, and balancing risk related to the project's three key constraints -- scope, budget, and schedule -- on an ongoing basis.

C.3.1.1 Project Management Plan

The contractor shall develop and implement a Project Management Plan (PMP) which shall define appropriate technical and financial project management over all tasks and deliverable documents in accordance with this PWS. The PMP shall establish the staffing and procedural framework for accomplishing the work required under this PWS. The PMP shall establish a transparent management, schedule and budget, baseline from which project monitoring and quality control is routinely conducted to verify adherence to the PMP. The contractor's PMP shall include the proposed labor mix of personnel who will be responsible for implementing this requirement, including an identification of key personnel with the percentage of their time to be devoted to the contract.

A draft PMP shall be submitted with the contractor's proposal. The contractor shall submit a finalized, PMP to the WTC Health Program for approval within 30 days of the contract award according to feedback from the WTC Health Program. Once approved by the WTC Health Program, any changes to the PMP shall be updated as part of the monthly report and an updated PMP shall be submitted annually at the end of each contract year for the duration of the contract.

C.3.1.2 Operations Manual

The contractor shall develop and maintain a transferable (to the government) Operations Manual that contains a complete set of written standard operating procedures (SOPs) which guide all contract-related services, processes, and activities. The SOPs shall be used to meet the objectives outlined in the PWS and provide the requisite security and process support for the WTC Health Program to include evidentiary documentation for formal Federal audits as well as Security Certification and Accreditation of contractor-provided systems for the program. The Operations Manual shall be aligned with the Policy and Procedures Manual(s) for the WTC Health Program. The WTC Health Program Policy and Procedure Manual and Notices can be found at http://www.cdc.gov/wtc/ppm_portal.html. The contractor shall provide support of ongoing updates to the WTC Health Program Policy and Procedures Manual(s).

A draft Operations Manual shall be submitted with the contractor's proposal. The contractor shall submit a finalized Operations Manual to the WTC Health Program for approval within 60 days of the contract award according to feedback from the WTC Health Program. Once approved by the WTC Health Program, any changes to the Operations Manual shall be updated as part of the monthly report and an updated Manual shall be submitted annually for the duration of the contract.

C.3.1.3 Communications Plan

The contractor shall develop and maintain a comprehensive Communications Plan ***(post award)*** that will enable them to deliver appropriate messages or communicate effectively to all identified stakeholders in a consistent and timely manner. The Communications Plan will identify all communication activities among the contractor and the WTC Health Program, the CCEs, the DCs, the NPN contractor, other contractors to the WTC Health Program, the WTC Health Program Steering Committees, and WTC Health Program members.

As a part of the Communications Plan, the contractor shall provide administrative and meeting support to WTC Health Program including hosting conference calls, preparing agendas and action item lists, and recording issue summaries, with key decisions and action items for meeting documentation and make them available to the WTC Health Program in a timely and secure manner. The contractor shall also maintain, and when requested, provide formal program documentation to support any type of Federal Program Audits. The contractor shall provide, as requested and approved by the WTC Health Program, formatting for informational letters and materials and distribute to target audiences (e.g., members, CCEs, NPN, Data Centers); coordinate the review and approval of communication materials; coordinate the printing and mailing of communication materials, as requested and approved by the WTC Health Program; and prepare and provide educational materials on billing practices and procedures including the CCE/Provider Guide to Billing. The contractor shall implement quality assurance measures to ensure communication process efficiency and compliance with federal privacy laws and regulations by testing any automated outputs to ensure accuracy of content and addressee before materials aredelivered.

A draft Communications Plan shall be submitted to the WTC Health Program for review within 60 days of contract award according to feedback from the WTC Health Program. Once approved by the WTC Health Program, any changes to the Communications Plan shall be updated as part of the monthly report and an updated Manual shall be submitted annually at the end of each contract year for the duration of the contract.

C.3.1.4 Data Security and Continuity of Operations Plan

The contractor shall develop and maintain a comprehensive Data Security and Continuity of Operations Plan ***(post award)***. The Data Security and Continuity of Operations Plan shall outline the contractor's policies and procedures for ensuring data security, continuity of operations, and the transfer of data to the WTC Health Program. The data security and continuity of operations plan should follow standard FISMA, HIPAA, and CDC guidelines. The Data Security and Continuity of Operations Plan shall follow the requirements of a certification and accreditation process and allow for continuous monitoring as outlined by CDC's Office of the Chief Information Officer (OCIO). A draft Data Security and Continuity of Operations Plan shall be submitted to the WTC Health Program for review within 60 days of contract award. As stipulated by HHSAR 352.222-70, the draft plan shall include identification of system of records and disposition as listed below:

> (a) Identify the system(s) of records and the design, development, or operation work the contractor is to perform and provide detailed description and point of contact (team or group) to the WTC Health Program;
>
> (b) Specify the disposition to be made of such records upon completion of contract performance.

The contractor shall transfer all government records and all government furnished data within 30 days after the contract has ended or within the (to be) specified transition period. (Further details can be found in Attachment J-2, Information Technology Security and Privacy).

The contractor shall submit a finalized Data Security and Continuity of Operations Plan 2 weeks from receipt of comments by the WTC Health Program. Once approved, an updated Data Security and Continuity of Operations Plan shall be submitted to the WTC Health Program monthly for the duration of the contract. (See Attachment #2, Information Technology Security and Privacy and PWS Section on Health Insurance Portability and Accountability (HIPAA) Act compliance for more information.)[1]

C.3.1.5 Risk Management Plan

The contractor shall prepare a Risk Management Plan ***(post award)*** in accordance with SEI (Software Engineering Institute) Risk Management Methods (or equivalent) and CDC policy for a continuous improvement approach to project performance. The Risk Management Plan shall include any potential program integrity risks and proposed strategies to reduce/mitigate these risks.

A draft Risk Management Plan shall be submitted to the WTC Health Program within 60 days of contract award. The contractor shall submit a finalized Risk Management Plan 2 weeks from receipt of comments by the WTC Health Program. Once approved by the WTC Health Program, any changes to the Risk Management Plan shall be updated as part of the monthly report and an updated Plan shall be submitted annually at the end of each contract year for the duration of the contract.

C.3.1.6 Fraud, Waste, and Abuse Compliance Plan

The contractor shall prepare a Fraud, Waste, and Abuse (FWA) Compliance Plan ***(post award)***. As part of the FWA Compliance Plan, the contractor shall designate executive and essential personnel to attend mandatory training in FWA detection, prevention, and reporting, and designate an officer or director in its organization with responsibility and authority for carrying out the provisions of the FWA Compliance Plan.

A draft FWA Compliance Plan shall be submitted to the WTC Health Program within 60 days of contract award. The contractor shall submit a finalized FWA Compliance Plan 2 weeks from receipt of comments by the WTC Health Program. Once approved by the WTC Health Program, any changes to the FWA Compliance Plan shall be updated as part of the monthly report and an updated Manual shall be submitted annually at the end of each contract year for the duration of the contract. (See Section C.3.6 for more details about the Fraud, Waste, and Abuse requirements.)

C.3.1.7 Quality Assurance and Internal Audit Plan

The contractor shall prepare a Quality Assurance and Internal Audit Plan. A draft Quality Assurance and Internal Audit Plan shall be submitted with the contractor's proposal. The contractor shall submit a finalized Quality Assurance and Internal Audit Plan within 60 days of contract award. Once approved by the WTC Health Program, any changes to the Quality Assurance and Internal Audit Plan shall be updated as part of the monthly report and an updated Manual shall be submitted annually at the end of each contract year for the duration of the contract. (See Section C.3.7 for more details about the Quality Assurance and Internal Audit requirements.)

C.3.1.8 Cost Tracking and Invoicing

The contractor shall develop and manage budgets for all work to be performed. Budget development and management includes tracking and analysis of costs and reporting budget-related issues to the Contracting Officer (CO).

The contractor shall submit monthly invoices with its Monthly Report which reflect adherence to budget and funding requirements. Monthly invoices shall be itemized and provide appropriate detail and backup to reflect reasonable and justifiable costs allocated to activities associated with its approved budget such as program management, member enrollment, certifications, provider enrollment, case management, claims processing, IT support, and other support duties.

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[1] Please refer to FISMA compliance section for further details. IT Security and SA&A process is constantly evolving and subject to change with additional timeline and documentation. The successful award is advised to discuss the SA&A process with the WTC Health Program as soon as the award is made.

In addition to the above, the contractor shall provide quarterly a WTC Health Program invoice Reconciliation Report on the overall financial status of the contract. This report shall include detailed budget projections for the upcoming two quarters, including burn rate.

C.3.1.9 Subcontracting

The HPS contractor shall provide management and oversight of all subcontractors[2] as described in FAR Parts 42 and 44. This would include post-award conferences with subcontractors that involve the Contracting Officer and Contracting Officer's Representative.

The HPS contractor shall provide a subcontracting plan with their proposal to inclnde all subcontracts to the Contracting Officer (CO) and Contracting Officer's Representative (COR) prior to award and any time during the period of the contract where subcontracts are added or modified. The HPS contractor shall manage any required subcontracts in a manner that best fulfills the requirements of the prime contract. The HPS contractor shall ensure its subcontractors and/or their agents who reqnire the use of or access to individually identifiable information or protected health information under the provisions of this contract comply with WTC Health Program bnsiness associate agreements and federal law, policies and regulations, including FISMA, HIPAA, and CDC certification and authorization to operate requirements. The prime contractor will obtain a written assurance from each subcontractor to this effect. The HPS contractor shall notify the COR and CO in advance of placing any subcontract or modification, including the following information:

(i) A description of the supplies or services to be subcontracted.
(ii) Identification of the type of subcontract to be used.
(iii) Identification and selection of the proposed subcontractor.
(iv) A description of an oversight and quality assurance plan.

C.3.1.10 Change Control Process

The contractor shall manage the contract to ensure the required services are being performed and the required contract deliverables are being submitted. As part of this requirement, the contractor shall maintain a WTC Health Program-approved change control process to identify, record, assess and secure approval for changes to the project. The contractor and the WTC Health Program will follow this process to classify, prioritize, approve, or reject changes, so that implementation does not negatively impact core mission applications, functions, or live production environments. Change requests need to be defined clearly, including their cost and schedule implications, to allow the WTC Health Program to make appropriate decisions. The contractor shall receive prior authorization and approval of expenditures by the WTC Health Program before starting any work on changes. Further guidance on the Change Request process will be provided upon the award.

C.3.1.11 Steering Committees Updates

The contractor shall provide a monthly status which includes information on items such as enrollment, transfers, and medical and pharmacy claims to the WTC Health Program Steering Committees for monthly committee meetings. The contractor shall prepare the status in the form of an approved PowerPoint presentation. The contractor shall be requested to attend up to four steering committee meetings annually held in Manhattan, New York and/or Brooklyn, New York.

C.3.1.12 Technical Expert Support

The contractor shall identify and provide personnel with appropriate technical expertise to perform required technical advisory support to the WTC Health Program.

[2] Pursuant to FAR 44.101, Definitions, a "Contractor" means "the total contractor organization or a separate entity of it, such as an affiliate, division, or plant that performs its own purchasing." A "Subcontractor" means "any supplier, distributor, vendor, or firm that furnishes supplies or services to or for a prime contractor or another subcontractor."

The contractor's technical experts shall include a cost-effective mix of specialists in Federal Health Care Delivery Policy and Procedure, End-to-End Health Care Claims Processing, Medical Billing and Coding, Medical Review and Utilization, Federal Health Information Technology, Federal Legislative/Regulatory Affairs as pertains to Federal health care system and benefit design, Provider Network Management, Education and Relations, Risk Management and Health Communication. The contractor shall have appropriate consultative and operational expertise standing up and supporting high-visibility federal healthcare programs that require exceptional end-user responsiveness. Technical experts shall be familiar with the Zadroga Act, Affordable Care Act, Federal Employees' Compensation Act, Social Security Act (Title XVIII), HIPAA, and other related federal health laws and regulations and maintain knowledge and skills needed to administer a targeted federal health care program.

The contractor shall provide technical assistance, as requested, to develop updates on medical policies and procedures upon approval by the WTC Health Program. The contractor shall conduct training on medical billing, pharmacy benefit support and other WTC Health Program procedures for the CCEs and NPN contractor. The contractor shall provide Quality Assessment/Quality Control analyses on the claims adjudication system to ensure it is functioning according to intended benefit policy. The contractor shall review, analyze, and maintain the WTC Health Program Code Books and programmed Care Suites to adjudicate claims for medical care across the WTC Health Program. The Care Suites enable benefit plans to be assigned which agree with program requirements for targeted care. The contractor shall update and maintain the WTC Health Program Code Book in three different forms – one appropriate for the claims adjudication system integration, one for the NPN contractor, and the third for the CCEs to utilize for benefit management. The contractor shall review, analyze and prepare formal recommendations for coverage of new condition codes with appropriate mapping to medical benefit plan(s) (i.e., Care Suite logic), health care service codes and procedures, durable medical equipment codes, pharmaceutical products with appropriate mapping to pharmacy benefit plan(s), and other medical services/supplies (i.e., ICD-10, ICD- O9, CPT, HCPCS, DME, RCC). The contractor shall perform medical review, service utilization reviews (claims and onsite chart review) and other medical quality assurance activities.

The contractor shall designate a position to oversee case management services. The individual serving in this function shall be experienced in practicing occupational medicine, internal medicine or family medicine (e.g., conducting occupational health screening and medical surveillance); preferably a board-certified specialist. The position may be filled by a Medical Director or Clinical Director or other suitable candidate the offers finds will appropriately execute the requirements of this effort. The HPS shall develop training to properly instruct the case management team about the potential exposures and health outcomes that have been associated with the WTC attack and the conditions and services covered by the WTC Health Program.

C.3.1.13 Program Improvement Plan

The contractor shall prepare a Program Improvement Plan ***(post award)*** which will support the design, implementation, and evaluation of a spectrum of services focusing on improving program performance. The Program Improvement Plan shall include a periodic operational assessment to evaluate total program performance and effectiveness of policies, procedures and workflows and compliance with the WTC Health Program Policies and Procedures Manual. A draft Program Improvement Plan shall be submitted to the WTC Health Program within 60 days of contract award. The contractor shall submit a finalized Program Improvement Plan 2 weeks from receipt of comments by the WTC Health Program. Once approved by the WTC Health Program, any changes to the Risk Management Plan shall be updated as part of the monthly report and an updated Plan shall be submitted annually at the end of each contract year for the duration of the contract.

The contractor shall stay abreast of evolving federal rules and industry innovation in the health care arena, and proactively engage with NIOSH to propose, facilitate and develop program improvements which address the needs of beneficiaries, the CCEs, the DCs, the Scientific/Technical Committee, Steering Committees,

members of the academic communities and other stakeholders. The contractor shall work closely with NIOSH to provide full program integrity including Fraud, Waste and Abuse considerations. The contractor shall address all the required mandates of the Zadroga Act and accompanying regulations and policies, and incorporate approved innovations or enhanced means of providing superior care and service delivery to program participants and stakeholders. The contractor's plan shall demonstrate their understanding of NIOSH's requirement to address Congress and other government organizations by providing program transparency, contractor efficiencies in performance, as well as reduced performance risk.

C.3.1.14 Reporting

The contractor shall provide communications and reports to the WTC Health Program regarding ongoing implementation and operational status as well as routine reports documenting program progress and risks related to program management and administration, including critical-success-factors and key-performance-indicators. In addition to the weekly updates of the Project Management Plan, the contractor shall submit accurate and detailed reports as required by this PWS. (See Section C.6, Deliverables for more information on reporting requirements.)

C.3.2 Member Services and Medical Benefits Support

The contractor shall provide technical and administrative support to the WTC Health Program in the areas of member services and medical benefits management. These services include, but are not limited to, member enrollment, medical condition certification, credentialed provider enrollment/relations, nurse advice line and supplemental case management upon request by a CCE/NPN, and the provision of a secure location for WTC Health Program information submission, a secure web portal, and a call center. (See Section C.3.3.2, Secure Location for WTC Health Program Information Submission and Section C.3.3.3, Secure Web-based Information Exchange System for more information.)

C.3.2.1 Medical Condition Certification

The contractor shall process certification requests from CCEs and the NPN contractor which allow a member's qualifying health condition to be treated using WTC Health Program funds. As a part of the process, the contractor shall notify the CCEs of the status of certifications (approved, denied, pended, or un-process able), update member records with health condition certification information, notify members of certification decisions and, in the case of denials of health condition certifications, notify the member with instructions about appeal rights. The contractor shall also manage member requests for appeals and coordinate with the WTC Health Program Appeals Coordinator to establish a complete case file within the WTCHP System of Record and provide weekly certification reports to the CCEs/Data Centers. An average of 1027 (Nov2017 – Oct2018) and 1076 (Year to Date Jan-Oct 2018) administrative requests to certify a member's medical condition for treatment are processed each month. This average reflects an ongoing upward trend with the average over Aug-Oct 2018 being 1312 certification requests per month. Figure 2 below shows monthly Certification Requests received during the period Jan 2017 – Oct 2018 along with a trend-line showing the increase in Average monthly receipts over that time. (See the World Trade Center Health Program website for more information: http://www.cdc.gov/wtc/index.html).

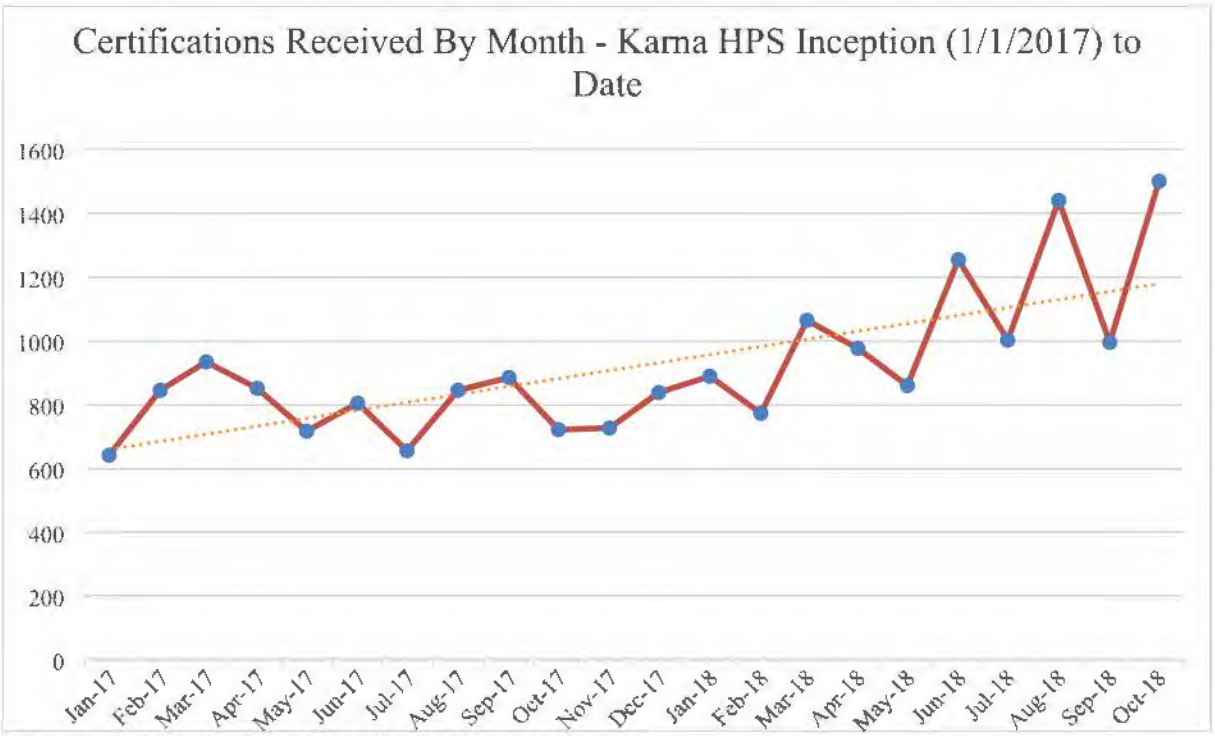


Figure 2 – Certification Requests per Month Continue to Increase

C.3.2.2 Care Coordination and Case Management

After January 1, 2019, the contractor shall support NIOSH in monitoring care coordination and case management services provided by the CCEs or the NPN contractor to promote consistency and quality of services across the Program.

All contractor case management and care coordination activities associated with active member care were transitioned back to the CCEs and NPN effective 12/31/2018. The following section defined the contractor's scope of effort prior to January 1, 2019:

> *The contractor shall provide supplemental care coordination and case management for particular members as requested by CCEs or the NPN contractor. The major difference in case management of the HPS Contractor vs. that of the CCEs and NPN contractor is the HPS Contractor's capability to provide and manage case management service for severely ill and complexed cases at all phases of care continuum including post-acute care and hospitalization. The contractor's proposal should demonstrate experience providing case management services to assist with cancer care navigation, after-hours nurse advice line to support members with complex physical and mental health conditions, and technical support to providers regarding service authorization limitations.*
>
> *The contractor's case management system and staff shall have the capability to provide comprehensive clinical support to members including:*
> *Diagnosis education, treatment options, and strategies to manage symptoms and side effects*

Access to evidence-based care (consistent with NCCN Guidelines®)
Complication management
End-of-life planning, hospice and psychosocial support to both patients and caregivers
After-hours access to a nurse advice line or case management team, with documented linkage back to a member's CCE/NPN. The call center shall include access to case management service seven days per week/24 hours a day.

The HPS's Case Management staffing plan shall include the professional qualifications and skills required for clinical case management personnel, clinical coordination, appropriate recognition and handling of urgent or emergent psychiatric or medical situations, managing conflict, and administrative problem solving. The Case Management staffing plan shall be led by the Medical Director and certified registered nurse case manage at the management and team lead level with job description and required credential included in the PMP's HPS staffing plan.

The case managers should be responsible for maintaining excellent communication with the member's home CCE or NPN, and member's Patient Centered Medical Home (to notify this provider of evaluations and treatment performed throagh the WTC Health Program and to notify them about conditions and care that is not covered by the WTC Health Program but will need to be addressed by the Patient Centered Medical Home provider). The case managers should also have a system/process to deal with emergency/high-risk members and cases (e.g., multiple hospital readmissions, psychiatric emergencies, emergency discharge planning, emergent social issues, etc.). The case managers should also have a system/process for smoothly dealing with Hospitalizations, Skilled Nursing Facilities, Extended Care Facilities/Plans (for both physical and mental health conditions covered by the WTC Health Program), Rehabilitation Facilities, Home Health Care, Hospice Care and End of Life Care.

The HPS Case Management plan shall indicate knowledge and understanding of the WTC Health Program member population and ensure continuum of care is being coordinated from pre-acute, acute to post-acute phases once the case is referred. The HPS case management process shall ensure all referrals and referrals of prior referrals are coordinated and service summary reports are maintained. All referred rendering and billing providers shall be registered with the WTC Health Program prior to billing and must be informed of the prior approved services needed (in a non-emergency situation).[3]

C.3.3 Healthcare Provider Network Management

The contractor shall establish and maintain an adequate health care provider network on behalf of the WTC Health Program. This network shall be managed by the contractor, but the network arrangement (as constructed) shall be the property of the WTC Health Program. The contractor shall act on behalf of the WTC Program Administrator to manage the provider network and the providers shall be selected on the basis of their credentials and experience treating or diagnosing the health conditions covered by the WTC Health Program.

This network may include, but not be limited to, hospitals and ancillary service providers, specialty clinics, trauma centers, specialists, and primary care physicians, in sufficient numbers and at locations both safe and convenient to the member population.

The providers who will be conducting the standardized medical monitoring examinations shall be experienced in diagnosing and treating occupational and/or environmental disease and conducting medical surveillance; preferably board-certified physicians with a specialty in occupational, internal, or family medicine. The contractor shall work with the CCEs to identify providers to add to the network. The contractor shall also proactively identify providers to be added to the network. The contractor shall provide training to properly instruct the participating WTC Health Program providers as to the requirements of participation (i.e., fee structure, billing limitations, and authorization process), information regarding 9/11 exposure and health effects, and the limited care model of the program.

[3] Further policy guidance on post-acute care case management is forthcoming.

The contractor shall be responsible for ensuring that the health care providers who provide diagnostic and treatment services meet all requirements of the contract, including verification of appropriate credentials.

As the Provider Network Manager, the contractor shall ensure all providers are appropriately credentialed and have an approved network enrollment agreement in place. The contractor's provider verification services shall include a provider screening solution which continuously monitors all actively enrolled providers and related parties, generates alerts based on changes in risk factors, and automates the end-to-end provider screening process during all enrollment and re-enrollment periods.

The contractor shall ensure that all provider network agreements specify that this is a limited care benefit program, the fee ceiling is set by FECA (see "a" below), services must be authorized by the member's CCE/NPN before the claim will be processed, medical records shall be requested to support a claim for services, and that the balance billing of a member is not allowed. The contractor shall provide a secure means for electronic claims authorization.

The fee schedule (described below) and other requirements are described and updated in the WTC Health Program Policy and Procedures Manual that can be found at http://www.cdc.gov/wtc/ppm_portal.html

a. The Zadroga Act requires that the payment rate for treatment under the WTC Health Program be in accordance with the rates paid under the Federal Employees' Compensation Act (FECA), administered by the U.S. Department of Labor (DOL). For treatment not covered under FECA, the payment rate is pursuant to the applicable Medicare fee for service, as deemed appropriate by the Administrator. More information about FECA, including the methodologies used to calculate FECA rates, can be found at http://www.dol.gov/owcp/regs/feeschedule/fee.htm
Maximum payments under the WTC Health Program are the lesser of the calculated fee schedule amount or the billed amount.

b. The WTC Health Program also follows DOL's medical fee schedule used by the FECA program for covering inpatient hospital services. These services are subject to a reimbursement schedule based on the Medicare Inpatient Prospective Payment System (IPPS). That system assigns services to Diagnosis-Related Groups (DRGs) and adjusts rates for individual hospitals according to their specific cost index. Inpatient services not covered under the Medicare IPPS are reimbursed under a formula that is based on the cost-to-charge ratio (CCR) data tables published by CMS for rural and urban hospitals in each state. The full explanation of how fees are calculated under FECA may be found at:
http://www.dol.gov/owcp/regs/feeschedule/fee/fee12/READ_ME_FIRST_fs12_instructions.htm

Licensing and Insurance

The contractor shall ensure that all providers in the provider network are properly licensed under applicable state laws and/or regulations and are properly insured. Licensing and insurance documentation shall be reviewed at a frequency that meets industry standards.

Training Records

The contractor shall ensure that all monitoring or screening providers are properly trained in coordination with CCEs, DCs and the NPN contractor in the conduct and documentation of the standardized health exams.

The contractor shall ensure that all providers are educated regarding the limited care nature of this program, service authorization requirements, fee limitations, billing and dispute pathways, treatment or finding summaries for CCE/NPN or program staff. This includes initial and ongoing training. All training records shall be maintained and verified as part of the quality assurance plan. The contractor shall clearly describe the Provider Network Management process including automated solutions and quality assurance

plan in the Operations Manual.

C.3.3.1 Provider Enrollment

The CCEs and NPN contractor will identify medical providers based on the type of care their certified members require and will request that the contractor acquire provider enrollment information. Enrollment elements shall include, but are not limited to, the provider's National Provider Identifier (NPI), tax ID, practice address, and a signed form (e.g., Electronic Funds Transfer (EFT)/CMS-588 form) authorizing the enrollment of financial/banking information. The enrollment information shall be reviewed by the contractor prior to database entry, and if there are missing data elements or other questions, the contractor shall work closely with the CCEs, the NPN contractor, and providers to obtain the required information. The provider NPI and name shall be verified by the contractor against a NPI registry such as the National Plan & Provider Enumeration System (NPPES), administered by the Centers for Medicare and Medicaid Services(CMS). The routing number is also verified against a trusted routing directory, such as the Federal Reserve Bank Services (FRBS) repository. In addition to required data elements, the contractor shall capture information that further distinguishes providers, such as authorization to prescribe to members, special case or CCE agreement status, and the provider's medical specialty.

The contractor shall work with the CCEs and/or the NPN contractor to identify provider additions to the network. The contractor shall also proactively identify providers to be added to the network.

There are currently approximately 280 different provider specialties which include cancer care, medical and surgical specialty care, ancillary support (e.g., anesthesiology, pathology, radiology, etc.), organ transplantation services, emergency care services, skilled nursing care, durable medical equipment (DME) providers, and inpatient service facilities, among others. The number of enrolled providers currently includes approximately 16,000 unique providers. As an indicator of the growth in Provider Network enrollment activity, **Error! Reference source not found.** below shows the number of new providers enrolled to the WTC HPS by month from inception of that service on Karna's HPS contract (4/1/2017) to date.

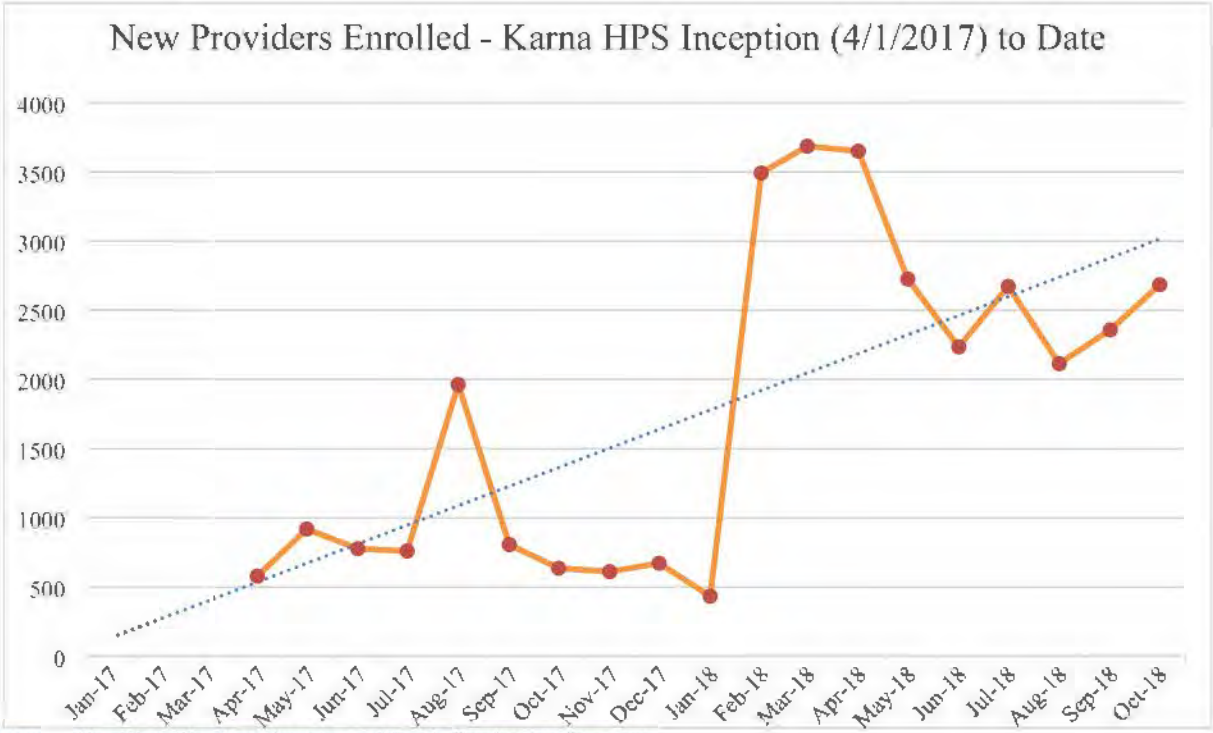


Figure 3 - New Provider Enrollments per Month Continue to Increase

The contractor shall produce a shared provider list of all enrolled providers and make it available for CCE and NPN use. The contractor shall employ an automated electronic solution to enhance efficiency in managing the provider network. Provider enrollment into the WTC Health Program and the management of a reliable provider network are critical to efficient claims processing. The complete and accurate enrollment of providers ensures that claims are not suspended for review of provider validity issues, and that claims may be promptly adjudicated.

Annual Confirmation of Authorized Providers

The contractor shall ensure that the list of authorized providers in the WTC Health Program Provider Network is accurate and up-to-date. At the minimum, the contractor shall annually distribute a comprehensive listof enrolled providers to each CCE and/or the NPN contractor, which the CCE and/or the NPN contractor must review and make any updates or note any discrepancies between the providers held on record by theWTC Health Program and those held on record by the CCE and/or the NPN contractor. The contractor is responsible for working with the CCE and/or the NPN contractor to resolve any discrepancies in a timely fashion.

C.3.3.2 Secure Location for WTC Health Program Information Submission

The contractor shall provide a secure location for secure submission of applications for enrollment of members, requests for certification of health conditions, appeals of decisions by the WTC Health Program, and other communications from members and providers. The location should support the X12N Electronic Data Interchange (EDI), Secure File Transfer Protocol (SFTP), a secure, web-based information exchange systemusing Secure Socket Layer (SSL) protocols, and other solutions that are acceptable based on the type of data being transferred. The secure location shall serve as a centralized mailing center for written WTC Health Program communications or claims (inbound & outboundmail).

C.3.3.3 Secure Web-based Information Exchange System

The contractor shall provide a secure, web-based information exchange system that shall offer role-based easy- to-navigate menus for government users, HPS support teams, the CCEs and NPN, and providers. The secure web information exchange system must be highly configurable to address the unique needs of the WTC Health Program.

The information exchange system shall support all standard browsers (Microsoft Edge, Internet Explorer, Firefox, Safari, Chrome). Access shall be encrypted through 128/256-bit HTTPS/SSL certificates and authorized through the system's configurable HIPAA-compliant role and group-based security interface. CCEs and the NPN contractor shall be able to view the status of their claims and approve claims for services rendered on their behalf by other providers, which may entail information processing across CCE/NPN-customized authorization systems.

The information exchange system shall comply with all sections of the Americans with Disabilities Act (ADA), accessibility standards established under section 508 of the Rehabilitation Act (standards that provide greater accessibility for individuals with disabilities), the Older Americans Act, the Web Content Accessibility Guidelines WCAG 2.0, FISMA, and federal civil rights laws

The secure, self-service web information exchange system shall, at a minimum, address the needs of the CCEs, the NPN contractor, providers, and the WTC Health Program Administration Office (NIOSH).

Provider Web Information Exchange System

The purpose of the provider web information exchange system is to provide a self-guided user interface to enter or track claims/authorizations and download program content, such as Code Books. The information exchange system shall also provide self-administration screens that allow providers to change information, such as mailing addresses.

The provider web information exchange system shall offer a single point of entry for all provider-related functions. Information captured in the system's provider database shall be capable of fully supporting the WTC Health Program in monitoring and maintaining its enrolled providers to ensure that WTC responders and survivors have access to high-quality, cost-effective, and effectively managed health care. The secure provider web information exchange system shall have the capability for the online entry, verification, and update of provider information to ensure that only qualified providers who comply with program rules and regulations are reimbursed for services rendered to eligible program beneficiaries.

The secure provider web information exchange system shall be available 24/7 from any computer and allow access to self-service features including verification of WTC Health Program member eligibility, online entry of claims, inquiry of claim and payment status, electronic funds transfer (EFT) capabilities, 1099 download capabilities, secure inbox-messaging and computer-based training (CBT). The provider web information exchange system shall also facilitate the WTC Health Program's ability to communicate with providers and to receive valuable feedback that can be used to improve services.

The secure provider web information exchange system shall integrate user-configurable and detailed rules, processes, and procedures for the maintenance of all required provider information. Real-time entry, verification, and updating of provider-related information ensure that it is accurate and readily available for provider inquiry and system processes such as claims adjudication, claims payment, and reporting. This information can also assist the WTC Health Program in monitoring and maintaining its provider network to ensure that all program beneficiaries have access to high-quality, cost-effective, and effectively managed health care.

Program Administration Web Information Exchange System

The program administration web information exchange system shall provide a dashboard view into the overall system that includes system information such as claims, enrollments, and certifications on all

members. This provides insight to NIOSH staff about internal operations and will play an important role in decision making. WTC Health Program-authorized users shall be able to run standard or customized reports and download or print report results. The secure web information exchange system's reporting capability must be highly configurable to address the unique needs of the WTC Health Program.

C.3.3.4 Call Center

The contractor shall maintain a call center to receive and respond to inquiries from potential enrollees, members, and providers. The contractor shall also provide escalation as needed for properly responding to inquiries. The WTC Health Program call center currently receives approximately 260 calls per day. The contractor's proposal shall support up to 300 calls per day and define the hours of operation and other operation details (for example, translation services) about the call center. Figure 4 shows how the average calls per day to the contractor's call center continues to increase over time.

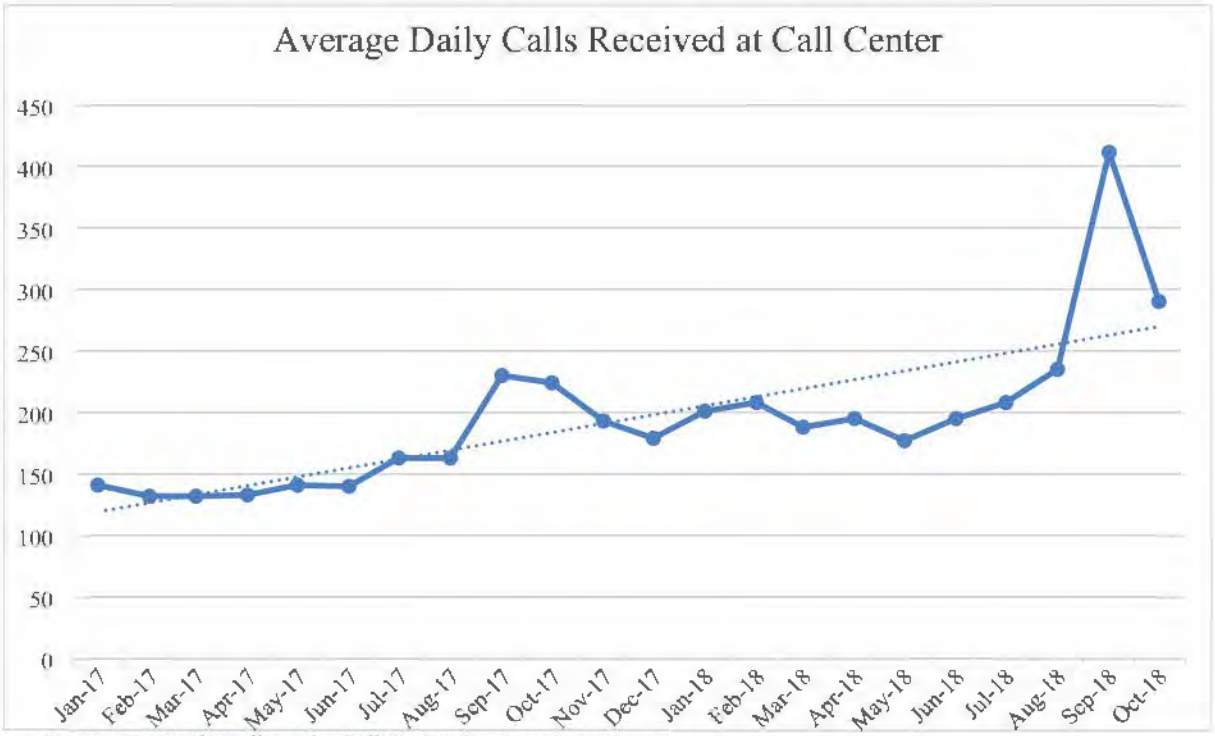


Figure 4 - Average Daily Calls to the Call Center Continue to Increase

The contractor's call center shall have the ability to capture a broad range of data to track and measure inquiry disposition, including the capture and retention of various program-specific data. These data include caller name, applicant name, date and time of the contact, results of the call, inquiry status, inquiry type (Enrollees, Members, and Providers), method of inquiry receipt, method of inquiry response, follow-up assignments and related transfers and escalations, follow-up and closure details, and FAQs. This data needs to be protected to be fully compliant with required HIPAA and FISMA security standards. The call center shall have the ability to link duplicate inquiries made to the call center in a seamless manner to support call flow. Whether the customer contact is a new or repeat inquiry, the call center systems shall enable call center personnel to establish any necessary follow-up, escalation, or callback activities.

Call center personnel shall have the aptitude to respond directly to contacts in both English and Spanish. For other languages, the use of a company that provides translation and interpretation services for inbound and outbound communications, both written and verbal, is acceptable.

C.3.4 Claims Processing

The contractor shall provide a complete array of claims-related services including claims receipt, claims

adjudication, submitting claims to another entity for payment, and assistance with claims reconciliation. The contractor shall assess and provide a high volume of technical experts that may be necessary during the transition phase to be operationalized within a timely manner. The contractor shall work closely with the WTC Health Program, CMS, the CCEs, the NPN contractor, and other stakeholders of the program to ensure interoperability of the current, diverse and unique systems being used by the incumbent contractors to avoid major development work; enhance capacities and capabilities; and minimize cost while maximizing efficiency. The monthly volume of claims received by the HPS contractor for processing and adjudication is currently approximately 36,000 and continues to increase as shown in Figure 5.

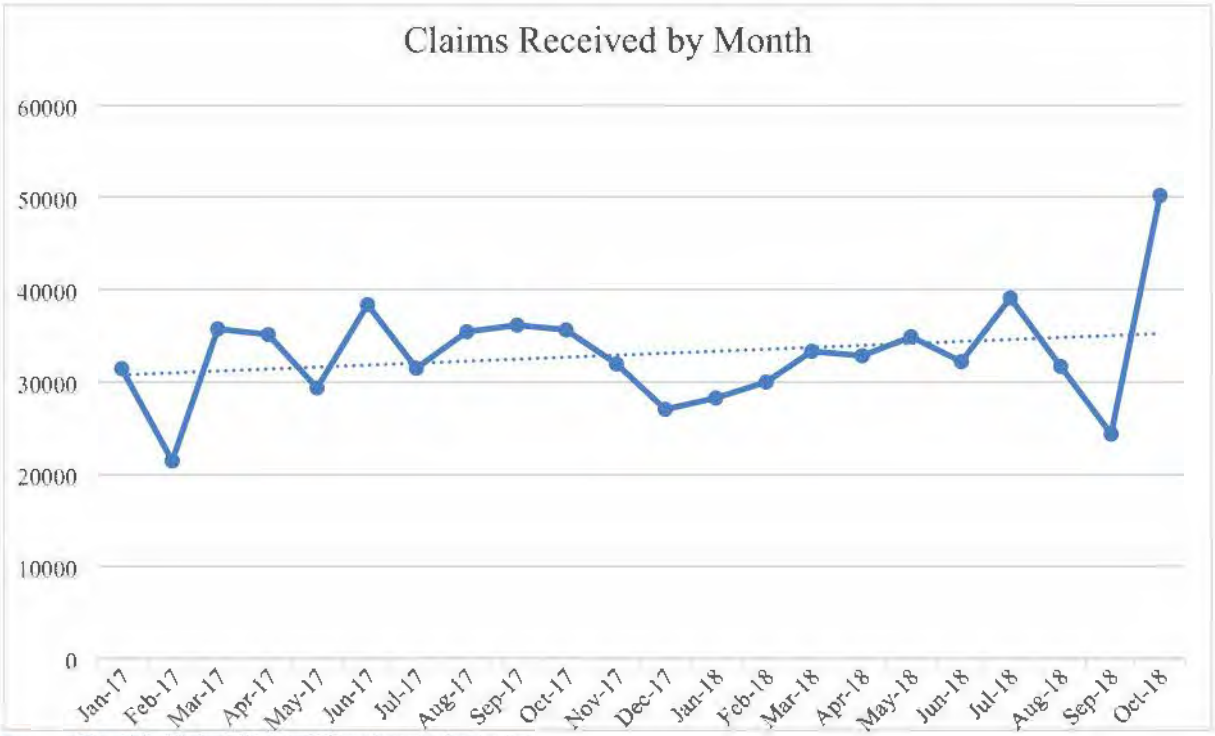


Figure 5 - Monthly Claims Receipts Continue to Increase

The contractor shall conduct ongoing reviews of claims to detect and resolve fraud, waste, and abuse (FWA) incidents before and after claims adjudication, taking into account the return on investment of the FWA activities. See Section C.3.6 for more information on Fraud, Waste, and Abuse activities.

The contractor's FWA activities shall provide a proactive deterrent to FWA which provides for greater containment of health care costs through the recovery of payments for erroneous billings and deterrence of fraudulent behaviors. The contractor's FWA solution shall be fully integrated with their claims processing solution and use current and historical data from that system for FWA detection analysis. The FWA system should be able to conduct advanced analytics, mine the data, and learn from the data in order to find new patterns of FWA.

C.3.4.1 Electronic Data Exchange (EDI) and Paper Claims

The contractor shall receive ANSI-X12 837 files for ICD-10 electronic data interchange (EDI) claims from CCEs and the NPN contractor, as well as either EDI or paper claims (on an approved exception basis) from the network of providers that are external to the CCEs. The contractor shall create EDI claims from paper claims when submitted.

The contractor's system shall fully comply with HIPAA Privacy and Security standards and related ANSI-X12 transactions standards for electronic Protected Health Information (ePHI) transmission.

C.3.4.2 Authorization of Services by CCE or NPN

The contractor shall ensure that all medical claims are appropriately authorized by the assigned program CCE or NPN (based on member assignment) before service claims are further adjudicated.

C.3.4.3 Medical Claims Adjudication

The contractor shall adjudicate medical claims using a system that meets the needs of the WTC Health Program, covering the following:

- Professional and Institutional
- Limited set of health conditions that are covered and certified (approved) by the WTC Health Program and certified (approved) for members
- Limited durable medical equipment
- Plan service limits and pre-authorizations for certain procedures
- Pricing at a maximum of Federal Employees' Compensation Act (FECA) rates and some pricing at Medicare rates, including Diagnosis Related Group (DRG) pricing
- Pricing for limited dental coverage using 85% of the American Dental Association fee structure
- National Correct Coding Initiative (NCCI) edits
- National Comprehensive Cancer Network Guidelines http://www.nccn.org/professionals/physician_gls/f_guidelines.asp
- Milliman Healthcare Guidelines http://www.milliman.com/solutions/healthcare/

The contractor shall provide real-time entry and adjudication processing for all claim types, including dental claims. This shall allow the WTC Health Program and providers' access to claims information via the Web portal and downloadable reports.

Claims shall be subjected to a number of pre-payment audits, including duplicate checking, utilization review, and editing to ensure claims meet National Correct Coding Initiative (NCCI) edits, OWCP/FECA reimbursement, DRG pricing for inpatient institutional claims, member and provider enrollment status, and member certification conditions.

The contractor claims adjudication system shall use care suites or benefit plan packages and can be configured to determine covered services, service locations, covered providers, service limitations, and rate information for WTC Health Program beneficiaries. The system shall maintain the benefit plan structure online and allows authorized personnel to define any number of plans or programs.

Each care suite is associated in the system with services that are covered under the plan, subject to applicable conditions established by WTC Health Program policies. The services eligible for coverage are associated with one or more specific care suites. Each service-care suite association is governed by rules maintained in the system based on WTC Health Program policy.

The contractor shall submit a transition plan that includes how the Medical Claims Adjudication process with the incumbent contractor will be transferred or rebuilt to ensure optimized operations.

C.3.4.4 Medical and Pharmacy Claims Payment

The contractor shall electronically submit medical and pharmacy claim files to a contractor of the Centers for Medicare & Medicaid Services (CMS) for provider payment. Alternatively, pharmacy claims shall not be routed through the contractor, and shall be routed directly to the CMS contractor. Upon the award, the WTC Health Program will provide guidance on how pharmacy claims will be handled.

Each week, the contractor shall submit two data files to a CMS contractor. One file contains payment information and one file contains claims detail. Using these data, the CMS contractor pays the providers and provides Remittance Advice. The CMS contractor then returns a file to the contractor that provides payment details such as payment confirmation and EFT check numbers.

This process shall be supported by numerous quality assurance, reconciliation, and exception reporting activities, including the following:

- Secure file transmission in both directions with validation of receipt
- Weekly status meetings with the payment contractor
- Weekly payment reconciliation
- Monthly data quality reporting
- Monthly budgetary account reconciliation
- Prompt Pay Interest payment
- Weekly payment integrity process to prevent improper payments
- Expenditure forecasting by budgetary fiscal year
- Payment reissue processing
- Remittance advice troubleshooting and replacement
- Monthly expenditure reporting

C.3.4.5 Claims Reconciliation

The contractor shall support the CCEs, their external providers, and the NPN contractor on processed claims to facilitate reconciliation with submitted claims, corrective actions on denied claims, and dispute resolutions.

The CCEs and the NPN contractor can choose between electronic or a paper remittance advice that provides claim line detail to facilitate bill reconciliation. The CCEs and NPN can choose to receive an electronic payment file in the ANSI 835 format or structured data file format to reconcile claim payment for all their members, including both internal and external providers. The WTC Health Program, the CCEs and NPN receive monthly claims dashboards that provide lists of denied claims and summary charts that show the number and reasons for claims denials. Appeals of denied claims received by the contractor will be forwarded to the designated WTC Health Program representative with supporting documentation as requested to facilitate the Program's decision.

The contractor shall provide dedicated WTC Health Program-trained customer service representatives available to CCEs, the NPN contractor, and external providers to answer questions about their claims payments and denials through meetings, as needed.

The contractor shall provide a secure web-based information exchange system accessible to the WTC Health Program, the CCEs, the NPN contractor, and their providers. The web information exchange system shall provide all of the information necessary to resolve claims issues.

C.3.4.6 Coordination of Benefit and Recovery

The contractor shall prepare a Coordination of Benefit and Recovery (COB&R) Plan. The COB&R Plan shall establish and maintain a robust operationalized system and rules for COB&R functions for all claims involving COB with private and public insurers which shall pay before the WTC Health Program for survivor members and for members who have workers' compensation benefits. The plan shall outline which entity pays first using a variety of methods and programs to identify situations in which the WTC Health Program beneficiaries have other insurance that is primary to WTC Health Program. The contractor's (or its subcontractor's) COB&R activities shall include the collection, management, and reporting of other insurance coverage for beneficiaries and recovery of paid claims which were paid incorrectly or should have been paid by other insurances or coverages.

A draft COB&R Plan shall be submitted with the contractor's proposal. The contractor shall submit a finalized COB&R Plan to the WTC Health Program for approval within 60 days of the contract award. Once approved, an updated COB&R Plan shall be submitted to the WTC Health Program monthly for the duration of the contract.

C.3.5 Data Management, Analytics, and Reporting

In addition to the core claims processing system, the contractor shall develop a consolidated data repository for program reporting, along with a business analytics and reporting capability. This repository shall contain all data on claims processed through the WTC Health Program, allowing for detailed analysis of program operations and health care delivery. In addition, this repository shall be a key tool for program analysis.

This data repository shall also contain data from the current WTC Health Program to allow for various trends and comparisons. As part of this task, the contractor shall convert existing data into the data repository and integrate any other required data sources to provide consolidated reporting for the overall program.
This conversion process and the design of the data repository shall be developed in compliance with FISMA requirements, fully respecting patient privacy and information security.

In addition to the data repository, the contractor shall develop an analytics and reporting capability which will allow the government and other stakeholders (as authorized) to report on program metrics. Key standardized program management reports shall also be developed which will provide a management dashboard for the core operational metrics for the WTC Health Program, including the per-member, per- month (PMPM) costs, the medical loss ratio, and other gauges of the financial health of the program and its overall efficiency and efficacy.

The contractor shall provide the following services related to analytics, decision support, and reporting:

- Scrub Data for completeness and accuracy
- Collect, Transform, and Load Analytics Data
- De-identify data for certain reports
- Develop Analytics Reports for NIOSH Use
- Replicate and transport complete and clean datasets that can be concatenated by unique member ID and CCE for certification (includes WTC Health Program-related health conditions and the health conditions medically associated with the WTC Health Program-related condition), medical (includes medical coding for condition and service/procedure /product, date of service and place of service) and pharmacy service utilization (includes plan cost paid and supply details) on a routine and timely basis to NIOSH.

The contractor shall manage all data to meet a security and privacy framework that supports FISMA requirements and can scale as necessary to support changes in statutes (HIPAA, HITECH, ACA) and associated regulatory and policy guidance.

The contractor shall collect, de-identify, and aggregate data from multiple sources (providers, PBM, etc.) into a data repository to allow NIOSH the ability to generate the necessary reports for analysis and research.

The contractor shall develop analytics reports for NIOSH use. Both analytical and ad hoc query data reports shall be designed to support NIOSH needs in WTC Health Program-related health effects/exposure analysis. The contractor shall provide additional support to NIOSH to support data management and research requirements arising from the program such as:

- Program reporting
- Reporting on program efficiency, efficacy, and program integrity

C.3.5.1 Records Management

The contractor shall develop, implement, and administer a comprehensive records management program. This program shall ensure that accurate WTC Health Program records are maintained in a manner such that the information is secure and readily accessible to be sent to NIOSH. This program shall be approved by the WTC Program Administrator and implemented by the contractor within 1 month of the approval. The contractor's Operations Manual shall provide the details of the records management program.

C.3.5.2 Data Warehouse

The contractor shall create and maintain a data warehouse of all member information, including certified health conditions, and claims for medical and pharmacy benefits provided. The data warehouse shall be compliant with Federal Information Security Management Act (FISMA) and Health Insurance Portability and Accountability Act (HIPAA) requirements.

The contractor data warehouse shall have the capability for ad-hoc reporting, data analyses, graphical data presentation, parameter-driven report generation, and standard reporting. In addition to standardized reports, users may design their own queries and reports.

The contractor's data warehouse shall be fully compliant with HIPAA and the data warehouse security shall incorporate the processes and procedures that ensure the required physical and operational security is in accordance with HIPAA, HITECH, FISMA, and other applicable security and privacy standards.

The contractor shall establish and maintain the data warehouse during the base year of the contract while the WTC Health Program is building its internal capacities that may alter where the contractor obtains source data in subsequent years. The contractor shall ensure continuity of operations that include claims adjudication and other services. The contractor will work closely with the program to tailor processes to establish the system of records as the program transitions data warehousing solutions.

C.3.5.3 HIPAA Compliance and Business Associate Agreement

The WTC Health Program, including any other NIOSH, CDC, or HHS components to the extent that they assist in administering the WTC Health Program involving protected health information (PHI), is a Covered Entity for purposes of the Health Insurance Portability and Accountability Act of 1996 (HIPAA). The contractor is a business associate of the WTC Health Program as that term is defined in the HIPAA Privacy, Security, Breach Notification, and Enforcement Rules (45 C.F.R. pts. 160, 162, and 164). To the extent that the contractor performs functions or activities on behalf of, or provides certain services to, the WTC Health Program where the contractor creates, receives, maintains, or transmits "protected health information" (PHI), the following "HIPAA provisions" apply: the Health Insurance Portability and Accountability Act of 1996 (HIPAA) (Pub. L. 104–191; 42 U.S.C. § 1320d); the Health Information Technology for Economic and Clinical Health (HITECH) Act (Pub. L. 111-5; 42 U.S.C. §§ 300jj etseq.); the HIPAA Privacy, Security, Breach Notification, and Enforcement Rules (45 C.F.R. pts. 160, 162, and 164); and HHS HIPAA policies.

The contractor shall require its providers and any and all agents and subcontractors to comply with all applicable provisions of HIPAA and shall document in writing the policies and procedures that will be used to meet such obligations. The contractor shall provide the WTC Program Administrator with a copy of their Notice of Use and Disclosures of Protected Health Information required by the HIPAA Privacy rule for review and approval and ensure in that notice that the members are aware the WTC Program Administrator will have access to the member's protected health information for purposes of the treatment provided by and administration of the WTC Health Program established under the James Zadroga 9/11 Health and Compensation Act. Furthermore, the contractor's Operations Manual shall provide the details of how the contractor will meet the obligations and activities of a Business Associate of the WTC Health Program.

At all times throughout the duration of this contract and until the contractor fulfills its obligations under this contract and the Business Associate Agreement, the contractor is subject to and shall comply with all applicable HIPAA provisions regarding business associates, as well as any updates to those provisions.

As a Business Associate of the WTC Health Program, the contractor is directly liable under HIPAA for the following violations:

- Impermissible uses and disclosures of PHI
- Failure to provide breach notification to the Covered Entity
- Failure to provide access to a copy of electronic PHI to either the Covered Entity, the individual, or the individual's designee, as specified in the Business Associate Agreement
- Failure to disclose PHI where required by the Secretary of HHS to investigate or determine the Business Associate's compliance with HIPAA
- Failure to provide an accounting of disclosures
- Failure to comply with the requirements of the Security Rule

C.3.5.4 Transfer of Data and Information

At the end of the contract or at a time determined by the WTC Program Administrator, the contractor shall transfer to the WTC Program Administrator or a subsequent contractor all data and information necessary to transition operations, including: medical records, data and reference tables; WTC-specific data, configurations, process documents, and specifications that are available at the completion of the contract; documentation relating to software and interfaces; functional business process flows; provider network data and agreements; and operational information, including correspondence, documentation of ongoing or outstanding issues, operations support documentation, and operational information regarding subcontractors. For purposes of this provision, "documentation" means all operations, technical and user manuals used in conjunction with the software, services and deliverables, in whole or in part, that the WTC Program Administrator determines are necessary to view and extract application data in a proper format. The contractor shall provide the documentation in the formats in which such documentation exists at the expiration or completion of the contract.

The contractor shall provide to the WTC Program Administrator all data, information, and services necessary and sufficient to enable the WTC Program Administrator to map all data from the contractor's system(s) to the replacement system(s) of the WTC Program Administrator or a successor contractor, including a comprehensive data dictionary that gives the names and definitions of all data fields in the database.

The contractor shall provide to the WTC Program Administrator all necessary data, information, and services in the format defined by the WTC Program Administrator, and must be HIPAA compliant. All of the data, information and services mentioned in this section shall be provided and performed in a manner bythe contractor using its best efforts to ensure the efficient administration of the WTC Health Program. The data and information must be supplied in a medium and format specified by the WTC Program Administrator and according to the schedule approved by the WTC Program Administrator. All relevant data and information must be received and verified by the WTC Program Administrator. The contractor's Operations Manual shall provide the details of the transfer of data and information and turnover-of-services procedure.

C.3.6 Fraud, Waste, and Abuse Compliance Plan and Reporting

The contractor's Fraud, Waste, and Abuse Compliance Plan shall address how the contractor will mitigate, detect, respond to and prevent Fraud, Waste and Abuse (FWA) incidents.

1. Reporting requirements for the FWA Compliance Plan are as follows:
   The contractor shall notify the CO and COR within ten (10) business days of the discovery of

possible or actual FWA incidents related to covered services that warrant investigation.

a. If a possible FWA incident is determined not to be an actual incident, the contractor shall submit a final report to the CO and COR within ten (10) business days of that determination date but no later than forty (40) business days from the incident discovery date.

b. If a possible FWA incident is determined to be an actual case, the contractor shall provide a preliminary FWA report of their actions to the CO and COR within ten (10) business days of that determination. The preliminary report shall contain a brief synopsis of the situation and actions the contractor has taken and will take to address the matter, and to prevent it from recurring.

c. For an actual incident, the contractor shall provide a full report of their actions to the CO and COR within forty (40) business days of the determination date. Such report shall be reviewed and signed by the key staff or designee and contain at least the following information:

1) The name of the individual or entity that committed the fraud, waste, orabuse;
2) The source or surveillance method that identified the fraud, waste, or abuse;
3) The type of provider, entity or organization that committed the fraud, waste, or abuse;
4) A description of the fraud, waste, or abuse;
5) The approximate dollar amount of the fraud, waste, or abuse if applicable;
6) The legal and administrative disposition of the case including actions taken by law enforcement officials to whom the case has been referred; and
7) Other data/information deemed pertinent by the contractor or as prescribed by the WTC Program Administrator.

2. A monthly report shall include summaries of all open possible and actual FWA incidents and the status of the contractor's actions on each incident.

3. FWA requirements apply to the full scope of contract efforts/activities.

4. FWA definitions:

Fraud is defined as an intentional deception or misrepresentation made by a person with the knowledge that the deception could result in some unauthorized benefit to him or some other person. It includes any act that constitutes fraud under applicable Federal or State law.[1]

Examples include but are not limited to:

- Billing or filing a claim for services that were not rendered
- Altering claim forms to receive a higher payment rate
- Receiving services by nonmembers using a legitimate member's enrollment card[2]
- Prescribing medications for member conditions where the member is not certified for that particular condition

Waste is defined as failure to control costs or regulated payment associated with federal program funding. Furthermore, waste results in taxpayers not receiving reasonable value for their money in connection with any Government-funded activities due to an inappropriate act or omission by payers with control over or access to Government resources. Importantly, waste relates primarily to mismanagement, inappropriate actions, or inadequateoversight."[3]

Examples include but are not limited to:

- Use of higher-priced services that have negligible or no health benefits over less-

expensive alternatives
- Administrative complexity[4]
- Writing high-cost prescriptions over less expensive but equivalent generic prescriptions
- Failures of care delivery or of care coordination
- Overtreatment

Abuse is defined as practices that are inconsistent with professional standards of care; medical necessity; or sound fiscal, business, or medical practices.[5] [6]

Examples included but are not limited to:
- Billing for services that were not medically necessary
- Charging excessively for services or supplies[7]
- Coding and billing for 99215 Office or other outpatient visit Evaluation and Management (E/M) for one hour when the visit was actually only 15 minutes.

Importantly, intent is the key distinction between Fraud and Abuse. An allegation of waste and abuse can escalate into a fraud investigation if a pattern of intent is determined.

C.3.7 Quality Assurance and Internal Audits

The contractor's administrative procedures shall include a Quality Assurance component. Responsibilities for Quality Assurance shall include all scope of work items including but not limited to:
- Program Management
- Member Services
- Claims Processing
- Data Management, Analytics, Decision Support, and Reporting
- Fraud Waste and Abuse Compliance Plan and Reporting

The contractor shall be responsible for developing and documenting its approach to Quality Assurance. The contractor shall perform all required services in accordance with their established Quality Assurance Program. The contractor shall develop a project-specific Quality Assurance Plan (QAP) which describes the Quality Assurance Program and the procedures which will be used to monitor and improve all project activities including reporting.

The g Quality Assurance Program shall include a customer satisfaction survey of the CCEs, the NPN contractor, and NIOSH to be conducted at least annually.

As a part of their Quality Assurance Program, the contractor shall conduct internal audits of each of the parts of the WTC Health Program listed below. The contractor shall submit a summary audit report to the WTC Program Administrator within 45 days of the end of each quarter. These audits shall include all scope of work items, including but not ~~be~~ limited to:

- Member Enrollment Processing
- Certification Processing
- Provider Enrollment
- Call Center Activities
- Claims Processing
- Data Management
- Project Management Activities

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[1] Definitions, 42 C.F.R. § 455.2 (2010). Retrieved August 20, 2014 from http://www.gpo.gov/fdsys/pkg/CFR-2010-title42-vol4/xml/CFR-2010-title42-vol4-sec455-2.xml

[2] U.S. Department of Health and Human Services, Centers for Medicare and Medicaid Services. (2013). *2013 national training program: Module 10: Medicare and Medicaid fraud prevention workbook*. Retrieved August 20, 2014 from

http://www.cms.gov/Outreach-and-Education/Training/CMSNationalTrainingProgram/Downloads/2013-Fraud-and-Abuse-Prevention-Workbook.pdf

[3] Walker, D. (2007). *Transforming government, management, and accountability*. (GAO-07-1214CG). Retrieved August 20, 2014 fromhttp://www.gao.gov/cghome/d071214cg.pdf

[4] Lallemand, N. C. (2012). Health policy brief: Reducing waste in health care. *Health Affairs*. Retrieved August 20, 2014 from http://www.healthaffairs.org/healthpolicybriefs/brief.php?brief_id=82

[5]Definitions, 42 C.F.R. §455.2 (2010). Retrieved August 20, 2014 from http://www.gpo.gov/fdsys/pkg/CFR-2010-title42-vol4/xml/CFR-2010-title42-vol4-sec455-2.xml

[6] U.S. Department of Health and Human Services, Centers for Medicare and Medicaid Services. (2013). *2013 national training program: Module 10: Medicare and Medicaid fraud prevention workbook*. Retrieved August 20, 2014 from http://www.cms.gov/Outreach-and-Education/Training/CMSNationalTrainingProgram/Downloads/2013-Fraud-and-Abuse-Prevention-Workbook.pdf

[7] U.S. Department of Health and Human Services, Centers for Medicare and Medicaid Services. (2012, November). Medicare fraud & abuse: prevention, detection, and reporting. (ICN 006827). Retrieved August 20, 2014 from http://www.cms.gov/Outreach-and-Education/Medicare-Learning-Network-MLN/MLNProducts/downloads/Fraud_and_Abuse.pdf

Contractor Metrics/Quality Assurance Surveillance Plan (QASP)

The contractor shall establish and continuously operate an internal quality management/quality improvement program covering every aspect of its operation, both clinically and administratively. The contractor shall provide a quarterly written report and briefing in person or via video teleconference to the COR and NIOSH staff on its ongoing internal quality improvement program. The contractor shall submit a draft Quality Assurance Surveillance Plan based on the following suggested metrics:

Performance Objectives	Performance Standard References	Performance Standard	Acceptable Quality Levels (AQL)	Methods of Surveillance	Incentives
1. Submit deliverables and monthly project management report of scheduled tasks, completed tasks, outstanding tasks, and any additional administrative reports	C.3.1 HPS Contract Management C.6.1 Deliverables Required by the PWS C.6.2 Monthly Reports C.6.3 Additional Administrative Reports	Submitted within 10 days after the end of the previous month	No more than 2 submissions are delivered greater than 10 - 20 days after the end of the previous month No submissions are delivered greater than 20 days after the end of the previous month	Document reviews, periodic and random inspections, observations, and customer complaints	Failure to meet 100% AQL in two consecutive months will require a corrective action plan and result in a lower performance rating
2. Develop and implement an on-going Quality Assurance Program (QAP) which includes conducting internal audits, management of subcontractors, resolving complaints, and conducting customer satisfaction surveys	C.3.1 HPS Contract Management C.3.1.7 Quality Assurance and Internal Audit Plan C.3.1.9 Subcontracting C.3.7 Quality Assurance and Internal Audits C.3.6 FWA Compliance Plan and Reporting	Submissions depict accurate findings All components of the QAP are addressed in each report with sufficient detail to provide a clear understanding of any corrective actions Submitted within 45 business days after the end of the previous quarter	No submissions are delivered greater than 45 days after the end of the previous quarter	Document reviews, periodic and random inspections, observations, and customer complaints	Failure to meet 100% AQL in two consecutive quarters will require a corrective action plan and result in a lower performance rating

Performance Objectives	Performance Standard References	Performance Standard	Acceptable Quality Levels (AQL)	Methods of Surveillance	Incentives
3. Ensure customer satisfaction (customer refers to NIOSH and NIOSH contractors)	C.3.1 HPS Contract Management C.3.2 Member Services and Medical Benefit Support	All customer complaints are reported monthly All valid customer complaints are resolved within a timely manner A quarterly satisfaction survey is conducted nd reported as part of the Quarterly Quality Assurance Internal Audits	No more than 2 valid complaints per month or 3 valid complaints per quarter	Document reviews, periodic and random inspections, observations, and customer complaints	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating
4. Provide member services including eligibility, certification, and appeal processes	C.3.2 Member Services and Medical Benefits Support C.3.2.1 Medical Condition Certification C.6.1 Deliverables Required by the PWS	Work flow is sufficient to provide time-sensitive services to accommodate an average of 1400 applications, 1400 certification requests, and 100 appeals monthly	95% of all services are completed within the required time periods as defined in C.6.1, Deliverables Required by the PWS	Document reviews, periodic and random inspections, observations, and customer complaints	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating
5. Maintain a call center operation to receive and respond to inquiries from potential enrollees, members, and providers	C.3.3.4 Call Center	Work flow is sufficient to accommodate at least 300 calls per day on average in English and at least 15 'other languages' Use of a third-party translation and interpretation services must be available for 'other languages'	Volume shall never drop below 90% of thresholds Zero occurence of consecutive months in which the requirements are not met	Document reviews, periodic and random inspections, observations, and customer complaints Integration test with claim processing system, member database and application system	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating

Performance Objectives	Performance Standard References	Performance Standard	Acceptable Quality Levels (AQL)	Methods of Surveillance	Incentives
6. Establish and maintain a credentialed provider network	C.3.2 Member Services and Medical Benefits Support C.3.3 Healthcare Provider Network Management C.3.2.2 Case Management	Development and maintenance of a network of credentialed providers with access to specialties which are sufficient to treat WTCHP disease conditions	100% of provider availability to meet the needs of the Program monthly No more than 5 providers recruited monthly as a result of lack of provider coverage to meet the needs of the Program	Document reviews, periodic and random inspections, observations, and customer complaints Integration test with other automated systems Validation of providers network (to include: cancer care systems, medical and surgical specialists, ancillary support services, CMS-approved organ transplantation, emergency care, same-day surgery, inpatient facilities, and mental health services)	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating

Performance Objectives	Performance Standard References	Performance Standard	Acceptable Quality Levels (AQL)	Methods of Surveillance	Incentives
8. Provide an interactive and secure self-service portal for government users, case managers, other contractors and providers	C.3.3.2 Secure location for WTC Health Program Information Submission C.3.3.3 Secure Web-based Information Exchange System	The secure web portal must meet federal government and CDC's standards with zero deviation The portal must be highly configurable to address the unique needs of the WTCHP for federal users, providers, case managers, and other contractors	No more than 2 valid user complaints per month	Document reviews, periodic and random inspections, observations, and customer complaints Integration test Validation Audit report Program Administration Web Portal Test	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating.
9. Provide a comprehensive and secure claim processing, adjudication and reconciliation system through a web-based information exchange system	C.3.4 Claims Processing C.3.4.3 Medical Claims Adjudication C.3.4.4 Medical and Pharmacy Claims Payment C.3.4.5 Claims Reconciliation	Claims System shall accommodate at least 500,000 claims annually While, over 95% of claims are submitted electronically, there must be a capability to process paper claims within 5 days of receipt Adequate industry-standard information shall be provided for claims reconciliation (when requested by CCEs and the NPN contractor)	Volume shall never drop below 90% of thresholds Zero occurrence of consecutive months in which the requirements are not met	Document reviews, periodic and random inspections, observations, and customer complaints Integration test Validation Audit report	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating

Performance Objectives	Performance Standard References	Performance Standard	Acceptable Quality Levels (AQL)	Methods of Surveillance	Incentives
10. Develop a consolidated data repository for program reporting, along with an analytics and reporting capability	C.3.5 Data Management, Analytics, and Reporting C.3.5.1 Records Management C.3.5.2 Data Warehouse C.3.5.3 HIPAA Compliance C.3.5.4 Transfer of Data and Information C.6.3 Additional Administrative Reports Attachment #2 Information Technology Security and Privacy	Standardized, accurate, and secure database, clean datasets, analytics and reports are available and accessible for authorized users within time parameters established in a Data Access Plan Ensure data management meets federal security standards with scalable framework necessary to support changes in statutes nd associated regulatory and policy guidance	Zero occurrence of consecutive months in which the requirements are not met Zero occurrence in which timeliness requirements are more than 10 days late	Document reviews, periodic and random inspections, observations, and customer complaints Integration test Validation Audit report Analytical products, reports and program metrics Data System Security Plan Review Operations Manual that includes details of the records management program	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating
11. Implement Fraud, Waste, and Abuse Program according to federal law, regulations, policies and the FWA Compliance Plan, and report monthly	C.3.6 Fraud, Waste, and Abuse Compliance Plan and Reporting	All suspected, investigated and or confirmed FWA incidents are timely reported to NIOSH according to FWA guidance 100% of the time	100% of contractor's availability to meet the needs of the Program monthly	Document reviews, periodic and random inspections, observations, and customer complaints Validate monthly report, address approaches and changes to practice in quarterly report	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating
12. Operationalize the Coordination of Benefit and Recovery Program	C.3.4.6 Coordination of Benefit and Recovery	The claims processing and adjudication system shall process COB&R functions for all members under the survivor category	100% of contractor's availability to meet the needs of the Program monthly	Document reviews, periodic and random inspections, observations, and customer complaints Validate monthly report, address approaches and changes to practice in quarterly report	Failure to meet 100% AQL will require a corrective action plan and result in a lower performance rating

C.4 Background and Experience

The contractor shall have experience in providing healthcare services of a type similar or greater in scope, size, and complexity to those required under the WTC Health Program. Relevant experience shall include providing all items in this PWS, including but not limited to:

- Program Management
- Member Services and Case Management
- Claims Processing
- Data Management, Analytics, Decision Support, and Reporting
- Fraud Waste and Abuse Compliance Plan and Reporting
- Quality Assurance and Internal Audits

C.5 Personnel

The contractor shall have personnel assigned to this project that have the experience, educational background, and record of past accomplishment appropriate to the scope of the effort. The contractor's Staffing Plan shall provide the qualifications and experience of the personnel who will perform duties under the contract and which services they will perform.

C.5.1 Key Personnel

The contractor shall list all personnel designated as key personnel for this contract along with their years of relevant experience and the functions they will perform on this contract. The key personnel shall not be removed from the contract or replaced without the prior approval of theCO.

At a minimum, key personnel shall include a Program Manager who is highly qualified and have experience in managing and administering similar scope and scale of the federal health program.

The Program Manager shall have overall responsibility for this contract and shall serve as the contractor's primary point of contact with the WTC Health Program on all issues, including non-medical issues. The contractor's Program Manager shall be highly knowledgeable of all aspects of the WTC Health Program. (Refer to HHSAR 352.273-75 Key Personnel.)

C.6 Deliverables

All deliverables required under this contract shall be packaged, marked, and shipped in accordance with the guidance found in this section. The contractor shall guarantee that all required materials shall be delivered in immediately usable and acceptable condition.

Reports submitted under the contract shall reference and cite the contract number and identify HHS as the sponsoring agency. All contract deliverables must be reviewed and approved by the CO and the COR. Satisfactory reporting performance under the contract shall be deemed to occur upon delivery and acceptance hy the CO, or a duly authorized representative.

The data and analyses to be reported to the WTC Program Administrator shall include all information required tn effectively administer and manage this effort. Minimum required data are listed below. The contractor shall provide a listing and short description of standard reports that will provide these data and analyses of these data, as well as any other data and analyses that the contractor considers to be valuable to the WTC Program Administrator. All reports shall be submitted monthly unless otherwise specified. Based on the contractor's best knowledge and belief, the contractor shall attest to the accuracy, completeness, and truthfulness of the reports being submitted. The contractor shall ensure that the WTC Program Administrator does not receive any personal data, including protected health information, on the members (names or other identifiers) in these reports.

The contractor shall base their proposed reporting formats and content on their experience with other similar programs. The contractor shall analyze the data collected during each reporting period and ensure

that all reports provide summaries which highlight significant trends. All reports shall include a narrative companion text document that summarizes the report data and explains any unusual occurrences or trends.

C.6.1 Deliverables Required by the PWS

PWS J-1 Reference	Deliverable Title	Date Due
C.3.1	Project Management Plan	Draft with proposal; finalized version within 30 days of contract award; updates to be reported in the Monthly Reports.
C.3.1	Operations Manual	Draft with proposal; finalized version within 60 days of contract award; updates to be reported in the Monthly Reports. Substantial re-write of this manual to be provided annually.
C.3.1	Communications Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates to be reported in the Monthly Reports. Substantial re-write of this plan to be provided annually.
C.3.1	Data Security and Continuity of Operations Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates to be reported in the Monthly Reports. Substantial re-write of this plan to be provided annually.
C.3.1	Risk Management Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates to be reported in the Monthly Reports. Substantial re-write of this plan to be provided annually.
C.3.1	Fraud, Waste, and Abuse Compliance Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates to be reported in the Monthly Reports. Substantial re-write of this plan to be provided annually.
C.3.1	Quality Assurance and Internal Audit Plan	Draft with proposal; finalized version within 60 days of contract award; updates to be reported in the Monthly Reports. Substantial re-write of this plan to be provided annually.
C.3.1	WTC Health Program Reconciliation Report on the overall Financial Status of the contract.	The 10th day of the month for the previous quarter's information
C.3.1	Subcontracting	Draft prior to award
C.3.1	Program Improvement Plan	Draft within 60 days of contract award; finalized version 2 weeks from receipt of comments; updates monthly
C.3.4	Coordination of Benefit and Recovery Plan	Draft with proposal; finalized version within 60 days of contract award; updates monthly
C.3.7	Quality Assurance and Internal Audits	Summary report due 45 days after the end of each quarter
C.3.2.3	Transition Plan for the Medical Claims Adjudication Process with the Incumbent Contractor	Draft with proposal; finalized version within 30 days of contract award; updates to be reported in the Monthly Reports.
C.6.2	Monthly Reports	The 10th day of the month for the previous month's information
-	Annual Reports	The 10th day of the month following the contract year end
C.6.3	Additional Administrative Reports	As requested
IT J-2 Reference	**Deliverable Title**	**Date Due**
7.3	PII Security Plan	Draft due with proposal; finalized version due within 45 days of contract award
7.3.3	PTA/PIA	Due within 30 days of contract award
17.3-17.5	FIPS 199; SA&A; SSP; SAR; POA&M; Contingency Plan	Due within 45 days of contract award

C.6.2 Monthly Reports

The contractor shall develop and submit Monthly Reports to the COR and CO. The Monthly Reports shall include a summary of activities for the month, issues, and goals for the next month. The Monthly Reports shall summarize the contractor's accomplishments during the previous month, planned work during the upcoming month and any significant problems or issues requiring resolution, along with proposed corrective actions. It shall also include a description of any substantive changes to any manuals and plans detailed in C.3. The Monthly Reports shall include all scope of work items including but not limited to, the following information:

- Contract Number
- Program Management Operations and Activities
- Member Services Support including case management services
- Call Center Operations
- Medical Benefits Support
- Provider Network Support
- Claims Processing Activities
- IT Architecture, Security, and Implementation Testing
- Fraud, Waste, and Abuse Activities
- Quality Assurance Activities
- Cost Alerts

Format

Except as otherwise specified herein, the contractor shall prepare and submit to NIOSH the reports required under this section in an agreed-upon media format on the 10th day of the month for the monthly reports. All reports including protected health information or other personal data of members must be securely transmitted in compliance with HIPAA and CDC, NIOSH, and WTC Health Program protocols.

C.6.3 Additional Administrative Reports

Upon request by the CO, the contractor shall prepare and submit additional administrative reports. The contractor shall prepare reports to track applicants, members, certifications, and claims; review paid, pended, and denied claims; and conduct utilization review for quality assurance and appropriate health care. The contractor's reporting capabilities shall be based on the entire set of WTC Health Program data including, but not limited to, members' demographic and enrollment information, members' certification information, medical claims information, pharmacy claims information, and provider data. Such requests will be limited to situations in which the desired data is considered essential and cannot be obtained through existing reports. Whenever possible, the contractor will be provided with a 90-day notice and the opportunity to discuss and comment on the proposed requirements before work is begun. However, the COR on behalf of the Program Administrator reserves the right to give a 5-day notice in circumstances where time is of the essence. The contractor shall prepare and submit to the CO and Program Administrator, through the COR, any other data reports in e-mail format.

The CO may extend due dates or modify report requirements or formats upon a written request by the contractor to the CO, where the contractor has demonstrated a good and compelling reason for the extension or modification. The determination to grant a modification or extension of time shall be made by the CO.

C.7 Period of Performance

The period of performance for this contract is as follows:

Base Period:	December 01, 2016 – May 31, 2017
Option Period 1:	June 01, 2017 – May 31, 2018
Option Period 2:	June 01, 2018 – May 31, 2019
Option Period 3:	June 01, 2019 – May 31, 2020
Option Period 4:	June 01, 2020 – May 31, 2021